adhere+
chronic inflammatory demyelinating polyneuropathy study

Clinical Study Protocol

Protocol Title:	Open-label Extension of the ARGX-113-1802 Trial to Investigate the Long-term Safety, Tolerability, and Efficacy of Efgartigimod PH20 SC in Patients With Chronic Inflammatory Demyelinating Polyneuropathy (CIDP)
Protocol Number:	ARGX-113-1902
Version Number:	8.0
Compound:	Efgartigimod (ARGX-113)
Study Phase:	2
Acronym:	ADHERE+
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Regulatory Agency Identifier Number(s):	EudraCT number: 2019-003107-35 EU CT number: 2023-507885-21 IND number: 143294
Approval Date:	02 Jul 2024

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NOTIFICATION: POSSIBLE ADAPTATIONS OF STUDY PROTOCOL DURING THE COVID-19 PANDEMIC

The aim of study ARGX-113-1902 is to assess the long-term safety and tolerability of efgartigimod PH20 SC in participants with chronic inflammatory demyelinating polyneuropathy (CIDP). This treatment, which consists of efgartigimod for subcutaneous (SC) administration coformulated with recombinant human hyaluronidase PH20 (rHuPH20), could offer clinically significant benefits to patients with CIDP.

argenx has performed a critical assessment of the use of efgartigimod during the COVID-19 pandemic. After careful evaluation, the benefit-risk profile of efgartigimod use in ongoing clinical studies has not changed in the context of this pandemic. This decision was made based on efgartigimod’s mechanism of action, the safety data generated to date, and provisions made in all clinical studies with efgartigimod for safety reporting and withholding treatment upon evidence of infection. This assessment will be reviewed regularly to consider new information about the pandemic and the ongoing, continuous assessment of adverse events reported during argenx clinical studies.

During the COVID-19 pandemic, it may not be possible to perform all study assessments as planned (Section 1.3).

To provide participants with CIDP the opportunity to continue the study during the COVID-19 pandemic, an Appendix with possible adaptations to ARGX-113-1902 has been developed for sites to follow if a participant cannot complete a visit at the study site and a visit at home or an alternative convenient location will be performed. This Appendix describes a minimum number of assessments required to guarantee the safety and well-being of participants during the study and to secure the collection of the critical parameters for analysis. This Appendix is included in Section 10.8 (Appendix 8) of this protocol.

SIGNATURE OF THE SPONSOR

Protocol Title:	Open-label Extension of the ARGX-113-1802 Trial to Investigate the Long-term Safety, Tolerability, and Efficacy of Efgartigimod PH20 SC in Patients With Chronic Inflammatory Demyelinating Polyneuropathy (CIDP)
Protocol Number:	ARGX-113-1902

Sponsor Signatory:

See appended signature page

Luc Truyen, MD, PhD **Chief Medical Officer**	**Date**

SIGNATURE OF THE INVESTIGATOR

Investigator's Acknowledgment

I have read the protocol for study ARGX-113-1902.

Title: Open-label Extension of the ARGX-113-1802 Trial to Investigate the Long-term Safety, Tolerability, and Efficacy of Efgartigimod PH20 SC in Patients With Chronic Inflammatory Demyelinating Polyneuropathy (CIDP)

I have fully discussed the objective(s) of this study and the contents of this protocol with the sponsor's representative.

I understand that the information in this protocol is confidential and will not be disclosed, except to those directly involved in the execution or the scientific/ethical review of the study, without written authorization from the sponsor. It is, however, permissible to provide the information contained herein to a participant in order to obtain their consent to participate.

I agree to conduct this study according to this protocol and to comply with its requirements, subject to ethical and safety considerations and guidelines, and to conduct the study in accordance with International Council for Harmonisation of Technical Requirements for Pharmaceuticals for Human Use guidelines on Good Clinical Practice and with the applicable regulatory requirements.

I understand that failure to comply with the requirements of the protocol can lead to the termination of my participation as an investigator for this study.

I understand that the sponsor can decide to suspend or prematurely terminate the study at any time for any reason; such a decision will be communicated to me in writing. Conversely, if I decide to withdraw from execution of the study, I will communicate my intention immediately in writing to the sponsor.

Investigator Name ______________________________
Institution ______________________________
Address ______________________________

(please handprint or type) ______________________________

Signature ______________________________

Date ______________________________

PROTOCOL AMENDMENT SUMMARY OF CHANGES

Global protocol document history	Date
Amendment 7 v8.0	02 Jul 2024
Amendment 6 v7.0	05 Feb 2024
Amendment 5 v6.0	28 Sep 2022
Amendment 4 v5.0	22 Jul 2022
Amendment 3 v4.0	07 Jan 2021
Amendment 2 v3.0	26 May 2020
Amendment 1 v2.0	10 Jan 2020
Original v1.0	08 Nov 2019

Amendment 7 (02 Jul 2024)

This amendment is considered substantial based on the definition in Article 2 §2 (13) of the Regulation (EU) No 536/2014 of the European Parliament and the Council of the European Union.

Overall Rationale for the Amendment

The primary rationale for this amendment was to change the source of investigational medicinal product (IMP) from a single-dose vial to a single-dose prefilled syringe (PFS).

The major changes from global protocol version 7.0 compared to global protocol version 8.0 are summarized in the following table. Minor editorial changes (eg, to improve readability or correct spelling mistakes) are not included. **Bold** font indicates new text, and ~~strikethrough~~ font indicates deleted text.

The changes in previous protocol amendments are provided in Section 10.10. Refer to the List of Abbreviations for any undefined terms or abbreviations.

Section	Change in study conduct or planned analysis	Brief rationale
Throughout the protocol	As of protocol v8.0, participants will change their source of IMP from a single-dose vial to PFS. After PFS implementation, the site staff will provide a “PFS Instructions for Use” document at the first supervised PFS IMP administration visit.	To increase participant convenience with a PFS administration
Table 2: Schedule of Activities for Subsequent Treatment	The end date of the study (30 Apr 2027) has been added.	To clarify the end of the study

Section	Change in study conduct or planned analysis	Brief rationale
Periods After the First 48-Week Treatment Period 6.7. Continued Access to Efgartigimod After the End of the Study Table 6: Schedule of Activities for Subsequent Treatment Periods After the First 48-Week Treatment Period During COVID-19 Pandemic Table 7: Schedule of Activities During the Optional Substudy to Explore Less Frequent Dosing		
4.6. Protocol Deviations	Added text **Upon awareness, the investigator (or designee) must report a likely significant protocol deviation to the sponsor (or designee).**	To clarify expectations of the investigator and site regarding the timing of protocol deviation reporting
6.7. Continued Access to Efgartigimod After the End of the Study	Added text **At the end of the study, argenx cannot guarantee continued access for participants but will comply with all local laws and regulations.**	To implement Health Authorities' recommendations
8.4.4. Regulatory Reporting Requirements for SAEs	Updated text The sponsor or designee will be responsible for reporting SUSARs to the ~~relevant regulatory authorities and IEC/IRB,~~ **appropriate reporting system (eg, EudraVigilance database, FDA AE reporting system)** per **the** applicable regulatory requirements. The sponsor or designee will also be responsible for forwarding SUSAR reports to all study investigators, who ~~will be required to~~ **must** report these SUSARs to their respective IECs/IRBs per local regulatory requirements.	To implement Health Authorities' recommendations

Section	Change in study conduct or planned analysis	Brief rationale
Table 7: Schedule of Activities During the Optional Substudy to Explore Less Frequent Dosing (footnote f)	Added text **Participants will be asked to sign the current ICF version before receiving the first PFS dose and before starting a new 48-week treatment period to continue the study. The first day of a new treatment year will coincide with the last visit of the previous treatment year.**	To clarify guidance regarding informed consent for the optional substudy and new source of IMP

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LIST OF ABBREVIATIONS AND DEFINITIONS OF TERMS

List of Abbreviations

Abbreviation	Expansion
21 CFR	Title 21 of the Code of Federal Regulations
AChR-Ab	anti-acetylcholine receptor antibody(ies)
ADA	antidrug antibody(ies)
AE	adverse event
AESI(s)	adverse event(s) of special interest
aINCAT	adjusted Inflammatory Neuropathy Cause and Treatment (INCAT) disability score
ASCT	autologous stem cell transplant
BPI-SF	Brief Pain Inventory – Short Form
CIDP	chronic inflammatory demyelinating polyneuropathy
CIOMS	Council for International Organizations of Medical Sciences
CNS	central nervous system
CRO	contract research organization
CTCAE	Common Terminology Criteria for Adverse Events
CSR	clinical study report
DSMB	data and safety monitoring board
ECG	electrocardiogram
eCRF	electronic case report form
ED	early discontinuation
efgartigimod PH20 SC	efgartigimod for SC administration coformulated with rHuPH20
EU	European Union
FcγR	Fc gamma receptor
FcRn	neonatal crystallizable fragment receptor
FSH	follicle-stimulating hormone
FSS	Fatigue Severity Scale
GBS	Guillain-Barré syndrome
GCP	Good Clinical Practice
gMG	generalized myasthenia gravis

Abbreviation	Expansion
HADS	Hospital Anxiety and Depression Scale
HBV	hepatitis B virus
HCV	hepatitis C virus
HDL	high-density lipoprotein cholesterol
IB	Investigator's Brochure
ICF	informed consent form
ICH	International Council for Harmonisation of Technical Requirements for Pharmaceuticals for Human Use
IEC	Independent Ethics Committees
Ig	immunoglobulin
IgG	immunoglobulin γ
IMP	investigational medicinal product
IRB	Institutional Review Board
I-RODS	Inflammatory Rasch-built Overall Disability Scale
ITP	immune thrombocytopenia
IUD	intrauterine device
IUS	intrauterine hormone-releasing system
IV	intravenous(ly)
IVIg	intravenous immunoglobulin
LDL	low-density lipoprotein cholesterol
MedDRA	Medical Dictionary for Regulatory Activities
MG	myasthenia gravis
MG-ADL	Myasthenia Gravis Activities of Daily Living
mITT	modified intent-to-treat
MGUSP	gammopathy-related polyneuropathy
MRC	Medical Research Council
NAb	neutralizing antibody(ies)
NCI	National Cancer Institute
OLE	open-label extension
PD	pharmacodynamic(s)

Abbreviation	Expansion
PFS	prefilled syringe
PHQ-9	Patient Health Questionnaire-9
PK	pharmacokinetic(s)
PPE	personal protective equipment
PT	Preferred Term
PRO	patient-reported outcome
QoL	quality of life
rHuPH20	recombinant human hyaluronidase PH20
RT-FSS	Rasch-Transformed Fatigue Severity Scale
SAE	serious adverse event
SAP	statistical analysis plan
SC	subcutaneous(ly)
SCIg	subcutaneous immunoglobulin
SD1	study day 1
SFV	safety follow-up visit
SoA	schedule of activities
SOC	System Organ Class
SOP	standard operating procedure
SUSAR	suspected unexpected serious adverse reaction
TEAE	treatment-emergent adverse event
TSQM(-9)	(9-item) Treatment Satisfaction Questionnaire for Medication
TUG	Timed Up and Go
Unsch	unscheduled
US	United States
V	visit
W	week
WOCBP	women of childbearing potential

Definitions of Terms

Term	Definition
Clinical deterioration	An increase of ≥1 point in aINCAT score compared with ARGX-113-1902 baseline
Terms from the antecedent study ARGX-113-1802	
Clinical deterioration	An increase of ≥1 point in aINCAT score compared with Stage B baseline
Stage A	Open-label stage in ARGX-113-1802 (up to 13 weeks)
Stage B	Randomized-withdrawal, double-blinded, placebo-controlled stage in ARGX-113-1802 (up to 48 weeks)

1. PROTOCOL SUMMARY

1.1. Synopsis

<table>
<tr><td>Protocol number: ARGX-113-1902</td><td>Drug: ARGX-113/efgartigimod</td></tr>
<tr><td colspan="2">Title of the study: Open-label Extension of the ARGX-113-1802 Trial to Investigate the Long-term Safety, Tolerability, and Efficacy of Efgartigimod PH20 SC in Patients With Chronic Inflammatory Demyelinating Polyneuropathy (CIDP)</td></tr>
<tr><td colspan="2">Number of participants (total):
The maximum number of participants in this study will be the number of participants who entered the antecedent study ARGX-113-1802, ie, up to 360 participants.</td></tr>
<tr><td colspan="2">Investigator(s): Multicenter study</td></tr>
<tr><td colspan="2">Site(s) and Region(s): Approximately 140 sites globally (planned)</td></tr>
<tr><td>Study start (planned): 2020</td><td>Clinical phase: Phase 2</td></tr>
<tr><td colspan="2">Objectives:
<u>Primary objective</u>:
• To assess the long-term safety and tolerability of efgartigimod PH20 SC (efgartigimod for subcutaneous [SC] administration coformulated with recombinant human hyaluronidase PH20 [rHuPH20]).
<u>Secondary objectives</u>:
• To determine the long-term treatment effect.
• To evaluate the immunogenicity (antidrug antibodies [ADA] against efgartigimod during the first 48-week treatment period [followed by a safety follow-up period, if applicable]).
• To evaluate the pharmacokinetics (PK) of efgartigimod PH20 SC (during the first 48-week treatment period [followed by a safety follow-up period, if applicable]).
• To evaluate the pharmacodynamic (PD) effect of efgartigimod PH20 SC (ie, total immunoglobulin γ [IgG] levels).
• To evaluate additional patient-reported outcomes (PROs; including patient-reported quality of life and treatment satisfaction).
• To explore self-administration of the treatment.
• To explore caregiver-supported administration of the treatment.
Note: Objectives for participants in the optional substudy to explore less frequent dosing are described in a separate appendix.</td></tr>
<tr><td colspan="2">Rationale:
An unmet medical need exists for an efficacious treatment of CIDP with a favorable safety and tolerability profile and more convenient administration than that provided by current treatments. A once-weekly SC treatment option consisting of efgartigimod PH20 SC administered within a few minutes could offer clinically significant benefits to patients with CIDP.
Participants in this open-label extension (OLE) study rolled over from ARGX-113-1802 in which participants with CIDP received efgartigimod PH20 SC 1000 mg once weekly. In this OLE study, the long-term safety, tolerability, and efficacy of efgartigimod PH20 SC (administered at the same dose and frequency as in ARGX-113-1802) will be evaluated. All participants will initially receive efgartigimod PH20 SC at the same dose and frequency (1000 mg once weekly) as in ARGX-113-1802.</td></tr>
</table>

An optional substudy will explore whether participants who are well-controlled (stable clinical status) while receiving once-weekly doses of efgartigimod PH20 SC in ARGX-113-1802 and ARGX-113-1902 will remain well-controlled while receiving doses less frequently (eg, once every 2 or 3 weeks).

Investigational product, dose, and mode of administration:

The investigational medicinal product (IMP) is efgartigimod PH20 SC 1000 mg.

Methodology:

This is an open-label extension of ARGX-113-1802 to investigate the long-term safety, tolerability, and efficacy of efgartigimod PH20 SC in participants aged 18 years and older with CIDP.

Participants will be given the opportunity to continue efgartigimod PH20 SC treatment in this OLE study if any of the following 4 conditions are occurring, provided the participant has not discontinued IMP:

- The participant has a clinical deterioration defined by an adjusted Inflammatory Neuropathy Cause and Treatment disability score (aINCAT) increase ≥1 point (worsening) compared to baseline in Stage B of ARGX-113-1802, as described in the ARGX-113-1802 protocol.
- The participant completes the week-48 visit of Stage B of ARGX-113-1802 without any clinical deterioration.
- The participant is in Stage A or Stage B of ARGX-113-1802 at the time sufficient events for the primary endpoint analysis of that study have been reached and it is stopped.
- The participant completes the week-48 visit of this OLE study (the participant can then start a new treatment period of 48 weeks).

Participants in the optional substudy will decrease the dosing frequency to once every 2 weeks if they received at least 24 weeks of once-weekly doses and have stable clinical condition between the last 2 scheduled visits (ie, approximately 12 weeks). The frequency may be reduced again to once every 3 weeks if the participant shows no clinical deterioration for at least 24 weeks while receiving doses once every 2 weeks.

The OLE study is planned in treatment periods, each lasting 48 weeks.

In this OLE study, efgartigimod PH20 SC 1000 mg will be administered once weekly or once every 2 or 3 weeks during the optional substudy to explore less frequent dosing.

During the first treatment period in the OLE study, the SC injection at study day 1 (SD1) will be performed at the site via single-dose vial and, if needed, (additional) training for self-administration or caregiver-supported administration will be provided.

As of protocol v8.0, IMP will be administered as a single SC injection via single-dose prefilled syringe (PFS). Before receiving the first PFS dose, participants will be asked to sign the current informed consent form (ICF) version giving their consent to continue the study. Site staff, the participant, the caregiver, a home nurse, or a concierge service can administer IMP. The first 3 consecutive IMP injections via PFS must be administered at the study site with safety monitoring by a health care provider. If home nursing care is already in place, the third consecutive administration may occur at the participant's home monitored by a home nurse.

To be considered competent, the participant or caregiver will (self-)administer IMP under the supervision of an investigator or study nurse for at least 1 injection. After the participant or caregiver is considered competent, the (self-)administration can continue, and supervision may no longer be needed at the discretion of the investigator.

The participants will have a scheduled site visit 4 weeks after SD1, followed by another site visit 8 weeks later. After that, participants will have scheduled site visits at 12-week intervals. For any consecutive treatment period (after a preceding 48-week treatment period in the OLE study), the participant will have scheduled site visits only at 12-week intervals after SD1 of the new treatment period. In addition, participants must come to the site (or another location as agreed upon with the site/home nurse) to collect IMP at weeks 6, 18, 30, and 42 (ie, ”IMP collection visits”) of each treatment period. Except for IMP dispensation and possibly also IMP administration, no other study assessments are planned at these IMP collection visits.

Once a participant has reached week 48 of treatment period 1, the next treatment period can start. After performing all assessments of the week-48 visit of the previous year, the current ICF version will be signed for the next treatment period.

The participant can also complete the OLE study after one 48-week treatment period. When the participant completes the OLE study (after 1 or more 48-week treatment periods), he/she will have a safety follow-up visit (SFV) 28 days after the last IMP dose (of the last treatment period).

At the end of the study, argenx cannot guarantee continued access for participants but will comply with all local laws and regulations.

Safety, efficacy, immunogenicity, PK, PD, and additional PROs (including patient-reported quality of life and treatment satisfaction) will be evaluated during the study.

Inclusion and exclusion criteria:

Inclusion criteria:

Participants are eligible to be included in the study only if **all** of the following criteria apply:

1.a. Ability to understand the requirements of the study, provide written informed consent (including consent for the use and disclosure of research-related health information), willingness and ability to comply with the study protocol procedures (including required study visits) of this study.

2.b. Male or female participant with 1 of the following options:

- Has completed the week-48 visit of Stage B of ARGX-113-1802 and is considered eligible for treatment with efgartigimod PH20 SC
- Condition has deteriorated during Stage B of ARGX-113-1802 and is considered eligible for treatment with efgartigimod PH20 SC
- Has been offered the participation in the OLE study due to early termination of ARGX-113-1802 (because sufficient events for the primary endpoint analysis of the that study have been reached and it is stopped) and is considered eligible for treatment with efgartigimod PH20 SC
- Has completed the week-48 visit of the previous treatment period of the OLE study and is considered eligible to continue with efgartigimod PH20 SC treatment.

3. Women of childbearing potential who have a negative urine pregnancy test at baseline before IMP administration.

4.b. Women of childbearing potential must use an acceptable method of contraception from signing the ICF until the date of the last dose of IMP.

5. Inclusion criterion removed via protocol amendment #4.

Exclusion criteria:

Participants are excluded from the study if **any** of the following criteria apply:

1.a. Week-48/early discontinuation (ED) visit in ARGX-113-1802 or the week-48 visit of the previous OLE participation occurred more than 14 days before SD1 of the OLE study or the start of a new treatment period in the OLE study and more than 21 days since the last dose of IMP.

2.b. Pregnant and lactating women and those intending to become pregnant during the study.

3.b. Participants with clinical evidence of other significant serious disease or participants who underwent a recent or have a planned major surgery, or participants who (intend to) use prohibited medications and therapies during the study, or any other reason that could confound the results of the study or put the participant at undue risk.

Duration of participant involvement in the study:

Participants will roll over from ARGX-113-1802 on SD1 for the OLE study. The OLE study includes a treatment period of 48 weeks and an SFV 28 days after the last IMP dose (which is normally at week 47).

If the participant stops the OLE study after 1 treatment period, the total study duration is 51 weeks.

The participant can also start a new treatment period when he/she has reached week 48 of the current treatment period and continue receiving efgartigimod PH20 SC once weekly or once every 2 or 3 weeks during the optional substudy to explore less frequent dosing; in that case, the study duration will be extended another year.

Endpoints and statistical analysis

Primary endpoint:

- Safety and tolerability of the drug:
  - Incidence of treatment-emergent adverse events (TEAEs) and serious adverse events (SAEs) by System Organ Class (SOC) and Preferred Term (PT)
  - Incidence of clinically significant laboratory abnormalities

Secondary endpoints:

- Change from baseline over time in the following scores and measurements:
  - aINCAT
  - Medical Research Council (MRC) sum score
  - 24-item Inflammatory Rasch-built Overall Disability Scale (I-RODS) disability scores
  - Mean grip strength assessed by Martin-Vigorimeter
  - Timed Up and Go (TUG) score
- Percentage of participants without clinical deterioration over time, defined by an aINCAT increase ≥1 point (worsening) compared to ARGX-113-1902 baseline.
- Percentage of participants with and titers of ADA toward efgartigimod; and the presence of neutralizing antibodies (NAb) against efgartigimod.
- Efgartigimod serum concentrations (during the first 48-week treatment period [followed by a safety follow-up period, if applicable]).
- Changes from baseline over time of serum IgG levels (total).
- Change from baseline over time in:
  - EQ-5D-5L
  - Brief Pain Inventory – Short Form (BPI-SF)
  - 9-item Treatment Satisfaction Questionnaire for Medication (TSQM-9)
  - Rasch-Transformed Fatigue Severity Scale (RT-FSS)
  - Hospital Anxiety and Depression Scale (HADS)
- Percentage of participants performing self-administration over time.
- Percentage of participants with caregiver-supported treatment administration over time.

Note: Objectives for participants in the optional substudy to explore less frequent dosing are described in a separate appendix.

Statistical methods and plan:

A statistical analysis plan (SAP) detailing the statistical methods and analyses will be developed before database lock for interim analyses and the final analysis. Interim analyses (with separate interim analysis SAPs providing details) may be performed for interactions with regulatory authorities and/or when all participants have completed (or discontinued) a treatment period. A first interim analysis of the OLE study, based on data from the once-weekly dosing regimen, is planned around the time of the database lock of ARGX-113-1802 (ie, when the required number of events has been observed for the primary endpoint analysis for Stage B of ARGX-113-1802).

The safety and modified intent-to-treat (mITT) population consists of all participants who rolled over from ARGX-113-1802 and who received at least 1 dose of open-label efgartigimod PH20 SC in ARGX-113-1902.

Participants will be classified according to the treatment they received in ARGX-113-1802.

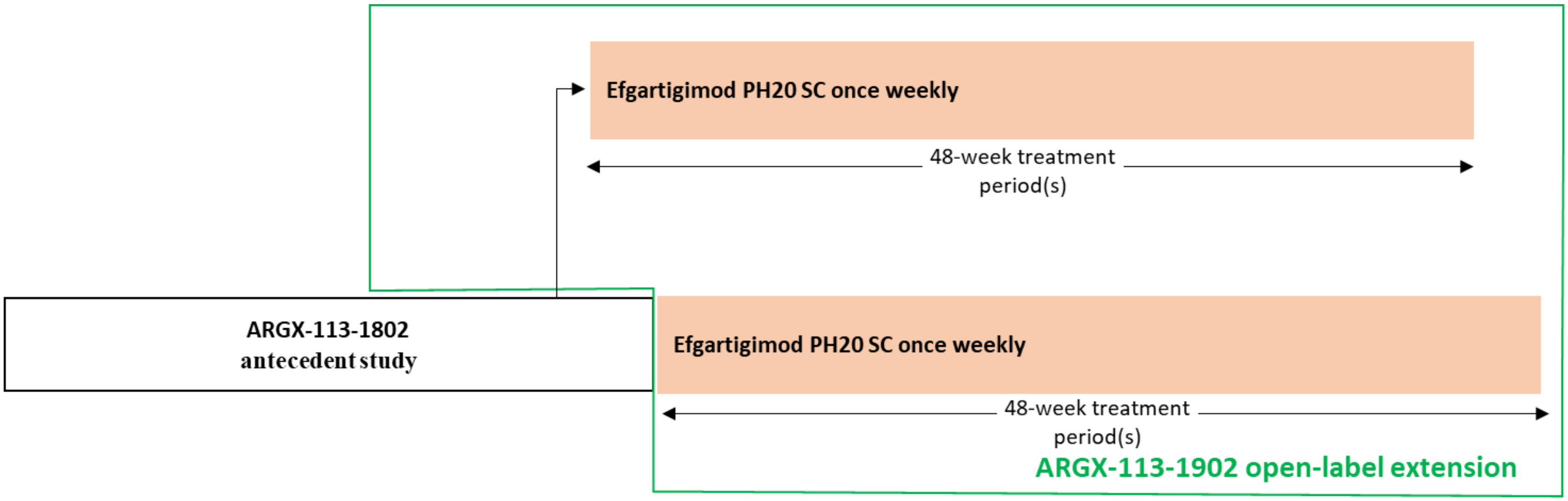
Summary statistics will be provided for the continuous endpoints in terms of absolute values and changes from baseline. Frequency tables will be made for all categorical variables.

- For the analysis of the safety data, the following will be performed:
  - Summaries of AEs and other safety parameters will be provided.
  - AEs will be classified using the latest version of the Medical Dictionary for Regulatory Activities (MedDRA) classification system. AEs reported from the first dose of IMP until 30 days after the last dose of IMP will be considered as TEAEs and will be summarized descriptively. TEAEs (including adverse events of special interest [AESIs]) will be listed corresponding to SOC and MedDRA PT. Multiple occurrences of a single PT in a participant will only be counted once at the maximum severity/grade. The severity will be assessed using the Common Terminology Criteria for AEs (CTCAE). All AEs will be summarized by relatedness to IMP. Any AEs leading to death or discontinuation of IMP will also be summarized.
  - Laboratory and vital sign parameters and their respective changes from baseline will be presented using standard summary statistics. Clinically significant laboratory abnormalities will also be summarized descriptively.
- Data from efficacy, additional patient-reported outcome, immunogenicity, PK, PD, and other endpoints will be presented using summary statistics.

Date of Protocol: 02 Jul 2024

1.2. Schema

Figure 1: Schema of ARGX-113-1902 Open-Label Extension Study



efgartigimod PH20 SC=efgartigimod for SC administration coformulated with rHuPH20; rHuPH20=recombinant human hyaluronidase PH20; SC=subcutaneous(ly)

Notes: Refer to Section 10.9.3 for the schema of the optional substudy to explore less frequent dosing.

The maximum number of participants in this study will be the number of participants who entered antecedent study ARGX-113-1802, ie, up to 360 participants.

1.3. Schedules of Activities (SoA)

Table 1: Schedule of Activities for the First 48-Week Treatment Period (NO LONGER APPLICABLE)

Assessment/Procedure	Rollover[a]	Treatment period 1[a]					Unscheduled	Safety follow-up[d]
	V1	V2	V3	V4	V5	V6		
	SD1[b]	W4	W12	W24	W36	W48/ED[a]		28 (±3) days after last IMP dose
	±0 days	±2 days				-2 to +14 days	UnschV[c]	
Informed consent	●					●[e]		
Demographics	●							
Inclusion/exclusion criteria	●					●[e]		
Medical history	●							
12-lead ECGs	●	●	●	●	●	●	●	●
Physical examination and vital signs[f]	●	●	●	●	●	●	●	●
Pregnancy test (urine)	●		●	●	●	●	●	●
Adjusted INCAT score	●	●	●	●	●	●	●	
MRC sum score	●	●	●	●	●	●	●	
I-RODS score	●	●	●	●	●	●	●	
Mean grip strength	●	●	●	●	●	●	●	
TUG test	●	●	●	●	●	●	●	
EQ-5D-5L	●		●	●	●	●	●	
BPI-SF	●		●	●	●	●	●	
TSQM-9	●		●	●	●	●	●	
RT-FSS	●		●	●	●	●	●	
HADS	●		●	●	●	●	●	
Suicidality assessment[g]	●	●	●	●	●	●	●	●

Assessment/Procedure	Rollover[a]	Treatment period 1[a]					Unscheduled	Safety follow-up[d]
	V1	V2	V3	V4	V5	V6		
	SD1[b]	W4	W12	W24	W36	W48/ED[a]		28 (±3) days after last IMP dose
	±0 days	±2 days				-2 to +14 days	UnschV[c]	
Blood and urine clinical laboratory safety tests	●	●	●	●	●	●	●	●
Blood sampling for immunogenicity analysis[h]	●	●	●	●	●	●	●	●
Blood sampling for PK and PD analysis[i]	●	●	●	●	●	●	●	●
Blood sample for potential future biomarker analysis[j]	●		●	●	●	●	●	
IMP (self-)administration training[k]	●	●	●	●	●			
IMP (self-)administration (SC)[l]	●[l]	●	●	●	●[n]	●[e]		
IMP dispensation[m]	●	●	●	●	●[n]	●[e]		
IMP accountability	●	●	●	●	●	●		
Adverse events[o]	‹------------------------------●------------------------------›							
Concomitant therapies[o,p]	‹------------------------------●------------------------------›							

ADA=antidrug antibodies; AE=adverse event; BPI-SF=Brief Pain Inventory-Short Form; ECG=electrocardiogram; ED=early discontinuation; HADS=Hospital Anxiety and Depression Scale; ICF=informed consent form; IgG=immunoglobulin γ; IMP=investigational medical product; aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment (INCAT) disability score; I-RODS=Inflammatory-Rasch-built Overall Disability Scale; OLE=open-label extension; PD=pharmacodynamic(s); PK=pharmacokinetic(s); rHuPH20=recombinant human hyaluronidase PH20; RT-FSS=Rasch-Transformed Fatigue Severity Scale; MRC=Medical Research Council; SAE=serious adverse event; SoA=Schedule of Activities; SC=subcutaneous(ly); SD1=study day 1; TSQM-9=9-itemTreatment Satisfaction Questionnaire for Medication; TUG=Timed Up and Go; Unsch=unscheduled; V=visit; W=week

Notes: In addition to the visits in this SoA, there are “IMP collection visits” as described in Section 1.3.1.

Refer to Section 10.9.4 for the SoA of the optional substudy to explore less frequent dosing. In case a participant will participate in the optional substudy, the last visit in the main part of study ARGX-113-1902 will coincide with the first visit of the substudy.

a. This visit is only for the first treatment period. Once entry criteria are confirmed, this visit will be performed on the same day of the week-48 visit or ED visit of Stage B of study ARGX-113-1802.
   For each next treatment period, the first day of a new treatment period will coincide with the week-48 visit of the previous treatment period.
b. W48/ED visit of a previous treatment period will coincide with the SD visit of the next treatment period.
c. The investigator can decide which of the assessments must be performed at each unscheduled visit (Section 4.1.7).
d. The safety follow-up visit is only applicable for participants who do not start a new treatment period and for participants who discontinue IMP less than 28 days before the last study visit.

e. This requirement is only needed for participants who will start a new 48-week treatment period. The first day of a new treatment period will coincide with the week-48 visit of the previous treatment period.
f. Assessment includes physical examination, weight, semisupine blood pressure and heart rate, and body temperature. Weight will be measured at the SD1 visit and during the study in case of an obvious weight change.
g. Suicidality will be assessed by specifically answering 1 question from the Patient Health Questionnaire-9 (PHQ-9) depression questionnaire (Section 8.13).
h. Blood samples for immunogenicity testing will be taken predose at the study visits to measure ADA directed toward efgartigimod (measured in serum). Plasma samples to determine immunogenicity against rHuPH20 plasma will be collected and stored, and will be analyzed in case of safety concerns.
i. For PK, predose (within 2 hours before start of IMP administration at the study visits) samples will be taken. At these time points, blood samples will also be taken for PD analysis. PD analysis includes the measurement of total IgG.
j. Optional blood samples will be taken for potential future biomarker analysis.
k. Training for self-administration or caregiver-supported administration will be provided on SD1 of the first treatment period and later during the study for participants or caregivers who need this until they are capable to administer IMP.
l. IMP will be administered at the site at the scheduled study visits. This will be after blood samples have been taken for laboratory safety, PK, PD, immunogenicity, and/or optional biomarker analyses. Other IMP administrations (between the scheduled visits) will be administered by the participant (self-administration), the participant's caregiver, or via a home nurse or concierge service for visit at the study site.
m. At weeks 6, 18, 30, and 42 (ie, "IMP collection visits," Section 1.3.1), participants must come to the site (or another location as agreed upon with the site/home nurse) to collect IMP. Except for IMP dispensation and possibly also IMP administration, no other study assessments are planned at these IMP collection visits.
n. The last planned IMP (self-)administration of each treatment period is at week 47.
o. AEs and intake of concomitant medication(s) will be monitored continuously from signing the ICF until the last study-related activity. In case of early discontinuation, any AEs/SAEs should be assessed for 30 days after the ED visit or until satisfactory resolution or stabilization.
p. Any vaccination received during the study (from screening onward) will be entered as concomitant medication.
Note: For participants who provide separate consent, the titers of produced vaccination antibodies will be measured using retained blood samples (leftover blood samples taken for other tests) during the study.

Table 2: Schedule of Activities for Subsequent Treatment Periods After the First 48-Week Treatment Period

Assessment/Procedure	Treatment period n[a]					Unscheduled	Safety follow-up[e]
	Baseline Cycle n[b]	Cycle n W12	Cycle n W24	Cycle n W36	Cycle n W48/ED[c]	UnschV[d]	28 (±3) days after last IMP dose
	±0 days	±2 days			-2 to +14 days		
Informed consent[f]	●						
Inclusion/exclusion criteria	●				●[f]		
Physical examination and vital signs[g]	●	●	●	●	●	●	●
Pregnancy test (urine)	●	●	●	●	●	●	●
aINCAT	●	●	●	●	●	●	
MRC sum score	●	●	●	●	●	●	
I-RODS score	●	●	●	●	●	●	
Mean grip strength	●	●	●	●	●	●	

Assessment/Procedure	Treatment period n[a]					Unscheduled	Safety follow-up[e]
	Baseline Cycle n[b]	Cycle n W12	Cycle n W24	Cycle n W36	Cycle n W48/ED[c]	UnschV[d]	28 (±3) days after last IMP dose
	±0 days	±2 days			-2 to +14 days		
TUG test	●	●	●	●	●	●	
EQ-5D-5L	●	●	●	●	●	●	
BPI-SF	●	●	●	●	●	●	
TSQM-9	●	●	●	●	●	●	
RT-FSS	●	●	●	●	●	●	
HADS	●	●	●	●	●	●	
Suicidality assessment[h]	●	●	●	●	●	●	●
Blood and urine clinical laboratory safety tests	●	●	●	●	●	●	●
Blood sampling for immunogenicity analysis[h]	●	●	●	●	●	●	●
Blood sampling for PD analysis[j]	●	●	●	●	●	●	●
IMP (self-)administration training[k]	●	●	●	●			
IMP (supervised) (self-)administration (SC)[l]	●	●	●	●[n]	●[f]		
IMP dispensation[m]	●	●	●	●[n]	●[f]		
IMP accountability	●	●	●	●	●		
Adverse events[o]	‹------------------------●------------------------›						
Concomitant therapies[o,p]	‹------------------------●------------------------›						

aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment (INCAT) disability score; AE=adverse event; BPI-SF=Brief Pain Inventory-Short Form; CIPD=chronic inflammatory demyelinating polyneuropathy; cycle=48-week treatment period; ED=early discontinuation; HADS=Hospital Anxiety and Depression Scale; ICF=informed consent form; IgG=immunoglobulin γ; IMP=investigational medical product; I-RODS=Inflammatory-Rasch-built Overall Disability Scale; MRC=Medical Research Council; n=treatment period number; PD=pharmacodynamic(s); rHuPH20=recombinant human hyaluronidase PH20; RT-FSS=Rasch-Transformed Fatigue Severity Scale; PFS=prefilled syringe; SAE=serious adverse event; SC=subcutaneous(ly); TSQM-9=9-item Treatment Satisfaction Questionnaire for Medication; TUG=Timed Up and Go; Unsch=unscheduled; V=visit; W=week

Notes: In addition to the visits in this SoA, there are "IMP collection visits" as described in Section 1.3.1.

Refer to Section 10.9.4 for the SoA of the optional substudy to explore less frequent dosing. In case a participant will participate in the optional substudy, the last visit in the main part of study ARGX-113-1902 will coincide with the first visit of the substudy.

a. Participants will return to the site for site visits at 12-week intervals; and in between these site visit, at 6-week intervals to collect IMP (ie, at weeks 6, 18, 30, and 42); refer to footnote m).

b. The W48 visit of a previous treatment period will coincide with the baseline visit of the next treatment period.

c. Participants can remain in the study until 2 years after marketing authorization in their country, until efgartigimod PH20 SC becomes commercially available or available via the continued access program for CIDP, or until 30 Apr 2027, whichever comes first.

d. The investigator can decide which of the assessments must be performed at each unscheduled visit (Section 4.1.7).

e. The safety follow-up visit is only applicable for participants who do not start a new treatment period and for participants who discontinue IMP less than 28 days before the last study visit.

f. Participants will be asked to sign the current ICF version before receiving the first PFS dose and before starting a new 48-week treatment period to continue the study The first day of a new treatment period will coincide with the week-48 visit of the previous treatment period.

g. Assessment includes physical examination, weight, semisupine blood pressure and heart rate, and body temperature. Weight will be measured in case of an obvious weight change.

h. Suicidality will be assessed by specifically answering 1 question from the Patient Health Questionnaire-9 (PHQ-9) depression questionnaire (Section 8.13).

i. Blood will be collected, stored, and analyzed to evaluate immunogenicity against rHuPH20 (measured in plasma) if there are safety concerns.

j. For PD, predose (before IMP administration at the study visits) samples will be taken. PD analysis includes the measurement of total IgG.

k. Training for self-administration or caregiver-supported administration will be provided as needed until they are capable to administer IMP.

l. IMP will be administered via PFS; further information is provided in Section 6.2.2. IMP administration will occur after blood samples have been taken for laboratory safety, PD, and/or immunogenicity analyses. After the participant or caregiver is considered competent, the (self-)administration can continue, and supervision may no longer be needed at the discretion of the investigator.

m. At weeks 6, 18, 30, and 42 (ie, “IMP collection visits,” Section 1.3.1), participants must come to the site (or another location as agreed upon with the site/home nurse) to collect IMP. The site staff will provide “PFS Instructions for Use” to the participant and caregiver at the first supervised PFS IMP administration visit. Except for IMP dispensation and possibly IMP administration, no other study assessments are planned at these IMP collection visits.

n. The last planned IMP (self-)administration of each treatment period is at week 47.

o. AEs and intake of concomitant medication(s) will be monitored continuously from signing the ICF until the last study-related activity. In case of early discontinuation, any AEs/SAEs should be assessed for 30 days after the ED visit or until satisfactory resolution or stabilization.

p. Any vaccination received during the study (from screening onward) will be entered as concomitant medication.

Note: For participants who provide separate consent, the titers of produced vaccination antibodies will be measured using retained blood samples (leftover blood samples taken for other tests) during the study.

1.3.1. IMP Collection Visits

IMP collection visits are planned at weeks 6, 18, 30, and 42.

The participant will collect their IMP at the site or another location agreed upon with the site or home nurse.

The participant will be asked to return all used and unused IMP at the next scheduled visit, during which IMP accountability will be ensured.

Except for IMP dispensation and possibly IMP administration, no other study assessments are planned at these IMP collection visits. In addition, data that are collected continuously during the study (eg, AEs, concomitant medication, etc) will be collected during IMP collection visits.

As of protocol v8.0, the source of IMP will change from a single-dose vial to a single-dose PFS. The site staff will provide a "PFS Instructions for Use" document to the participant or caregiver at the first supervised PFS IMP administration visit (Section 6.2.2).

2. INTRODUCTION

2.1. Study Rationale

The term chronic inflammatory demyelinating polyneuropathy (CIDP) has been used to identify a chronically progressive or relapsing symmetric sensorimotor disorder with cytoalbuminologic dissociation and interstitial and perivascular endoneurial infiltration by lymphocytes and macrophages. A number of variants of CIDP have been described that have immune or inflammatory aspects and electrophysiologic and/or pathologic evidence of demyelination in common. No consensus exists on the best approach to the nomenclature of these disorders. CIDP variants include disorders with predominantly sensory symptoms, distal acquired demyelinating symmetric neuropathy (DADS), multifocal acquired demyelinating sensory and motor neuropathy (MADSAM) also known as Lewis-Sumner syndrome, and CIDP with associated central nervous system (CNS) demyelination or with other systemic disorders.[1,16]

CIDP most commonly has an insidious onset with a chronic progressive or relapsing course. Occasionally, complete remission occurs. The estimated prevalence of CIDP is 1.8 to 8.9 per 100 000.[15] CIDP may occur at any age, but it is more common in the fifth and sixth decades. A relapsing course is associated with a younger age of patients (third and fourth decades).

The most common treatments are currently immunosuppressive or immunomodulatory interventions. These agents include intravenous immunoglobulin (IVIg), subcutaneous immunoglobulin (SCIg), plasmapheresis, prednisone, azathioprine, methotrexate, mycophenolate, cyclosporine, and cyclophosphamide. Their use is based on the proposed pathogenesis of CIDP as an immune-mediated condition, although for some of these agents, there is no clear evidence of a beneficial treatment effect.

However, an unmet medical need exists for an efficacious treatment of CIDP with a more specific mechanism of action maintaining quality of life (QoL) at the highest levels for the longest period possible, a favorable safety and tolerability profile, and more convenient administration that has a significantly lower impact on daily routines than that provided by current treatments.

While the abiding theory of CIDP pathogenesis is that cell-mediated and humoral mechanisms act together in an aberrant immune response to cause damage to peripheral nerves, the relative contributions of T cell and autoantibody responses remain largely undefined, although the role of humoral immune responses in these disorders has been emphasized.[17] However, several lines of evidence support the conclusion that CIDP is an autoimmune disease mediated by humoral and/or cellular immunity against yet undefined Schwann cell/myelin antigens.[14]

Both sera and immunoglobulin γ (IgG) molecules from patients with CIDP induce peripheral demyelination in susceptible animals and inhibit nerve conduction in several models of peripheral neuropathies.[18]

The autoimmune etiology is supported by the efficacy of treatments that target the immune system, including IVIg, plasma exchange and corticosteroids, and by evidence of an inflammatory response in the blood and peripheral nerves. Depletion of serum IgGs from patients with CIDP showed promising efficacy in small-scale studies.[28]

Critically, evidence suggests that nodal antigens are important in some CIDP cases. Devaux et al (2012)[9] found that 30% of patients with CIDP have serum IgG that binds to either the nodes of Ranvier or the paranodes in teased nerve fibers and in some cases identified the target antigens as neurofascin, gliomedin, or contactin. Further, several studies have specifically identified autoantibodies against cell adhesion molecules at the nodes of Ranvier and paranodal regions in patients with CIDP.[17]

While further work is needed to examine the pathophysiological significance of nodal antigenic targets in CIDP, any disruption of nodal function is likely to interfere with normal nerve excitability and membrane potentials, contributing to conduction failure by interfering with saltatory conduction and ion channel function. In support of this, axonal excitability studies in patients with CIDP have showed a range of findings demonstrating aberrant membrane excitability and membrane potential.[17]

The Fc gamma receptor (FcγR) regulatory system has been shown to be disturbed in patients with CIDP.[18] Balancing activating versus inhibitory FcγR expression might provide a clinical benefit for patients with CIDP.

A subcutaneous (SC) treatment option consisting of efgartigimod with the permeation enhancer recombinant human hyaluronidase PH20 (rHuPH20) could offer clinically significant benefits to patients with CIDP.

In this open-label extension (OLE) study, the long-term safety and efficacy of efgartigimod PH20 SC (administered at the same dose and frequency as in the antecedent study ARGX-113-1802) will be evaluated.

2.2. Background

Efgartigimod (ARGX-113) is a human IgG1-derived Fc of the za allotype that binds with nanomolar affinity to human neonatal crystallizable fragment receptor (FcRn). Efgartigimod encompasses IgG1 residues D221-K447 (European Union [EU] numbering scheme) and has been modified with the so-called ABDEG technology (ABDEG=antibody that enhances IgG degradation)[22] to increase its affinity for FcRn at both physiological and acidic pH. The increased affinity for FcRn of efgartigimod at both acidic and physiological pH results in a blockage of FcRn-mediated recycling of IgGs.

Given the essential role of the FcRn receptor in IgG homeostasis, inhibiting this FcRn function, as is achieved by efgartigimod, leads to rapid degradation of all IgGs, including disease-associated autoantibodies of the IgG isotype. This approach is thought to result in alleviation of signs and symptoms in IgG-driven autoimmune diseases.

The safety, tolerability, pharmacokinetics (PK), and pharmacodynamics (PD) of intravenous (IV) administrations of efgartigimod have been investigated in the first-in-human study ARGX-113-1501 in healthy adult participants. A second study (ARGX-113-1702) in healthy adult participants investigated the bioavailability, safety, tolerability, immunogenicity, PK, and PD after SC administration of efgartigimod and evaluated the reduction of the IV infusion time from 2 hours to 1 hour.

Phase 2 studies in immune thrombocytopenia (ITP; ARGX-113-1603) and myasthenia gravis (MG; ARGX-113-1602) have indicated that efgartigimod administered by IV infusion is well tolerated, induces a specific, rapid PD effect (ie, reduction in IgG levels, including autoantibody levels), and is associated with improvement in clinical signs and symptoms in participants with ITP and MG, separately.[11] Additionally, the safety and tolerability of efgartigimod has been evaluated for the treatment of participants with pemphigus in the phase 2 study ARGX-113-1701 and for the treatment of participants with MG in the phase 3 study ARGX-113-1704.

For the phase 2 studies in participants with CIDP, a fixed dose of efgartigimod PH20 SC 1000 mg was selected based on the results of the phase 1 study ARGX-113-1901 in healthy participants who achieved a similar PD effect as that observed in generalized myasthenia gravis (gMG) and ITP, ie, IgG reduction, at a steady state as achieved by once-weekly 10 mg/kg IV infusions. Doses of 10 mg/kg efgartigimod IV have demonstrated a favorable safety and efficacy profile across phase 2 studies in participants with MG and ITP. To select the fixed SC dose, an open-label, parallel-group study in healthy male participants (ARGX-113-1901) has been performed to investigate the PK, PD, safety, and tolerability of different single fixed SC dose levels of efgartigimod coadministered with rHuPH20.

In the current OLE study, and in the main study, efgartigimod is coformulated with the permeation enhancer rHuPH20 (efgartigimod PH20 SC). rHuPH20 acts as a spreading factor that increases the dispersion and absorption of other coadministered drugs and allows SC dosing of greater volumes than without rHuPH20. SC injections of rHuPH20 with fluids, small molecules, peptides, and proteins (eg, IgG) were well tolerated in all clinical study populations studied to date.[19]

2.3. Benefit/Risk Assessment

2.3.1. Benefit

Efgartigimod has been shown to effectively reduce IgG antibody levels in clinical studies in healthy participants and in participants with gMG and ITP.

In both phase 2 clinical studies, treatment with efgartigimod IV induced a specific, rapid, and profound PD effect and was associated with improvement in clinical signs and symptoms in participants with primary ITP and gMG, separately. In addition, phase 3 studies with efgartigimod IV in participants with gMG showed that the study's primary endpoint was met with statistical significance. The primary endpoint was defined as the "percentage of anti-acetylcholine receptor antibody (AChR-Ab) seropositive participants who, during the first cycle, have a reduction of ≥2 points in the Myasthenia Gravis Activities of Daily Living (MG-ADL) total score compared to study entry baseline for ≥4 consecutive weeks, with the first reduction occurring no later than 1 week after the last investigational medicinal product (IMP) infusion." A total of 67.7% of AChR-Ab seropositive participants treated with efgartigimod achieved the primary endpoint compared with 29.7% on placebo ($p<0.0001$). In the overall participant population (AChR-Ab seropositive and seronegative participants), 67.9% of the participants treated with efgartigimod were MG-ADL responders compared with 37.3% on placebo ($p<0.0001$). Other key secondary endpoints for this study were also met with high statistical significance.[11]

Efgartigimod is under development for both the IV and SC administration route. For the phase 2 CIDP studies, SC administration is planned with a newly developed coformulation of efgartigimod with rHuPH20 (efgartigimod PH20 SC). The coformulated material will allow a manual SC injection within a few minutes of greater volumes than without rHuPH20.

Due to the unmet medical need in CIDP for an efficacious treatment with a favorable safety and tolerability profile and more convenient administration for participants with CIDP, an SC treatment option for efgartigimod could offer clinically significant benefits to participants with CIDP.

The sponsor has developed a second-generation presentation of efgartigimod PH20 SC containing 200 mg/mL efgartigimod (1000 mg/5 mL) and 2000 U/mL rHuPH20 in a ready-to-use PFS. Efgartigimod PH20 SC enables self-administration or caregiver-supported administration at home. The PFS presentation simplifies the injection procedure and increases participant convenience.

Several lines of evidence support the conclusion that CIDP is an autoimmune disease mediated by humoral and/or cellular immunity against yet undefined Schwann cell/myelin antigens (Section 2.1).[14]

The autoimmune etiology is supported further by the efficacy of treatments that target the immune system, including IVIgs, plasma exchange, and corticosteroids, and by evidence of an inflammatory response in the blood and peripheral nerves. Depletion of serum IgGs from participants with CIDP showed promising efficacy in small-scale studies.[28]

2.3.2. Risk Assessment

Available data confirm that efgartigimod PH20 SC has been well tolerated across studies in different indications and has an acceptable safety profile.

Refer to the current efgartigimod IB for more information.

Table 3: Potential Risks and Mitigation Strategies

Potential clinically significant risk	Summary of data/rationale for risk	Mitigation strategy
Serious infection	Efgartigimod reduces IgG levels, potentially hindering immune response and increasing the risk for infection.	Exclude participants with clinically significant active infection not sufficiently resolved in the investigator's opinion (Section 5.2). Infections are considered AESIs (Section 8.4.6). Monitor for infections and temporarily interrupt IMP dosing as specified in Section 7.1.
Injection-/infusion-related reactions	All therapeutic proteins can elicit immune responses, potentially resulting in hypersensitivity or allergic reactions such as rash, urticaria, angioedema, serum sickness, and anaphylactoid or anaphylactic reactions.	Participants for whom IMP is administered at the site will be monitored for injection-/infusion-related reactions. Participants who are self-administering at home will be educated on symptoms of injection-/infusion-related reactions and instructed to self-monitor for them after administering the injections. Injection-/infusion-related reactions

Potential clinically significant risk	**Summary of data/rationale for risk**	**Mitigation strategy**
		are considered AEs of clinical interest (Section 8.4.7).
Injection-site reactions	Most AEs have been mild, transient injection-site reactions, including erythema, pain, bruising, pruritus, burning, tenderness, edema, induration, irritation, paresthesia, numbness, and rash. Moderate injection-site reactions occurring less frequently include burning, erythema, pain, and numbness. Localized injection-site reactions have been observed in studies in which efgartigimod is comixed with PH20 and administered SC.	Continuously monitor participants for injection-site reactions. Injection-site reactions are AEs of clinical interest (Section 8.4.7).

AE=adverse event; AESI=adverse event of special interest; IgG=immunoglobulin γ; IMP=investigational medicinal product; SC=subcutaneous(ly)

3. OBJECTIVES

3.1. Primary Objective

- To assess the long-term safety and tolerability of efgartigimod PH20 SC.

3.2. Secondary Objectives

- To determine the long-term treatment effect.
- To evaluate the immunogenicity (ADA against efgartigimod during the first 48-week treatment period [followed by a safety follow-up period, if applicable]).
- To evaluate the PK of efgartigimod PH20 SC (during the first 48-week treatment period [followed by a safety follow-up period, if applicable]).
- To evaluate the PD effect of efgartigimod PH20 SC (ie, total IgG levels).
- To evaluate additional patient-reported outcomes (PROs; including patient-reported QoL and treatment satisfaction).
- To explore self-administration of the treatment.
- To explore caregiver-supported administration of the treatment.

Note: Objectives for participants in the optional substudy to explore less frequent dosing are described in Section 10.9.

4. STUDY DESIGN

4.1. Overall Design

This is an open-label extension of ARGX-113-1802 to investigate the long-term safety, tolerability, and efficacy of efgartigimod PH20 SC in participants aged 18 years and older with CIDP.

Participants will be given the opportunity to continue efgartigimod PH20 SC treatment in this OLE study if any of the following 4 conditions are occurring, provided the participant has not discontinued IMP:

- The participant has a clinical deterioration defined by an aINCAT increase ≥1 point (worsening) compared to baseline in Stage B of ARGX-113-1802, as described in the ARGX-113-1802 protocol.
- The participant completes the week-48 visit of Stage B of ARGX-113-1802 without any clinical deterioration.
- The participant is in Stage A or Stage B of ARGX-113-1802 at the time sufficient events for the primary endpoint analysis of that study have been reached and it is stopped.
- The participant completes the week-48 visit of this OLE study (the participant can then start a new treatment period of 48 weeks).

Participants in the optional substudy (Section 10.9) will decrease the dosing frequency to once every 2 weeks if they received at least 24 weeks of once-weekly doses and have stable clinical condition between the last 2 scheduled visits (ie, approximately 12 weeks). The frequency may be reduced again to once every 3 weeks if the participant shows no clinical deterioration for at least 24 weeks while receiving doses once every 2 weeks.

The OLE study is planned in treatment periods, each lasting 48 weeks.

In this OLE study, efgartigimod PH20 SC 1000 mg will be administered once weekly, ie, at the same dose and frequency as in ARGX-113-1802, or once every 2 or 3 weeks during the optional substudy to explore less frequent dosing (Section 10.9).

As of protocol v8.0, the source of IMP (efgartigimod PH20 SC) will change from a single-dose vial to a single-dose PFS. Before receiving the first PFS dose, participants will be asked to sign the current ICF version giving their consent to continue the study. Instructions on IMP administration via PFS are provided in Section 6.2.2.

The participants will have a scheduled site visit 4 weeks after study day 1 (SD1), and then another visit 8 weeks later. Thereafter, participants will have scheduled site visits at 12-week intervals. For any consecutive treatment period (after a preceding 48-week treatment period in the OLE study), the participant will have scheduled site visits only at 12-week intervals after the start of the new treatment period. The first day of a new treatment period will coincide with the week-48 visit of the previous treatment period. In addition, participants must come to the site (or another location as agreed upon with the site/home nurse) to collect IMP at weeks 6, 18, 30, and 42 (ie, "IMP collection visits," Section 1.3.1) of each treatment period.

A window of ±2 days is permitted for the study visits (and up to 14 days for the first visit of a new treatment period, and ±3 days for the safety follow-up visit [SFV]) as described in the SoA (Section 1.3).

Once a participant has reached week 48 of treatment period 1, the next treatment period can start. After performing all assessments of the week-48 visit of the previous year, the current ICF version will be signed for the next treatment period.

The participant can also complete the OLE study after one 48-week treatment period. When the participant will complete the OLE study (after 1 or more 48-week treatment periods), he/she will have an SFV 28 days (±3 days) after the last IMP dose (of the last treatment period).

Participants whose condition clinically deteriorates will continue with once-weekly efgartigimod PH20 SC or withdraw from the study. Participants will be treated as considered appropriate by the participant's treating physician, after they are withdrawn from the study.

For participants who withdraw from the study before week-48 visit, a full set of assessments will be performed at the early discontinuation (ED) visit (at which the same assessment will be performed as at the week-48 visit; refer to the SoA in Section 1.3). In case of an ED visit, the participants will also be asked to return 28 days (±3 days) after the last IMP dose for an SFV.

The different study visits are explained in more detail in the following sections.

Safety assessments will include monitoring for AEs, vital sign measurements, electrocardiogram (ECG) recordings (ECG only during the first 48-week treatment period, and safety follow-up if applicable), physical examinations, and laboratory testing (blood chemistry, hematology, and urinalysis).

Efficacy assessments will include the aINCAT, the Medical Research Council (MRC) score, the Inflammatory Rasch-built Overall Disability Scale (I-RODS) score, the mean grip strength test, and the Timed Up and Go (TUG) test. Additional PROs (including patient-reported QoL and treatment satisfaction) will also be assessed.

Furthermore, the immunogenicity of efgartigimod, and the PK and PD effects of efgartigimod PH20 SC will be assessed (immunogenicity against efgartigimod and PK only during the first 48-week treatment period [followed by a safety follow-up period, if applicable]). In addition, serum samples collected during the first 48-week treatment period will be stored for potential future analysis of biomarkers from all participants who provided separate consent for biomarker analysis. Finally, suicidality will be assessed throughout the study.

AEs and intake of concomitant medication(s) will be monitored continuously from signing the ICF until the last study-related activity. In case of early discontinuation, any AEs and SAEs should be assessed for 30 days after the ED visit or until satisfactory resolution or stabilization.

If the participant withdraws from the OLE study after 1 treatment period, the total study duration is 51 weeks consisting of a treatment period of 48 weeks and an SFV 28 days after the last IMP dose (last dose will normally be at week 47). The participant can also start a new treatment period when he/she has reached week 48 of the current treatment period; in that case the study duration will be extended 48 weeks.

Study assessments and procedures are described in Section 8.

4.1.1. SD1 Visit

Once the assessments of the week-48 or the ED visit of ARGX-113-1802 have been completed, the participant can participate in the OLE study. A separate ICF will be signed to continue the treatment within the OLE study and to start a new treatment period in the OLE study. If the participant is eligible for the OLE study, assessments of the week-48 visit or the ED visit (eg, in the event of a clinical deterioration or due to early termination of ARGX-113-1802 because sufficient events for the primary endpoint analysis of that study have been reached and it is stopped) of ARGX-113-1802 will be used as a baseline of the OLE study. These assessments do not need to be repeated.

Once the participant has been included in the OLE study and all assessments have been completed (as described in the SoA in Section 1.3), the participant will receive refresher training on self-administration and perform the first administration of a single SC injection via single-dose vial under the supervision of a health care professional. The participant's caregiver could also be trained to administer IMP, and the first administration by the caregiver will be under supervision of a professional. Training for self-administration or caregiver-supported administration will be provided on SD1 and later during the study for participants or caregivers who need this until they are capable of administering IMP.

As of protocol v8.0, IMP will be administered as a single SC injection via single-dose PFS. Instructions on IMP administration are provided in Section 6.2.2.

After IMP has been administered, participants will be discharged from the study site if no safety concerns exist in the opinion of the investigator.

4.1.2. Visit at Week 4

Four weeks after SD1 of the first 48-week treatment period only, the participant will return to the site.

At this visit, several safety, efficacy, immunogenicity, PK, and PD assessments will be performed as described in the SoA in Section 1.3. After completing all assessments, IMP will be administered at the site.

The participant will be asked to return all used and unused medication at the next visit, during which IMP accountability will be ensured.

Participants will be discharged from the study site if no safety concerns exist in the opinion of the investigator.

At the second and consecutive treatment periods this visit will be skipped for participants in the main study, and the schedule of visits at weeks 12, 24, 36, and 48 will be followed as described in Section 4.1.4.

4.1.3. IMP Collection Visits at Weeks 6, 18, 30, and 42

At each of the IMP collection visits at weeks 6, 18, 30, and 42 (Section 1.3.1), the participants will collect their IMP at the site or another location agreed upon with the site or home nurse. The participant will be asked to return all used and unused medication at the next scheduled visit, during which IMP accountability will be ensured.

Except for IMP dispensation and possibly IMP administration, no other study assessments are planned at these IMP collection visits. In addition, data that are collected continuously during the study (AEs, concomitant medication, etc) will be collected during IMP collection visits.

As of protocol v8.0, the source of IMP will change from a single-dose vial to a PFS. The site staff will provide "PFS Instructions for Use" to the participant and caregiver at the first supervised PFS IMP administration visit (Section 6.2.2).

4.1.4. Visits at Weeks 12, 24, 36, and 48

At each of the visits at weeks 12, 24, 36, and 48, several safety, efficacy, immunogenicity, PK, PD, and patient-reported QoL and treatment satisfaction assessments will be performed as described in the SoA in Section 1.3.

Once all assessments have been completed, IMP will be administered at the site.

IMP will be dispensed to the participant for treatment in a home setting (eg, by the participant [self-administration], the participant's caregiver, or via a home nurse or concierge service for visit at the study site). The participant will be asked to return all used and unused medication at the next visit, during which IMP accountability will be ensured.

The visit at week 48 may be on the same day as SD1 visit for next consecutive treatment period for those participants who are eligible and willing to continue the study treatment in a following treatment period. The period between the week-48 visit and the SDI visit of a new treatment period should be within 14 days.

4.1.5. Early Discontinuation Visit

For participants who stop their participation before the OLE week-48 visit, a full set of assessments will be performed at the ED visit (as described in the SoA in Section 1.3). In case of an ED visit, the participants will also be asked to return 28 days (±3 days) after the last IMP dose for an SFV.

4.1.6. Safety Follow-Up Visit (SFV)

The SFV is only for participants who do not start a new treatment period at week 48 and for participants who prematurely discontinue the study in case IMP was stopped less than 28 days before the last study visit.

The SFV will occur 28 days (±3 days) after the last IMP injection.

During the SFV, several safety and other assessments will be performed as described in the SoA in Section 1.3.

4.1.7. Unscheduled Visit

An unscheduled visit will be initiated at the investigator's discretion or by participant request if necessary for the participant's safety and well-being.

An unscheduled visit is planned for safety reasons if no study visit is planned at that time.

All such visits will be documented on the electronic case report form (eCRF) with any additional required documentation based on the nature of unscheduled visit.

Procedures that may be performed at an unscheduled visit are defined in the SoA in Section 1.3. The investigator can decide which of the assessments must be performed at each unscheduled visit.

The date and reason for the unscheduled visit must be recorded in the source documentation.

If an unscheduled visit is necessary, then additional diagnostic tests may be performed based on investigator assessment as appropriate, and the results of these tests should be entered on the unscheduled visit page of the eCRF.

4.2. Scientific Rationale for Study Design

In the current study, the long-term safety, tolerability, and efficacy of efgartigimod PH20 SC will be evaluated in participants with CIDP.

This study is designed as an open-label study to evaluate the effect of efgartigimod PH20 SC.

The study consists of 48-week treatment periods in which all participants will initially receive once-weekly injections of efgartigimod PH20 SC 1000 mg at the same dose, mode, and frequency of administration as in ARGX-113-1802.

An optional substudy will investigate less frequent dosing.

At the week-48 visit of each treatment period, the participant can have an SFV 28 days after the last IMP dose and stop the study or start a new treatment period of 48 weeks.

To assess the long-term safety, tolerability, and efficacy of efgartigimod PH20 SC, the same efficacy, safety, and additional PRO (including patient-reported QoL and treatment satisfaction) assessments are used in this OLE study as in ARGX-113-1802. These assessments are standard, ie, widely used (including in CIDP studies) and recognized as reliable, accurate, and relevant.[13,26]

4.3. Justification for Dose

The dose of efgartigimod PH20 SC 1000 mg was selected for ARGX-113-1802 and the current ARGX-113-1902. Based on population PK/PD modeling, this dose of efgartigimod PH20 SC (once-weekly administration) is predicted to be comparable to efgartigimod IV 10 mg/kg administered once weekly with respect to effect on IgG levels. The efgartigimod 10 mg/kg IV dose:

1. Resulted in transient clinical efficacy in a phase 2 study in participants with ITP and a prolonged clinical effect in a phase 2 study in participants with MG after 4 once-weekly infusions.
2. Was shown to result in close-to-saturated PD effect: Higher doses or administration more frequent than once weekly is not expected to result in an improved PD effect (ie, further lowering of autoantibodies) and/or clinical effect and may be associated with a less optimal benefit-risk ratio. Lower doses are expected to result in a lower PD effect, likely resulting in a less consistent and/or incomplete clinical response, which is undesirable given the serious and chronic manifestations of CIDP. Therefore, a once-weekly dosing regimen is most favorable until participants are in a stable clinical condition, as defined in Section 10.9.1.

3. Demonstrated a favorable safety profile in phase 2 studies in participants with MG and ITP.

In addition, based on favorable phase 3 results in participants with MG, efgartigimod IV 10 mg/kg has been approved in the United States (US) and in the EU (in both regions for AChR-Ab seropositive participants) and in Japan (for participants who do not have sufficient response to steroids or nonsteroidal immunosuppressive therapies).

A study in healthy participants demonstrated that after 4 once-weekly injections of efgartigimod PH20 SC 1000 mg, the pattern of total IgG reduction over time was comparable to the 4 once-weekly infusions of efgartigimod IV 10 mg/kg.

rHuPH20, as Hylenex, has been approved in the US. Coformulations of rHuPH20 with other active ingredients have been approved in the US and the EU (eg, Herceptin Hylecta/Herceptin SC, Rituxan Hycela/MabThera SC, HyQvia), with an rHuPH20 concentration of 2000 U/mL for an SC injection volume in the range of 5 to 13.4 mL.

Refer to Section 10.9 for the rationale of the optional substudy to explore less frequent dosing.

4.4. Measures to Minimize Bias: Randomization and Blinding

Randomization and blinding are not applicable, because this is an OLE study in which all participants will receive the same IMP (consisting of efgartigimod PH20 SC).

4.5. End of the Study Definition

The end of the study is defined as the last participant last visit in ARGX-113-1902.

Refer to Section 10.1.9 for study and site start and closure.

4.6. Protocol Deviations

The investigator should not implement any deviation from or changes to the protocol without agreement by the sponsor and prior review and documented approval of an amendment from the Institutional Review Board/Independent Ethics Committee (IRB/IEC) and regulatory authority per local regulation, except where necessary to eliminate an immediate hazard to study participants, or when the change involves only logistical or administrative aspects of the study (eg, change of telephone numbers). The investigator (or designee) should document and explain any deviation from the approved protocol. Upon awareness, the investigator (or designee) must report a likely significant protocol deviation to the sponsor (or designee).

Protocol exemptions or waivers will not be approved by the sponsor.

5. STUDY POPULATION

Prospective approval of protocol deviations to recruitment and enrollment criteria, also known as protocol waivers or exemptions, is not permitted.

5.1. Inclusion Criteria

Participants are eligible to be included in the study only if **all** of the following criteria apply:

1.a. Ability to understand the requirements of the study, provide written informed consent (including consent for the use and disclosure of research-related health information), willingness and ability to comply with the study protocol procedures (including required study visits) of this study.

2.b. Male or female participant with 1 of the following options:

- Has completed the week-48 visit of Stage B of ARGX-113-1802 and is considered eligible for treatment with efgartigimod PH20 SC
- Condition has deteriorated during Stage B of ARGX-113-1802 and is considered eligible for treatment with efgartigimod PH20 SC
- Has been offered the participation in the OLE study due to early termination of ARGX-113-1802 (because sufficient events for the primary endpoint analysis of the that study have been reached and it is stopped) and is considered eligible for treatment with efgartigimod PH20 SC
- Has completed the week-48 visit of the previous treatment period of the OLE study and is considered eligible to continue with efgartigimod PH20 SC treatment.

3. Women of childbearing potential who have a negative urine pregnancy test at baseline before IMP administration.

4.b. Women of childbearing potential must use an acceptable method of contraception from signing the ICF until the date of the last dose of IMP (Section 10.3).

5. Inclusion criterion removed via protocol amendment #4.

5.2. Exclusion Criteria

Participants are excluded from the study if **any** of the following criteria apply:

1.a. Week-48/ED visit in ARGX-113-1802 or the week-48 visit of the previous OLE participation occurred more than 14 days before SD1 of the OLE study or the start of a new treatment period in the OLE study and more than 21 days since the last dose of IMP.

2.b. Pregnant and lactating women and those intending to become pregnant during the study.

3.b. Participants with clinical evidence of other significant serious disease or participants who underwent a recent or have a planned major surgery, or participants who (intend to) use prohibited medications and therapies (Section 6.4.2) during the study, or any other reason that could confound the results of the study or put the participant at undue risk.

5.3. Lifestyle Considerations

There are no specific lifestyle considerations for this study.

5.4. Screen Failures

Eligibility to roll over must be verified at the last visit of ARGX-113-1802.

Participants who consent to participate in the clinical study but fail to meet the inclusion and exclusion criteria before IMP administration will be considered screen failures and will be captured as "enrollment failures."

Individuals who do not meet the criteria for participation in this study (screen failures), based on the results from assessments in the preceding study, may not be rescreened.

6. TREATMENTS AND THERAPIES DURING THE CONDUCT OF THE STUDY

IMP is defined as any investigational intervention(s), marketed product(s), placebo, or medical device(s) intended to be administered to a study participant according to the study protocol.

In this study, IMP is efgartigimod PH20 SC.

6.1. IMP(s) Administered

Efgartigimod is a modified human IgG1 Fc fragment with increased affinity to human FcRn. rHuPH20 is an enzyme used to increase the dispersion and absorption of coadministered therapeutics when administered SC. Efgartigimod and rHuPH20 are coformulated and will be administered once weekly or once every 2 or 3 weeks during the optional substudy to explore less frequent dosing (Section 10.9) as a single SC injection.

As of protocol v8.0, the source of IMP will change from a single-dose vial (Section 6.1.1) to a single-dose PFS (Section 6.1.2).

IMP will be provided by the sponsor and is manufactured in accordance with Good Manufacturing Practice regulations.

6.1.1. Vial

The efgartigimod PH20 SC formulation will be provided in a vial at a concentration of 165 mg/mL or 180 mg/mL (new concentration) for efgartigimod and 2000 U/mL for rHuPH20 (also referred to as ARGX-113/rHuPH20). Each dose of efgartigimod PH20 SC will contain efgartigimod 1000 mg. Note that both formulations of efgartigimod PH20 SC (with efgartigimod at a concentration of 165 mg/mL or 180 mg/mL) will be used during a transition period. After this period, all participants will receive the efgartigimod PH20 SC formulation with efgartigimod at the higher concentration of 180 mg/mL. The formulation with the higher concentration of efgartigimod (180 mg/mL) reduces the dosing volume for each SC injection.

Refer to the pharmacy manual for detailed instructions on IMP administration.

The vials will be labeled as required per country requirements.

6.1.2. Prefilled Syringe

The efgartigimod PH20 SC formulation will be provided in a glass PFS at a concentration of 200 mg/mL for efgartigimod and 2000 U/mL for rHuPH20 (also referred to as ARGX-113/rHuPH20). Each dose of efgartigimod PH20 SC will contain 1000 mg of efgartigimod.

Refer to the pharmacy manual and Section 6.2.2 for detailed instructions on IMP administration via PFS.

Each PFS will be labeled as required per country requirements.

6.2. Preparation, Handling, Storage, Administration, and Accountability

Refer to the pharmacy manual for information about the temperature for IMP transit and storage and the IMP preparation, including the volume of efgartigimod PH20 SC to be administered.

As of protocol v8.0, the source of IMP will change from a single-dose vial (Section 6.2.1) to a single-dose PFS (Section 6.2.2).

6.2.1. Vial

For each injection, the initiation and completion times (hour and minute), and details of any interruptions or premature discontinuation of injections will be recorded and transferred to the eCRF. The start will be marked as the moment the plunger is pressed, and the end will be marked as the completion time of administration of the total volume of IMP.

Only participants enrolled in the study will receive IMP and only authorized site staff will supply IMP, administer IMP, or train the participant, the participant's caregiver, or the home nurse on IMP (self-)administration. Each participant will receive specific training for self-administration during ARGX-113-1802 (and at SD1 of the first treatment period and later during the study, if needed).

In case of IMP administration by the caregiver, the caregiver will be trained from the first visit of ARGX-113-1902 onward, until the caregiver and the investigator are confident in the ability of the caregiver to administer IMP. Each participant and each caregiver who will (self-)administer IMP will receive a manual on preparation, handling, storage, and administration of the IMP.

6.2.2. Prefilled Syringe

For each injection, the initiation time (hour and minute) and details of any interruption or premature discontinuation of injections will be recorded and transferred to the eCRF. The start will be marked as the moment the plunger is pressed.

The first 3 consecutive IMP injections via PFS must be administered at the study site with safety monitoring by a health care provider. If home nursing care is already in place, the third consecutive administration may occur at the participant's home monitored by a home nurse. Participants will be monitored for clinical signs and symptoms of injection-/infusion-related reactions and injection-site reactions.

Site staff, the participant, the caregiver, a home nurse, or a concierge service can administer IMP.

To be considered competent, the participant or caregiver will (self-)administer IMP under the supervision of an investigator or study nurse for at least 1 injection. After the participant or caregiver is considered competent, the (self-)administration can continue, and supervision may no longer be needed at the discretion of the investigator.

For participants who cannot self-administer IMP and do not have a caregiver who can administer IMP, the site staff or a home nurse will administer IMP.

Participants and caregivers will receive "PFS Instructions for Use," which has details on IMP preparation, handling, storage, and administration, at the first supervised PFS IMP administration visit. Additional supervised (self-)administrations may occur at any time during the study if considered necessary by the investigator. It is recommended that participants or caregivers who are considered competent (self-)administer IMP whenever possible.

Participants and caregivers performing (self-)administrations will receive instructions regarding (self-)monitoring and identifying clinical signs and symptoms of injection-/infusion-related reactions and injection-site reactions.

6.2.3. Vial and Prefilled Syringe

For both vial and PFS presentations, the mode of administration (at the site during a study visit, [supervised] self-administration, caregiver-supported administration, home nurse, or concierge service for visit at the study site) will be recorded on the eCRF or will be documented and transferred to the eCRF.

The investigator, institution, or the head of the medical institution (where applicable) is responsible for IMP accountability, reconciliation, and record maintenance (ie, receipt, reconciliation, and final disposition records).

Further guidance and information for the final disposition of used and unused IMP are provided in the pharmacy manual.

6.3. IMP Compliance

IMP should be administered SC once weekly during the study or once every 2 or 3 weeks during the optional substudy to explore less frequent dosing (Section 10.9). The following compliance text is for the once-weekly dosing regimen of efgartigimod PH20 SC.

The investigator should ensure treatment compliance by stating that compliance is necessary for the potential effectiveness of participant's treatment and for the validity of the study.

The prescribed dose, timing, and mode of administration cannot be changed. All dates and start and end time of IMP administration and any deviations from the intended regimen must be recorded (Section 6.2).

Any participant who misses a single dose (within the window of ±2 days of the planned administration day) will not make up for the missed dose and will receive the next dose as scheduled in the SoA (Section 1.3). Also, if a dose must be delayed for more than 5 days, the dosing should be skipped to ensure 2 consecutive doses are given with an interval of at least 3 days.

The participant must not miss more than 2 consecutive doses and must not miss more than 10% of the total planned doses (for example, a maximum of 4 missed doses in each 48-week treatment period will be permitted, if doses missed are not consecutive) (Section 8.5).

In case of lack of compliance, the number of missed doses will be captured, but the participant can remain in the study (unless the participant withdraws his/her consent or meets 1 of the criteria to withdraw from the study; Section 7). The investigator should discuss the reason with the participant and should stress the importance of IMP compliance. If needed, the participant or the participant's caregiver should be retrained to (self-)administer the IMP.

6.4. Concomitant Medications

All concomitant medications whether allowed or not must be recorded on the eCRFs.

Concomitant medications will be coded using the World Health Organization (WHO) Drug Dictionary drug codes. Concomitant medications will be further coded to the appropriate Anatomical Therapeutic Chemical code indicating therapeutic classification.

Any vaccine or medication, including over-the-counter and prescription medications, vitamins, and/or herbal supplements, that the participant is receiving at the time of enrollment or receives during the study must be recorded with the following information provided:

- Reason for use
- Dates of administration including start and end dates
- Dosage information including dose and frequency

The medical monitor should be contacted with any questions regarding concomitant therapy.

Symptomatic treatments will be left to the discretion of the investigators.

6.4.1. Permitted Medication

Medication not used for the treatment of CIDP is permitted, if not part of the groups of medication specifically prohibited in Section 6.4.2.

Any kind of vaccination, except for live-attenuated vaccines, is allowed at the discretion of the investigator. In participants who receive a vaccination during the study, data on the potential interaction vaccine-IMP and on the efficacy of the vaccine may be collected.

6.4.1.1. Collection of Vaccination Data

For participants who receive a vaccination during the study, the date of administration and the brand name will be captured (as for other concomitant medications).

For participants for whom vaccination history data were not collected in ARGX-113-1802, any vaccination information the participant or caregiver can remember should be recorded on the eCRF (with the brand name of the vaccine and date of vaccine administration recorded, if possible). If in the past a same vaccine was received more than once, including as a booster, only the last one is to be collected.

For participants who provide separate consent, the titers of produced vaccination antibodies will be measured using retained blood samples (leftover blood samples taken for other tests) during the study (Section 10.2).

6.4.1.2. Participant's Self-Medication and IMP Administration via Home Nurse

Because this is an outpatient study, special care will be taken in questioning the participants on any self-medication, and participants will be asked to document details of IMP administration, which will be evaluated by site staff and transferred by an authorized study team member to the eCRF.

In case of IMP administrations in between study visits by a caregiver or home nurse, the caregiver or home nurse will record the IMP administration information and provide this to the study site for entry on the eCRF.

6.4.2. Prohibited Medications

The following medications or treatments are not permitted during the study as of signing the ICF until the last IMP administration:

- IVIg or SCIg therapy
- Plasmapheresis
- Total lymphoid irradiation
- Any other IgG therapy
- Any stem cell-based therapy, including bone marrow transplantation like autologous stem cell transplant (ASCT) and allogenic cells transplants
- Any cytokine or anticytokine therapy
- Systemic corticosteroids, including oral, for use in CIDP (Note: Short-term use [up to 2 weeks] for treatment of other indications is allowed)
- Cyclophosphamide, interferon, tumor necrosis factor–alpha inhibitors, fingolimod, methotrexate, azathioprine, mycophenolate, or any other immunomodulating or immunosuppressive medications or procedures
- Rituximab, alemtuzumab, or any other monoclonal antibody for immunomodulation
- Any investigational drug or experimental procedure
- Live-attenuated vaccines (Note: Receiving an inactivated, subunit, polysaccharide, or conjugate vaccine any time before screening is not prohibited)
- Use of complementary therapies, including traditional Chinese medications and herbal remedies, containing any of the following: naturally derived glucocorticoids for use in CIDP; medication with clinical evidence-based immunosuppressive, immunomodulatory, peripheral or CNS effects; or procedures (eg, acupuncture) for any neurological conditions within 4 weeks or 5 half-lives (whichever is longer) before IMP administration and agreed not to use during the study. (Note: Short-term use [up to 2 weeks] of glucocorticoids for other indications is allowed)

If the administration of a nonpermitted concomitant drug becomes necessary during the clinical study, eg, because of AEs, the participant has to discontinue IMP, as described in Section 7.1.

6.5. Dose Modification

Each IMP administration will have only 1 dose of IMP. Dose modification is not allowed during the study.

6.6. Intervention After the End of the Study

After the end of the study, participants will no longer receive IMP and will be treated as considered appropriate by the participant's treating physician.

6.7. Continued Access to Efgartigimod After the End of the Study

Participants will be offered the opportunity to continue in this study until 2 years after marketing authorization in a participant's country or until efgartigimod PH20 SC becomes commercially available for patients with CIDP or becomes available via continued access program or until 30 Apr 2027, whichever comes first.

At the end of the study, argenx cannot guarantee continued access for participants but will comply with all local laws and regulations.

7. IMP DISCONTINUATION AND PARTICIPANT DISCONTINUATION/WITHDRAWAL

Discontinuation of specific sites or the entire study are described in Appendix 1 (Section 10.1.9).

7.1. IMP Discontinuation and IMP Interruption

7.1.1. IMP Discontinuation

IMP discontinuation occurs when the participant stops receiving IMP before the end of the study and does not resume receiving IMP. The participant also must not have withdrawn informed consent.

The investigator will document the primary reason for IMP discontinuation. Unless consent from the study has been withdrawn, the participant will attend an ED visit and an SFV. The SFV will occur 28 (±3) days after the participant's final IMP administration.

The following circumstances will result in the discontinuation of IMP:

- Participant becomes pregnant or intends to become pregnant (Section 8.4.5).
- Investigator decides that discontinuing IMP is in the participant's best interest (the sponsor will be informed).
- Participant develops an SAE or AE that contraindicates further administration of IMP in the investigator's opinion or an AE of National Cancer Institute Common Terminology Criteria for Adverse Events (NCI CTCAE) grade 4 considered related to IMP by the sponsor.
- Participant develops a new or recurrent malignancy (except for basal cell carcinoma of the skin), regardless of relationship to the IMP.
- Participant receives a prohibited medication or substance (Section 6.4.2); short-term use (up to 2 weeks) for treatment of other indications is allowed.

7.1.2. IMP Interruption

IMP interruption occurs when the participant stops receiving IMP before the end of the study and resumes receiving IMP once the cause of the interruption has been resolved. Participants who interrupt IMP will attend any previously scheduled visits (Section 1.3).

The following circumstances may result in IMP interruption:

- Any SAE considered related to IMP by the sponsor
- Clinically significant active infection considered related to the IMP by the sponsor
- Psychiatric conditions

Note: *In the case of severe depression, suicidal ideation, or attempted suicide, study medication must be at least interrupted until the participant's psychiatric condition has been fully evaluated. A psychiatrist should be consulted whenever necessary. IMP may be resumed only after consultation with the sponsor.*

7.2. Participant Discontinuation/Withdrawal From the Study

Study withdrawal is defined as the permanent cessation of further participation in any study assessment before its planned completion.

The primary reason for study withdrawal will be recorded.

The following circumstances will result in study withdrawal:

- Participant withdrawal of consent
- Sponsor request (eg, following DSMB advice)

If the participant withdraws consent to participate in the study, the sponsor can retain and continue to use any data collected before consent was withdrawn.

Future research on samples collected from participants who withdraw consent to participate in the study will not be impacted unless the participant also withdraws the consent for future research.

7.2.1. Lost to Follow-Up

A participant will be considered lost to follow-up if they repeatedly fail to return for scheduled visits and cannot be contacted by the study site.

The following actions must be completed if a participant fails to complete a required study visit:

- The site must attempt to contact the participant and reschedule the missed visit as soon as possible, counsel the participant on the importance of maintaining the assigned visit schedule, and ascertain whether the participant wishes to continue in the study.
- Before a participant is considered lost to follow-up, the investigator or designee must make every effort to regain contact with the participant (when possible, 3 phone calls, and if necessary, a certified letter to the participant’s last known mailing address or local equivalent methods). These contact attempts will be documented in the participant’s medical record.
  - Participants who continue to be unreachable will be considered to have withdrawn from the study.

8. STUDY ASSESSMENTS AND PROCEDURES

Every effort should be made to ensure that the protocol-required tests and procedures are completed as described. When a protocol-required procedure cannot be performed, the investigator will document the reason and any corrective and preventive actions that he/she has taken to ensure that the normal processes are adhered to in the source documents. The study team should be informed of these incidents promptly.

Participants should be seen for all visits on the designated days or as closely as possible to the original planned visit schedule.

The timing of all study procedures is provided in the SoA in Section 1.3. Protocol waivers or exemptions are not allowed. Immediate safety concerns should be discussed with the sponsor immediately upon occurrence or awareness to determine if the participant should continue or discontinue IMP. Adherence to the study design requirements, including those specified in the SoA, is essential and required for study conduct. All evaluations at the SD1 visit must be completed and reviewed to confirm that potential participants meet all eligibility criteria. The investigator will maintain a log to record details of all participants who performed a SD1 visit and to confirm eligibility or record reasons for screen failure, as applicable.

The maximum amount of blood collected from each participant for each 48-week treatment period, including any extra assessments that may be required, will be approximately 200 mL (with a maximum of 40 mL per study visit).

At each study visit, all assessments as listed in the SoA in Section 1.3 should be completed before IMP administration at the site.

8.1. Medical History

Clinically significant findings and preexisting conditions present in a participant before the SD1 visit must be reported on the relevant medical history/current medical conditions page of the eCRF.

Information should be provided on medical and surgical history and concomitant medical conditions specifying those ongoing at the SD1 visit.

8.1.1. Use and Storage of Biological Samples

Biological samples may be used to validate methods to measure antibodies, biomarkers, and efgartigimod. Participants must consent to their samples being used in this manner before such measurements are made.

After the protocol-defined laboratory analyses have been completed, any samples remaining may be stored for up to 15 years after the end of the study, in the laboratory or long-term storage designated by the sponsor or research partners worldwide. These samples may be used for future additional medical, academic, or scientific research to address any scientific questions related to efgartigimod, FcRn, biology or CIDP, unless prohibited by local regulations or the participant.

8.2. Efficacy Assessments

Planned time points for all efficacy assessments are provided in the SoA in Section 1.3.

The efficacy assessments should preferably be performed early during a visit, and must be conducted before IMP administration (if applicable).

Where required, training will be provided for the efficacy instruments.

8.2.1. Adjusted Inflammatory Neuropathy Cause and Treatment Disability Score (aINCAT)

The aINCAT(Section 10.5) is a 10-point scale that covers the functionality of legs and arms and has been successfully used to measure treatment effects in various CIDP studies. Scores for arm disability range from 0 (“No upper limb problems”) to 5 (“Inability to use either arm for any purposeful movement”), and scores for leg disability range from 0 (“Walking not affected”) to 5 (“Restricted to wheelchair, unable to stand and walk a few steps with help”). The INCAT (total) score is the sum of these 2 scores and ranges from 0 to 10. For the aINCAT, changes in the function of the upper limbs from 0 (normal) to 1 (minor symptoms) or from 1 to 0 will not be recorded as deterioration or improvement because these changes are not considered clinically significant.

8.2.2. Medical Research Council (MRC) Sum Score

The 6 point MRC sum score evaluates motor strength/weakness (Section 10.6):

- 0 = complete paralysis
- 1 = minimal contraction
- 2 = active movement with gravity eliminated
- 3 = weak contraction against gravity
- 4 = active movement against gravity and resistance
- 5 = normal strength

The MRC sum score is evaluated bilaterally on 6 muscle groups of upper and lower limbs to obtain a summed score between 0 and 60:

- Arm abductors
- Elbow flexors
- Wrist extensors
- Hip flexors
- Knee extensors
- Foot dorsiflexors

8.2.3. Inflammatory Rasch-Built Overall Disability Scale (I-RODS)

The I-RODS (Section 10.7) is a PRO measure on disability and assesses the limitations of activities and social participation in participants with inflammatory neuropathies like Guillain-Barré syndrome (GBS), CIDP, and gammopathy-related polyneuropathy (MGUSP). The I-RODS is a 24-item scale, with each item representing a common daily activity that ranges from very difficult to do, like running or dancing, to very easy to do, like eating or reading a book. Higher scores indicate less disability. The raw sum scores of the I-RODS (range, 0-48) will be converted to a centile metric score, ranging from 0 (most severe activity and social participation restrictions) to 100 (no activity and social participation limitations).[25]

Although the I-RODS as a PRO can be considered subjective, the scale has been shown to correlate well with grip strength, which is a direct measure of muscle strength.[10]

8.2.4. Mean Grip Strength

A handheld Martin-Vigorimeter will be used for this assessment. Participants will perform 3 assessments for each hand in an arbitrary order (with a rest period of approximately 30 seconds between each assessment) always at approximately the same time during the day. The mean grip strength for each hand will be used to determine disease activity.

8.2.5. Timed Up and Go (TUG) Test

The TUG test is a simple test used to assess a person's mobility and requires both static and dynamic balance.[21]

In the TUG test, the time expended to rise from a chair, walk 3 meters, turn around, walk back to the chair, and sit down is measured. During the test, the person is expected to wear their regular footwear and use any mobility aids that they would normally require.

8.3. Safety Assessments

Planned time points for all safety assessments are provided in the SoA in Section 1.3.

8.3.1. Vital Signs and Physical Examination

The assessment of vital signs (systolic and diastolic blood pressure, heart rate, and body temperature) will be performed at the time points indicated in the SoA (Section 1.3). It is recommended to measure vital signs before blood collections and, if applicable, vital signs must be measured before IMP administration.

Blood pressure (systolic and diastolic) and heart rate will be measured using standard equipment in semisupine position after having rested for at least 5 minutes.

It is recommended that the method used to measure body temperature at the SD1 visit is maintained throughout the study for each participant.

A physical examination will include at a minimum an assessment of general appearance, skin, lymph nodes, musculoskeletal/extremities, abdomen, cardiovascular, respiratory, and neurological system.

Weight will be measured at the SD1 visit and during the study as indicated in the SoA (Section 1.3) in case of an obvious weight change. For the assessment of weight, participants will be required to remove their shoes and wear light indoor clothing. Unintentional weight loss of >10% of normal body weight over a period of 6 months or less should be reported as an AE. Clinically relevant findings observed after signing informed consent and meeting the definition of an AE (new AE or worsening of previously existing condition) must be recorded on an AE page of the eCRF.

8.3.2. Electrocardiograms

At the SD1 visit and during the first 48-week treatment period (and safety follow-up, if applicable) of the study (as indicated in the SoA in Section 1.3), 12-lead ECGs will be recorded with the participant in a supine position, suitably rested (for at least 5 minutes).

ECGs will be acquired according to instructions provided by a centralized ECG reading facility where the ECGs will be centrally assessed. At a minimum, interval data (PR, QT, QTcF, and QRS intervals), ventricular rate, and overall interpretation will be recorded for each ECG. Data will be transferred electronically for inclusion in the database.

Clinically relevant findings observed after signing informed consent and meeting the definition of an AE (new AE or worsening of previously existing condition) must be recorded on an AE page of the eCRF.

8.3.3. Clinical Safety Laboratory Assessments

Blood and urine samples for safety assessments (hematology, biochemistry, and urinalysis) will be taken at the SD1, before IMP administration, and further during the study as specified in the SoA in Section 1.3. Refer to Section 10.2 for a list of clinical laboratory tests to be performed.

Blood and urine samples for clinical chemistry, hematology, and urinalysis testing will be collected according to the local standard procedures.

The samples will be analyzed at a centralized certified laboratory. In the exceptional circumstance that samples cannot be sent to a central laboratory, a local laboratory may be used.

Further details of sampling, handling, storage, and transportation of the samples will be described in the laboratory manual. The sample collection date and time will be collected. The fasting status will also be collected.

Clinical laboratory tests will be reviewed for results of potential clinical significance at all time points throughout the study, with the exception of total protein and albumin results from treatment period 1 baseline through week 4, which will be kept blinded for the site. A system will be implemented to notify the investigator in case of out-of-range values to allow for appropriate safety follow-up. Clinically relevant findings observed after signing informed consent and meeting the definition of an AE (new AE or worsening of previously existing condition) must be recorded on an AE page of the eCRF.

Additional safety samples may be collected if clinically indicated at the discretion of the investigator. These additional samples will be analyzed centrally, too. Only laboratory testing needed for the assessment of a specific AE/SAE, which is not already foreseen in the laboratory test battery, will be analyzed locally. Such a request and the medical need has to be discussed a priori with the medical monitor. The results of this local testing will be added to the AE/SAE follow-up documentation.

For all female participants of childbearing potential, a urine pregnancy test will be performed locally at the site at the time points specified in the SoA.

Besides the urine pregnancy test and additional safety samples performed at the discretion of the investigator, which will be conducted and analyzed locally, all samples for safety assessments will be analyzed centrally.

8.3.4. Pregnancy Testing

- WOCBP will be tested for pregnancy by serum at screening. Urine tests for pregnancy will occur at the time points specified in the SoA (Section 1.3).
- Pregnancy testing in WOCBP will be conducted at the end of relevant systemic exposure (ie, at the SFV).
- Additional pregnancy testing may be performed as necessary by the investigator or as required by local regulations, to establish the absence of pregnancy at any time during the study.
- Any pregnancy reported during a clinical research study, including the safety follow-up period, is routinely monitored as standard practice. The pregnancy could have arisen from a female clinical study participant or a male participant's female partner. In either situation, consent will be requested to collect medical information about the pregnancy and the baby's health for up to 12 months after the baby's birth.

8.4. Adverse Events, Serious Adverse Events, and Other Safety Reporting

The definitions of AE and SAE are provided in Appendix 3 (Section 10.4). An AESI is an AE of scientific and medical concern specific to the sponsor's product or program and described in Section 8.4.6.

AEs (including SAEs, AESIs, and AEs of clinical interest) will be reported by the participant (or, if appropriate, by the caregiver or surrogate).

The investigator and qualified designees are responsible for detecting, documenting, and recording events that meet the definition of an AE or SAE and monitoring all reported events, including those reported by the participant.

The method of recording, evaluating, and assessing the causality of AEs and SAEs and the procedures for completing and transmitting SAE reports are provided in Appendix 4 (Section 10.4).

8.4.1. Time Period and Frequency for Collecting AE and SAE Information

All AEs will be collected from the signing of the ICF until the SFV, as specified in the SoA (Section 1.3).

All SAEs and AESIs will be recorded and reported to the sponsor or designee immediately, and under no circumstance will this exceed 24 hours, as indicated in Appendix 4 (Section 10.4). The investigator will submit any updated SAE data to the sponsor within 24 hours of it being available.

Investigators are not obligated to actively seek information on AEs or SAEs after the conclusion of the study participation. However, if the investigator learns of any SAE, including death, at any time after a participant has been discharged from the study, and they consider the event reasonably related to IMP or study participation, the investigator must promptly notify the sponsor.

8.4.2. Method of Detecting AEs and SAEs

Care will be taken not to introduce bias when detecting AEs and SAEs. Open-ended and nonleading verbal questioning of the participant is preferred to inquire about AE occurrences.

8.4.3. Follow-Up of AEs and SAEs

After the initial AE/SAE report, the investigator must proactively follow each participant at subsequent visits/contacts. All SAEs and AESIs defined in Section 8.4.6 will be followed until resolution, stabilization, the event is explained, or the participant is lost to follow-up (defined in Section 7.2.1). Further information on follow-up procedures is provided in Section 10.4.

8.4.4. Regulatory Reporting Requirements for SAEs

- Prompt notification by the investigator to the sponsor of an SAE is essential so that legal obligations and ethical responsibilities toward the safety of participants and the safety of IMP under clinical investigation are met.
- The sponsor has a legal responsibility to notify both the local regulatory authority and other regulatory agencies about the safety of IMP under clinical investigation. The sponsor will comply with country-specific regulatory requirements relating to safety reporting to the regulatory authority, IRBs/IECs, and investigators.
- An investigator who receives a safety report describing an SAE or other specific safety information (eg, summary or listing of SAEs) from the sponsor will review and file it with the IB and notify the IRB/IEC, if appropriate, according to local requirements.
- The sponsor or designee will be responsible for reporting SUSARs to the appropriate reporting system (eg, EudraVigilance database, FDA AE reporting system) per the applicable regulatory requirements. The sponsor or designee will also be responsible for forwarding SUSAR reports to all study investigators, who must report these SUSARs to their respective IECs/IRBs per local regulatory requirements.

8.4.5. Pregnancy

- If pregnancy is reported, the investigator will record the pregnancy information on the appropriate form and submit it to the sponsor within 24 hours of learning of the pregnancy in the female participant or the female partner of the male participant. Contact details are provided in Serious Adverse Event Reporting.
- The participant and pregnant female partner, if consented (Section 10.1.3), will be followed to determine the outcome of the pregnancy. The investigator will collect follow-up information on the participant/pregnant female partner and the neonate and forward it to the sponsor.
- While pregnancy itself is not considered an AE or SAE, any pregnancy complication or elective termination of a pregnancy for medical reasons will be reported as an AE or SAE. Abnormal pregnancy outcomes (eg, spontaneous abortion, fetal death, stillbirth, congenital anomalies, ectopic pregnancy) are considered SAEs and will be reported accordingly.

- Any poststudy pregnancy-related SAE considered reasonably related by the investigator to IMP will be reported to the sponsor as described in Section 8.4.4.
- Any female participant who becomes pregnant during the study will discontinue IMP.

8.4.6. Adverse Events of Special Interest (AESIs)

An AESI is an AE of scientific and medical concern specific to the sponsor's product or program. An AESI can be serious or nonserious, related or not related to the IMP or study procedures. These events will be reported according to the same time frame as that for SAEs specified in Section 8.4.1 and Section 10.4.4.

Efgartigimod treatment leads to reduced IgG levels. Because low IgG levels can be associated with increased infection risks, events in the Medical Dictionary for Regulatory Activities (MedDRA) SOC *Infections and infestations* are considered AESIs. These events will be reported according to the time frame specified in Section 8.4.1 and Section 10.4.4, with the following information provided:

- Causal pathogen
- Location of infection
- Relationship to an underlying medical condition, medical history, and concomitant medications
- Reoccurrence of a previous infection
- Any confirmatory procedure, culture, or urgent medical intervention, if applicable

8.4.7. AEs of Clinical Interest

8.4.7.1. Injection-/Infusion-Related Reactions

All therapeutic proteins can elicit immune responses, potentially resulting in hypersensitivity or allergic reactions such as rash, urticaria, angioedema, serum sickness, and anaphylactoid or anaphylactic reactions. As with any SC or IV injection, injection- or injection-/infusion-related reactions can occur during or after administration.

Overall, the frequency of injection-/infusion-related reactions in clinical studies has been low.

Refer to the current efgartigimod IB for more information on injection-/infusion-related reactions.

8.4.7.2. Injection-Site Reactions

An injection-site reaction is any AE developing at the injection-site. Localized injection-site reactions are frequently observed in studies in which efgartigimod is comixed with PH20 and administered SC. The most frequently reported injection-site reaction AEs are *Injection-site erythema*, *Injection-site pain*, and *Injection-site swelling*.

Any injection-site reaction will be reported as an AE (Section 8.4). Certain types of local reactions could be photographed and shared with the sponsor for review and assessment.

As a routine precaution, participants will be trained or observed closely by a trained health care professional for any potential injection-site reaction.

Refer to the current efgartigimod IB for more information on injection-site reactions.

8.5. Medication Error

8.5.1. Vial

A variation of less than ±10% of the amount of efgartigimod PH20 SC that should be injected for each administration will not be considered an overdose/underdose or medication error, and a variation greater than ±10% of the intended amount of efgartigimod PH20 SC will be considered a medication error and/or overdose with or without AEs.

8.5.2. Prefilled Syringe

Efgartigimod PH20 SC is provided as a single-dose PFS, with approximately 4% additional drug product to ensure the correct extractable amount. Accordingly, an overdose is not possible if 1 PFS is used for each administration.

8.5.3. Vial and Prefilled Syringe

For both PFS and vial presentations, a participant must not miss more than 2 consecutive doses and must not miss more than 10% of the total planned doses that should have been taken (for a participant receiving efgartigimod PH20 SC once weekly). Furthermore, 2 consecutive doses should be given with an interval of at least 3 days (Section 6.3).

An overdose is defined as a deliberate or accidental administration of efgartigimod PH20 SC to a participant, at a dose greater than that which was assigned to that participant per the study protocol.

A medication error is any preventable incident that may cause or lead to inappropriate efgartigimod PH20 SC use or participant harm while the efgartigimod PH20 SC is in the control of the health care professionals. Such incident may be due to health care professional practice, product labeling, packaging and preparation, procedures for administration, and systems, including the following: prescribing, order communication, nomenclature, compounding, dispensing, distribution, administration, education, monitoring, and use.

In case of suspected overdose or medication error, the participant should be treated according to standard medical practice based on the investigator’s judgment. The suspected overdose or medication error with the quantity of the excess dose should be documented on the eCRF including the additional AE, if any.

8.6. Participant Card

Participants must be provided with the address and telephone number of the investigator, and of the main contact for information about the clinical study. The investigator must therefore provide a “Participant Card” to each participant. In an emergency situation this card serves to inform the responsible attending physician that the participant is in a clinical study and that relevant information may be obtained by contacting the investigator or sponsor designee. Participants must be instructed to keep the card in their possession at all times.

8.7. Pharmacokinetics

During the first 48-week treatment period, blood samples for determination of serum concentrations of efgartigimod will be collected predose from SD1 onward and at certain time points up to the SFV (if applicable) as specified the SoA in Section 1.3.

Blood for the PK samples will be collected as described in the laboratory manual.

Efgartigimod serum concentrations will be determined using a validated method.

8.8. Pharmacodynamics

Blood samples for determination of total IgG in serum will be taken predose at several time points during the study from the SD1 visit onward as indicated in the SoA in Section 1.3.

Blood for the PD samples will be collected as described in the laboratory manual.

Total IgG levels will be determined using a validated method.

8.9. Genetics

Not applicable.

8.10. Biomarkers

Refer to Section 8.8 for the collection of PD samples to measure IgG.

During the first 48-week treatment period, blood samples (serum) will be collected from all participants who provided separate consent for biomarker analysis at the time points provided in the SoA in Section 1.3 for potential future analysis of biomarkers, which may include but are not limited to chemokines, cytokines, complement, protein expressions, and others.

The blood draws for evaluation of the biomarkers will be clearly mentioned in the ICF of this study.

Biomarkers will be analyzed and reported separately from the main clinical study report (CSR).

8.11. Immunogenicity Assessments

Blood samples will be collected at the time points indicated in the SoA (Section 1.3) and predose during efgartigimod PH20 SC administration visits to evaluate serum levels of ADA against efgartigimod or plasma levels of antibodies against rHuPH20. Additionally, blood will be collected, stored, and analyzed to evaluate immunogenicity against rHuPH20 (measured in plasma) and if there are safety concerns.

Neutralizing antibodies (NAb) will be tested for all confirmed positive ADA samples.

Blood for the immunogenicity samples will be collected as described in the laboratory manual.

Samples will be analyzed by the designated laboratory in a tiered approach using validated immunogenicity assays.

8.12. Additional Patient-Reported Outcome (PRO) Tools

Additional PRO data, associated with medical encounters, are measured at the SD1 visit and at several time points as indicated in the SoA in Section 1.3 and will be collected in the CRF by the investigator and site staff for all participants. Protocol-mandated procedures, tests, and encounters are excluded.

The data collected may be used to conduct exploratory economic analyses.

The data collected are based on the PROs of the questionnaires described as follows.

8.12.1. EQ-5D-5L

QoL will be assessed through the EQ-5D-5L, which allows responses recording based on 5 levels of severity.[12] It is a standardized instrument for use as a measure of health for clinical and economical appraisal.

8.12.2. Brief Pain Inventory – Short Form (BPI-SF)

The Brief Pain Inventory-Short Form (BPI-SF) is a validated, self-administered questionnaire designed to measure a participant's perceived level of pain and the impact of this pain on the participant's daily functioning. The BPI-SF measures the participant's intensity of pain (sensory dimension), the interference of pain in the participant's life (reactive dimension), and asks the participant about pain relief, pain quality, and the participant's perception of the cause of pain.[7] The BPI-SF consists of 9 items that use a numeric rating scale to assess pain severity and pain interference in the past 24 hours.

8.12.3. Treatment Satisfaction Questionnaire for Medication (TSQM)

The original version (Version 1.4) of the Treatment Satisfaction Questionnaire for Medication (TSQM) questionnaire was developed in 2004 to measure participants' satisfaction with their medication using 4 scales (side effects, effectiveness, convenience, and global satisfaction).[2] This questionnaire was further developed in 2005 (Version II) using 4 fewer items and more consistent wording and has equivalent measurement characteristics as the original questionnaire.[3] In addition, an abbreviated 9-item TSQM (TSQM-9) derived from the TSQM Version 1.4, but without the 5 items of the side effects domain, was created. The TSQM-9 was found to be a reliable and valid measure to assess treatment satisfaction in naturalistic study designs, in which there is potential that the administration of the side effects domain of the TSQM would interfere with routine clinical care.[4] In this study, the TSQM-9 will be used.

8.12.4. Rasch-Transformed Fatigue Severity Scale (RT-FSS)

The original Fatigue Severity Scale (FSS) was a 9-item scale to measure the severity of fatigue and its impact on the participant's daily activities and life style. It was found that a 7-item Rasch-Transformed FSS (RT-FSS) gave a more proper assessment of fatigue in participants with immune-mediated neuropathies. Therefore, in this study, the abbreviated RT-FSS will be used.[24]

8.12.5. Hospital Anxiety and Depression Scale (HADS)

The Hospital Anxiety and Depression Scale (HADS) was originally developed by Zigmond and Snaith in 1983.[27] HADS aims to measure symptoms of anxiety and depression in people with physical health problems. HADS is a 14-item scale with 7 items related to anxiety and 7 to depression. HADS is widely used and has been validated.[5]

8.13. Suicidality Assessment

A prospective assessment for suicidal ideation and behavior is included in this clinical study, as recommended for studies involving a biological product for a neurological indication.[6]

The suicidality assessment will be conducted by specifically answering the following question, derived from the PHQ-9 (Patient Health Questionnaire-9) depression questionnaire:

– "Over the last 2 weeks, how often have you been bothered by thoughts that you would be better off dead, or of hurting yourself in some way?"

The participant will be asked this question at each study visit (refer to the SoA in Section 1.3) and the response will be documented. Response options per the PHQ-9 are limited to the following: "*not at all*," "*several days*," "*more than half the days*," or "*nearly every day*."

This specific question was selected for the reported significant linear relationship between the item 9 score of the PHQ-9 and the risk of subsequent suicide attempt.[20]

9. STATISTICAL CONSIDERATIONS

The statistical analyses will be performed by the sponsor's designated CRO using statistical analysis systems SAS (SAS Institute, Cary, NC, US) version 9.4 or higher, and the software package R, if applicable. The standard operating procedures (SOPs) and work instructions of the sponsor's designated CRO will be used as the default methodology if not specified.

A detailed and comprehensive statistical analysis plan (SAP) will be written and signed off before database lock for interim analyses and the final analysis. Interim analyses may be performed for interactions with health authorities and/or when all participants have completed (or discontinued) a treatment period (Section 9.5.6). The final statistical analysis will be performed once all participants have completed the study (or are withdrawn) and the database has been cleaned and locked.

Minor changes to the statistical methods set out in this protocol do not require a protocol amendment but must be documented (as changes from the protocol) in the SAP and in the CSR(s).

9.1. Statistical Methods

9.1.1. Summary Tables, Figures, and Listings

Summary tables will summarize endpoints (outcomes) by treatment group and time point (eg, visit). For continuous variables, the number of participants (with nonmissing values), mean, standard deviation, median, quartiles, minimum, and maximum will be presented. For categorical variables, for each category, the number of participants (count) and percentage will be presented for participants without missing values.

All summary tables and figures will be supported by full participant listings.

9.1.2. Hypothesis Testing and Confidence Intervals

No formal hypothesis testing will be conducted. If appropriate, 2-sided 95% CIs may be presented.

9.1.3. Populations for Analyses

The safety and modified intent-to-treat (mITT) population consists of all participants who rolled over from ARGX-113-1802 and who received at least 1 dose of open-label efgartigimod PH20 SC in ARGX-113-1902.

Participants will be classified according to the treatment they received during Stage B of ARGX-113-1802 (ie, efgartigimod PH20 SC or placebo).

9.1.4. Randomization and Stratification

Not applicable for this OLE study.

9.1.5. General Considerations

9.1.5.1. Visits and Dates

All study visits will be recalculated based on dates and will be referred to as "analysis visits" that will be used in the statistical analyses. The rules for calculating the analysis visits will be documented in the SAP. The visit recorded in the database and the analysis visits will be presented in the listings.

Rules for imputing partial dates or missing dates will also be documented in the SAP.

9.2. Statistical Hypotheses

No hypotheses are planned to be tested in this study.

9.3. Sample Size Determination

Eligible participants who rolled over from ARGX-113-1802 will be included. Refer to Section 4.1 and Section 5 for the criteria to enter this OLE study.

9.4. Endpoints

9.4.1. Primary Endpoint

9.4.1.1. Safety and Tolerability

- Incidence of TEAEs and SAEs by System Organ Class (SOC) and Preferred Term (PT)
- Incidence of clinically significant laboratory abnormalities

9.4.2. Secondary Endpoints

9.4.2.1. Efficacy

- Change from baseline over time in the following scores and measurements:
  - aINCAT
  - MRC sum score
  - 24-item I-RODS disability scores
  - Mean grip strength assessed by Martin-Vigorimeter
  - TUG score
- Percentage of participants without clinical deterioration over time, defined by an aINCAT increase ≥1 point (worsening) compared to baseline

9.4.2.2. Immunogenicity

– Percentage of participants with ADA against efgartigimod and the presence of NAb against efgartigimod (during the first 48-week treatment period [followed by a safety follow-up period, if applicable])

9.4.2.3. Pharmacokinetics

– Efgartigimod serum concentrations (during the first 48-week treatment period [followed by a safety follow-up period, if applicable])

9.4.2.4. Pharmacodynamics

– Changes from baseline over time of serum IgG levels (total)

9.4.2.5. Additional Patient-Reported Outcome

– Change from baseline over time in:
  - Health-related quality of life questionnaire (EQ-5D-5L)
  - BPI-SF
  - TSQM-9
  - RT-FSS
  - HADS

9.4.2.6. Other Endpoints

– Percentage of participants performing self-administration over time

– Percentage of participants with caregiver-supported treatment administration over time

Population PK/PD analysis can be performed based on the PK and PD data collected and will be reported separately.

Note: Endpoints for the optional substudy to explore less frequent dosing are defined in Section 10.9.

9.5. Statistical Analyses

The SAP (including the SAP for an interim analysis; Section 9.5.6) will be finalized before each corresponding database lock and will include a more technical and detailed description of the statistical analyses described in this section. This section is a summary of the planned statistical analyses.

9.5.1. Demographic and Baseline Characteristics

Participant demographic and baseline characteristics data, including prior and concomitant therapies, will be summarized using standard summary statistics (Section 9.1.1).

9.5.2. Populations for Analyses and Protocol Deviations

The number of participants for each population for analysis, as described in Section 9.1.3, will be presented in a table and listing.

Major protocol deviations, by treatment group and study stage, will be summarized in a table and presented in a listing.

9.5.3. Participant Disposition

A tabular presentation of the participant disposition will be provided. It will include the number of participants who performed the SD1 visit, received IMP treatment, completed a treatment period, completed the study, and the number of early discontinuations from IMP treatment and study, with reasons for discontinuation from IMP treatment or study, and major protocol deviations.

For each participant, a listing will be presented to describe dates of the SD1 visit, screen failure with reason, completion or early discontinuation, and the reason for discontinuation, if applicable.

9.5.4. Primary Endpoint (Safety and Tolerability)

Summaries of AEs and other safety parameters will be provided.

AEs will be classified using the latest version of the MedDRA classification system. AEs reported from the first dose of IMP until 30 days after the last dose of IMP will be considered as TEAEs and will be summarized descriptively. TEAEs (including AESIs) will be listed corresponding to SOC and MedDRA PT. Multiple occurrences of a single PT in a participant will only be counted once at the maximum severity/grade. The severity will be assessed using the CTCAE (Section 10.4). All AEs will be summarized by relatedness to IMP. Any AEs leading to death or discontinuation of IMP will also be summarized.

Laboratory and vital sign parameters and their respective changes from baseline will be presented using standard summary statistics (Section 9.1.1). Clinically significant laboratory abnormalities will also be summarized descriptively.

9.5.5. Secondary Endpoints

For a definition of the efficacy, immunogenicity, PK, PD, additional PROs, and other endpoints, refer to Section 9.4.2.

Data from these secondary endpoints will be presented using summary statistics (Section 9.1.1).

In addition, exploratory analyses may be performed as needed and will be clearly marked as “exploratory.”

9.5.6. Interim Analyses

Interim analyses (with separate interim analysis SAPs providing details) may be performed for interactions with regulatory authorities and/or when all participants have completed (or discontinued) a treatment period (refer to Section 10.9). A first interim analysis of the OLE study, based on data from the once-weekly dosing regimen, is planned around the time of the database lock of ARGX-113-1802 (ie, when the required number of events has been observed for the primary endpoint analysis for Stage B of ARGX-113-1802).

9.6. Independent Data and Safety Monitoring Board

Details on the DSMB are provided in Section 10.1.5 (Appendix 1).

10. SUPPORTING DOCUMENTATION AND OPERATIONAL CONSIDERATIONS

10.1. Appendix 1: Regulatory, Ethical, and Study Oversight Considerations

10.1.1. Regulatory and Ethical Considerations

This study will be conducted and the informed consent will be obtained according to the ethical principles stated in the current Declaration of Helsinki, the applicable guidelines for Good Clinical Practice (GCP), or the applicable drug and data protection laws and regulations of the countries where the study will be conducted.

The protocol, protocol amendments, ICF, IB, and other relevant documents (eg, advertisements) must be submitted to an IRB/IEC by the investigator and reviewed and approved by the IRB/IEC before the study is initiated.

Any amendments to the protocol will require IRB/IEC approval before implementation of changes made to the study design, except for changes necessary to eliminate an immediate hazard to study participants.

The investigator will be responsible for the following:

- Providing written summaries of the status of the study to the IRB/IEC annually or more frequently in accordance with the requirements, policies, and procedures established by the IRB/IEC
- Notifying the IRB/IEC of SAEs or other significant safety findings as required by IRB/IEC procedures
- Providing oversight of the conduct of the study at the site and adherence to requirements of Title 21 of the Code of Federal Regulations (21 CFR), International Council for Harmonisation of Technical Requirements for Pharmaceuticals for Human Use (ICH) guidelines, the IRB/IEC, European regulation 536/2014 for clinical studies (if applicable), and all other applicable local regulations

10.1.2. Financial Disclosure

Investigators and subinvestigators will provide the sponsor with sufficient, accurate financial information as requested to allow the sponsor to submit complete and accurate financial certification or disclosure statements to the appropriate regulatory authorities. Investigators are responsible for providing information on financial interests during the study and for 1 year after study completion.

10.1.3. Informed Consent Process

The investigator or his/her representative will explain the nature of the study to the participant and answer all questions regarding the study.

Participants must be informed that their participation is voluntary. Participants will be required to sign a statement of informed consent that meets the requirements of 21 CFR 50, local regulations, ICH guidelines, Health Insurance Portability and Accountability Act (HIPAA) requirements, where applicable, and the IRB/IEC or study center.

The medical record must include a statement that written informed consent was obtained before any assessment of the SD1 visit was performed and the date the written consent was obtained. The authorized person obtaining the informed consent must also sign the ICF.

Participants must be reconsented to the current version of the ICF(s) during their participation in the study for each treatment period.

A copy of the ICF(s) must be provided to the participant.

10.1.4. Data Protection

Participants will be assigned a unique identifier by the sponsor. Any participant records or data sets transferred to the sponsor will contain this identifier; participant names or any information that would make the participant identifiable will not be transferred.

The participant must be informed that his/her personal study-related data will be used by the sponsor in accordance with local data protection law. The level of disclosure must also be explained to the participant who will be required to give consent for their data to be used as described in the informed consent.

The participant must be informed that his/her medical records may be examined by clinical quality assurance auditors or other authorized staff appointed by the sponsor, by appropriate IRB/IEC members, and by inspectors from regulatory authorities.

10.1.5. Independent Data and Safety Monitoring Board

An independent DSMB will be used in this study (and for ARGX-113-1802).

The sponsor will appoint a DSMB consisting of an independent group of clinical experts who are not participating in the study. They will be supplemented by an independent statistician. The objective of the DSMB will be to review all safety data (including the overall number of participants treated up to that point, rates, and participant-level details).

The DSMB will review data from ARGX-113-1802 and ARGX-113-1902 together before ARGX-113-1802 is completed. Thereafter, data from ARGX-113-1902 will be presented to the board every 6 months or otherwise as specified by the DSMB, based on findings from ARGX-113-1802. In addition, ad hoc meetings may be requested at any time during the study by the sponsor or the DSMB. The DSMB will advise the sponsor concerning continuation, modification, or termination of the study after every meeting.

The composition, objectives, and role and responsibilities of the DSMB will be described in a charter. The DSMB charter will also define and document the content of the safety summaries and general procedures (including communications).

10.1.6. Dissemination of Clinical Study Data

All information regarding efgartigimod PH20 SC supplied by the sponsor to the investigator and all data generated as a result of this study are considered confidential and remain the sole property of the sponsor. The results of the study will be reported in a CSR.

The CSR, written in accordance with the ICH E3 guideline, will be submitted in accordance with local regulations.

Any manuscript, abstract or other publication, presentation of results, or information arising in connection with the study must be prepared in conjunction with the sponsor and must be submitted to the sponsor for review and comment before submission for publication or presentation. Study participant identifiers will not be used in the publication of results.

Authorship will be granted according to the International Committee of Medical Journal Editors criteria, based on scientific input and recruitment efforts, and will be granted upon decision of a publication committee. This committee will include among others the coordinating investigator and the sponsor.

The sponsor will register and/or disclose the existence of clinical studies and their results as required by law.

10.1.7. Data Quality Assurance

All participant data relating to the study will be recorded on the eCRF unless transmitted to the sponsor or designee electronically (eg, laboratory data). The investigator is responsible for verifying that data entries are accurate and correct by electronically signing the CRF.

The investigator must maintain accurate documentation (source data) that supports the information entered in the CRF.

The investigator must permit study-related monitoring, audits, IRB/IEC review, and regulatory agency inspections and provide direct access to source data documents.

Monitoring details describing strategy (eg, risk-based initiatives in operations and quality such as Risk Management and Mitigation Strategies and Analytical Risk-Based Monitoring), methods, responsibilities, and requirements, including handling of noncompliance issues and monitoring techniques (central, remote, or on-site monitoring) are provided in the Clinical Monitoring Plan.

The sponsor or designee is responsible for the data management of this study including quality checking of the data.

The sponsor assumes accountability for actions delegated to other individuals (eg, CROs).

Study monitors will perform ongoing source data verification to confirm that data entered into the CRF by authorized site staff are accurate, complete, and verifiable from source documents; that the safety and rights of participants are being protected; and that the study is being conducted in accordance with the currently approved protocol and any other study agreements, ICH GCP, and all applicable regulatory requirements.

Records and documents, including signed ICFs, pertaining to the conduct of this study must be retained by the investigator for 25 years after study completion unless local regulations or institutional policies require a longer retention period. No records may be destroyed during the retention period without the written approval of the sponsor. No records may be transferred to another location or party without written notification to the sponsor.

10.1.8. Source Documents

Source documents provide evidence for the existence of the participant and substantiate the integrity of the data collected. Source documents will be filed at the investigator's site. These data are usually later entered on the eCRF.

Data reported on the CRF or entered on the eCRF that are transcribed from source documents must be consistent with the source documents, or the discrepancies must be explained. The investigator may need to request previous medical records or transfer records, depending on the study. Also, current medical records must be available.

The Investigator Source Data Agreement Form describes the source data for the different data on the eCRF. This document should be completed and signed by the investigator and should be filed in the investigator's study file. Any data item for which the eCRF will serve as the source must be identified, agreed upon, and documented in the Investigator Source Data Agreement Form.

10.1.9. Study and Site Start and Closure

The study start date is the date on which the first participant signs the ICF in this study.

The sponsor designee reserves the right to close the study site or terminate the study at any time for any reason at the sole discretion of the sponsor. Study sites will be closed upon study completion. A study site is considered closed when all required documents and study supplies have been collected and a site closure visit has been performed.

The investigator may initiate site closure at any time, provided there is reasonable cause and sufficient notice is given in advance of the intended termination.

Reasons for the early closure of a study site by the sponsor or investigator may include but are not limited to:

- Failure of the investigator to comply with the protocol, the requirements of the IRB/IEC or local health authorities, the sponsor's procedures, or GCP guidelines;
- Inadequate recruitment of participants by the investigator;
- Discontinuation of further IMP development.

If the study is prematurely terminated or suspended, the sponsor will promptly inform the investigators, the IECs/IRBs, the regulatory authorities, and any CRO(s) used in the study of the reason for termination or suspension, as specified by the applicable regulatory requirements. The investigator will promptly inform the participant and should ensure appropriate participant therapy and/or follow-up.

10.2. Appendix 2: Clinical Laboratory Tests

Table 4: Safety Laboratory Testing

Hematology		
Red blood cell count	Hemoglobin	Hematocrit
Mean cell volume	Mean cell hemoglobin	Mean cell hemoglobin concentration
Red cell distribution width	Platelets	Monocytes
White blood cell count	Lymphocytes count	Neutrophils count
Eosinophils	Basophils	
Coagulation		
International normalized ratio	Activated partial thromboplastin time	Prothrombin time
Blood chemistry tests		
Glucose	Creatinine	Total protein[a]
Albumin[a]	Bilirubin	Alkaline phosphatase
Aspartate transferase	Alanine transferase	Gamma glutamyl transferase
High-sensitive C reactive protein	Blood urea nitrogen	Estimated glomerular filtration rate[b]
Lactate dehydrogenase	Uric acid	Hemoglobin A1c
Sodium	Calcium	Potassium
Urine analysis		
• Dipstick: pregnancy, pH, protein, glucose, ketone, bilirubin, urobilinogen, blood, nitrite, leucocytes, and specific gravity • In case of abnormal dipstick results,[c] light microscopy: erythrocytes, leucocytes, casts, and epithelial cells		
Pharmacokinetic analysis		
Serum concentrations of efgartigimod during the first 48-week treatment period		
Pharmacodynamic analysis		
Concentrations of total IgG		
Immunogenicity analysis		
ADA (via ELISA) and NAb against efgartigimod (serum samples) during the first 48-week treatment period; antibodies and NAb against rHuPH20 (plasma samples) if there are safety concerns		
Samples for future biomarker analysis[d]		
Serum samples will be stored for potential future analysis of biomarkers.		

ADA=antidrug antibody(ies); ELISA=enzyme-linked immunosorbent assay; NAb=neutralizing antibody(ies); rHuPH20=recombinant human hyaluronidase PH20
Note: Investigators must document their review of each laboratory safety report.

[a] In the exceptional circumstance that samples are not processed in a central laboratory: Total protein and albumin analysis at baseline up to and including week 4 of treatment period 1 only will not be performed.
[b] Chronic Kidney Disease – Improved Prediction Equations (CKD-EPI) will be used for estimating glomerular filtration rate; it will be reported as creatinine clearance.
[c] If urinary tract infection is suspected, investigations (such as microscopy, culture, and sensitivity) should be performed locally by the investigator.
[d] Blood samples for potential future biomarker analysis will be collected from all participants who provided separate consent for biomarker analysis during the first 48-week treatment period.

10.3. Appendix 3: Contraceptive and Barrier Guidance

10.3.1. General Instructions and Definitions

Contraceptive use by men and women should be consistent with local regulations regarding the methods of contraception.

Women of childbearing potential must have a negative serum pregnancy test at the screening visit and a negative urine pregnancy test at baseline before IMP may be administered.

A woman is considered of childbearing potential unless she is either:

- Postmenopausal, defined by continuous amenorrhea for at least 1 year without an alternative medical cause with a follicle-stimulating hormone (FSH) measurement of >40 IU/L. A historical pretreatment FSH measurement of >40 IU/L is accepted as proof of a postmenopausal state for women on hormone replacement therapy.
- Surgically sterilized: Women who have had a documented permanent sterilization procedure (ie, hysterectomy, bilateral salpingectomy, or bilateral oophorectomy).

10.3.2. Female Contraception for Women of Childbearing Potential

Women of childbearing potential should use an acceptable method of contraception during the study.

The following methods of contraception, described in the Recommendations Related to Contraception and Pregnancy Testing in Clinical Trials,[8] are allowed in this study:

- Combined (estrogen and progestogen containing) hormonal contraception associated with inhibition of ovulation:
  - Oral
  - Intravaginal
  - Transdermal
- Progestogen-only hormonal contraception associated with inhibition of ovulation:
  - Oral
  - Injectable
  - Implantable
- Progestogen-only oral hormonal contraception, where inhibition of ovulation is not the primary mode of action

- Intrauterine device (IUD)
- Intrauterine hormone-releasing system (IUS)
- Bilateral tubal occlusion
- Vasectomized partner
- Sexual abstinence
- Male or female condom with or without spermicide
- Cap, diaphragm, or sponge with spermicide

10.3.3. Male Contraception

No male contraception is required.

10.4. Appendix 4: Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow-Up, and Reporting

10.4.1. Definition of AE

AE Definition
– An AE is any untoward medical occurrence in a clinical study participant, temporally associated with the use of IMP, whether or not considered related to the IMP. – NOTE: An AE can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of IMP.

Events to Be Collected as AEs
– Any abnormal laboratory test results (hematology, clinical chemistry, or urinalysis) or other safety assessments (eg, ECG, radiological scans, vital sign measurements), including those that worsen from baseline, considered clinically significant in the medical and scientific judgment of the investigator (ie, not related to progression of underlying disease) – Exacerbation of a chronic or intermittent preexisting condition including either an increase in frequency and/or intensity of the condition – New condition detected or diagnosed after IMP administration even though it could have been present before the start of the study – Signs, symptoms, or the clinical sequelae of a suspected intervention-intervention interaction – Signs, symptoms, or the clinical sequelae of a suspected overdose of either IMP or a concomitant medication. Overdose per se will not be reported as an AE/SAE unless it is an intentional overdose taken with possible suicidal/self-harming intent. Such overdoses will be reported regardless of sequelae – Lack of efficacy or failure of expected pharmacological action per se will not be reported as an AE or SAE. Such instances will be captured in the efficacy assessments. However, the signs, symptoms, and/or clinical sequelae resulting from lack of efficacy will be reported as AE or SAE if they fulfill the definition of an AE or SAE.

Events NOT to Be Collected as AEs
– Any clinically significant abnormal laboratory findings or other abnormal safety assessments that are associated with the underlying disease, unless considered by the investigator to be more severe than expected for the participant's condition – The disease/disorder being studied or expected progression, signs, or symptoms of the disease/disorder being studied, unless more severe than expected for the participant's condition – Medical or surgical procedure (eg, endoscopy, appendectomy): the condition that leads to the procedure is the AE – Situations in which an untoward medical occurrence did not occur (social and/or convenience admission to a hospital)

– Anticipated day-to-day fluctuations of preexisting disease(s) or condition(s) present or detected at the start of the study that do not worsen

10.4.2. Definition of SAE

An SAE Is Defined as Any Untoward Medical Occurrence That, at Any Dose:
Results in death
Is life threatening The term *life threatening* in the definition of *serious* refers to an event in which the participant was at risk of death at the time of the event. It does not refer to an event, which hypothetically might have caused death, if it were more severe.
Requires inpatient hospitalization or prolongation of existing hospitalization – In general, hospitalization signifies that the participant has been admitted at the hospital or emergency ward for observation and/or treatment that would not have been appropriate in the physician's office or outpatient setting. Complications that occur during hospitalization are AEs. If a complication prolongs hospitalization or fulfills any other seriousness criteria, the event will be considered serious. When in doubt as to whether hospitalization occurred or was necessary, the AE is considered serious. – Hospitalization for elective treatment of a preexisting condition that did not worsen from screening will not be collected as an AE.
Results in persistent or significant disability/incapacity – The term disability means a substantial disruption of a person's ability to conduct normal life functions. – This definition is not intended to include events of relatively minor medical significance such as uncomplicated headache, nausea, vomiting, diarrhea, influenza, and accidental trauma (eg, sprained ankle) that can interfere with or prevent everyday life functions but do not constitute a substantial disruption.
Is a congenital anomaly/birth defect
Other situations: – Medical or scientific judgment will be exercised by the investigator in deciding whether SAE reporting is appropriate in other situations such as significant medical events that could jeopardize the participant or require medical or surgical intervention to prevent 1 of the other outcomes listed in the above definition. These events are usually considered serious. – Examples of such events include invasive or malignant cancers, intensive treatment for allergic bronchospasm, blood dyscrasias, convulsions or development of intervention dependency or intervention abuse. – Suspected transmission of any infectious agent via the IMP will also be considered an SAE.

10.4.3. Recording and Follow-Up of AE and/or SAE

AE and SAE Recording
– When an AE/SAE occurs, it is the responsibility of the investigator to review all documentation (eg, hospital progress notes, laboratory reports, and diagnostics reports) related to the event. – The investigator will then record all relevant AE/SAE information. – It is **not** acceptable for the investigator to send photocopies of the participant's medical records in lieu of completion of the required form. – There can be instances when copies of medical records for certain cases are requested. In this case, all participant identifiers, with the exception of the participant number, will be redacted on the copies of the medical records before submission. – The investigator will attempt to establish a diagnosis of the event based on signs, symptoms, and/or other clinical information. Whenever possible, the diagnosis (not the individual signs/symptoms) will be documented as the AE/SAE.
Assessment of Severity
The investigator will assess intensity for each AE and SAE reported during the study. All AEs observed will be graded using the NCI CTCAE definitions version 5.0. The grade refers to the severity of the AE. If a particular AE's severity is not specifically graded by the guidance document, the investigator is to use the general NCI CTCAE definitions of grade 1 through grade 5 following his or her best medical judgment, using the following general guideline: – Grade 1: Mild; asymptomatic or mild symptoms; clinical or diagnostic observations only; intervention not indicated – Grade 2: Moderate; minimal, local or noninvasive intervention indicated; limiting age-appropriate instrumental ADL (eg, preparing meals, shopping for groceries or clothes, using the telephone) – Grade 3: Severe or medically significant but not immediately life-threatening; hospitalization or prolongation of hospitalization indicated; disabling; limiting self-care ADL (ie, bathing, dressing and undressing, feeding self, using the toilet, taking medications, and not bedridden) – Grade 4: Life-threatening consequences or urgent intervention indicated – Grade 5: Death related to AE NOTE: An AE that is assessed as severe may not necessarily meet the criteria for an SAE. Severe is a category used for rating the intensity of an event; and both AEs and SAEs can be assessed as severe. An event is defined as "serious" when it meets at least 1 of the predefined outcomes as described in the definition of an SAE, not when it is rated as severe. Grade 4 and 5 AEs are always assessed as serious (ie, SAE).

Assessment of Causality
– The investigator is obligated to assess the relationship between IMP and each occurrence of each AE/SAE as **related** or **not related**. The investigator will use clinical judgment to determine whether there is reasonable possibility that the IMP caused the AE. – A *reasonable possibility* of a relationship conveys that there are facts, evidence, and/or arguments to suggest a causal relationship, rather than a relationship cannot be ruled out.

- Alternative causes, such as underlying disease(s), concomitant therapy, and other risk factors, and the temporal relationship of the event to IMP administration, will be considered and investigated.
- **Related** means that the AE cannot be explained by the participant's medical condition, other therapies, or an accident. The temporal relationship between the AE and IMP administration is compelling and/or follows a known or suspected response pattern concerning that IMP.
- **Not related** means that the AE can be readily explained by other factors such as the participant's underlying medical condition, concomitant therapy, or accident. No plausible temporal or biologic relationship exists between the IMP and the AE.
- The investigator will also consult the IB and/or product information, for marketed products, in his/her assessment.
- For each AE/SAE, the investigator must document in the medical notes that they have reviewed the AE/SAE and have provided an assessment of causality.

There could be situations in which an SAE has occurred, and the investigator has minimal information to include in the initial report. However, it is very important that the investigator always assess causality for every event before the initial transmission of the SAE data.

- The investigator could change his/her opinion of causality in light of follow-up information and send an SAE follow-up report with the updated causality assessment.
- The causality assessment is 1 of the criteria used when determining regulatory reporting requirements.

Follow-up of AEs and SAEs
• The investigator is obligated to perform or arrange for the conduct of supplemental measurements and/or evaluations as medically indicated or as requested to elucidate the nature and/or causality of the AE or SAE as fully as possible. This could include additional laboratory tests or investigations, histopathological examinations, or consultation with other health care professionals. • If a participant dies during participation in the study or during a recognized follow-up period, the investigator will provide a copy of any postmortem findings including histopathology. • The investigator will submit updated SAE data within 24 hours of receipt of the information.

10.4.4. Reporting of SAEs and AESIs

SAE and AESI Reporting
• All SAEs and AESIs will be recorded on the AE form of the eCRF. SAEs will also be recorded on the paper SAE report form. • The investigator or designated site staff will ensure all entered data are consistent. • An alert email for the SAE and AESI reports on the eCRF will automatically be sent by email to the sponsor or designee's safety mailbox via the EDC system. • The paper SAE report form will be faxed or emailed to the sponsor's designee (refer to the Serious Adverse Event Reporting details on page 1 of this protocol).

10.5. Appendix 5: Inflammatory Neuropathy Cause and Treatment (INCAT) Disability Scale

Arm disability (05)	0 = No upper limb problems 1 = Symptoms, in 1 or both arms, not affecting the ability to perform any of the following functions: Doing all zips and buttons; washing or brushing hair; using a knife and fork together; handling small coins 2 = Symptoms, in 1 arm or both arms, affecting but not preventing any of the above-mentioned functions 3 = Symptoms, in 1 arm or both arms, preventing 1 or 2 of the above-mentioned functions 4 = Symptoms, in 1 arm or both arms, preventing 3 or all of the functions listed, but some purposeful movements still possible 5 = Inability to use either arm for any purposeful movement
Leg disability (05)	0 = Walking not affected 1 = Walking affected, but walks independently outdoors 2 = Usually uses unilateral support (stick, single crutch, 1 arm) to walk outdoors 3 = Usually uses bilateral support (sticks, crutches, 2 arms) to walk outdoors 4 = Usually uses wheelchair to travel outdoors, but able to stand and walk a few steps with help 5 = Restricted to wheelchair, unable to stand and walk a few steps with help
Overall disability = Arm disability + Leg disability	

The INCAT score is a 10-point scale that covers the functionality of legs and arms. Scores for arm disability range from 0 to 5, and scores for leg disability also range from 0 to 5.

The INCAT (total) score is the sum of these 2 scores and ranges from 0 to 10.

For the aINCAT, changes in the function of the upper limbs from 0 (normal) to 1 (minor symptoms) or from 1 to 0 will not be recorded as deterioration or improvement, because these changes are not considered clinically significant.

10.6. Appendix 6: Medical Research Council Scales

MRC scale[23]	
0	Complete paralysis
1	Minimal contraction
2	Active movement with gravity eliminated
3	Weak contraction against gravity
4	Active movement against gravity and resistance
5	Normal strength

The scale is bilaterally applied to 6 muscle groups of the upper and lower limbs to obtain a summed score ranging from 0 to 60 for the MRC scale: 1) arm abductors; 2) elbow flexors; 3) wrist extensors; 4) hip flexors; 5) knee extensors; and 6) foot dorsiflexors.

10.7. Appendix 7: Inflammatory Rasch-Built Overall Disability Scale (I-RODS)

Note that the following I-RODS scale is for reference only. At the start of the study, the current version of this scale will be used.

RODS for GBS – CIDP - MGUSP

INSTRUCTIONS: This is a questionnaire about the relationship between daily activities and your health. Your answers give information about how your polyneuropathy affects your daily and social activities and to what degree you are able to perform your usual activities.

Answer each question by marking the correct box ("x"). If you are not sure about your ability to perform a task, you are still requested to mark an answer that comes as close as possible to your judged ability to complete such a task. All questions should be completed. You can only choose one answer to each question. If you situation fluctuates, your answer should be based on how you *usually* perform the task.

If you need assistance or you are using special devices to perform the activity, you are requested to mark "possible, but with some difficulty ". In case you never perform the activity due to your polyneuropathy mark "not possible".

Are you able to Mark the best option with **"x"**

	Task	**Not possible to perform [0]**	**Possible, but with some difficulty [1]**	**Possible, without any difficulty [2]**
1.	read a newspaper/book?	☐	☐	☐
2.	eat?	☐	☐	☐
3.	brush your teeth?	☐	☐	☐
4.	wash upper body?	☐	☐	☐
5.	sit on a toilet?	☐	☐	☐
6.	make a sandwich?	☐	☐	☐
7.	dress upper body?	☐	☐	☐
8.	wash lower body?	☐	☐	☐
9.	move a chair?	☐	☐	☐
10.	turn a key in a lock?	☐	☐	☐
11.	go to the general practitioner?	☐	☐	☐
12.	take a shower?	☐	☐	☐
13.	do the dishes?	☐	☐	☐

14.	do the shopping?	☐	☐	☐
15.	catch an object (e.g., ball)?	☐	☐	☐
16.	bend and pick up an object?	☐	☐	☐
17.	walk one flight of stairs?	☐	☐	☐
18.	travel by public transportation?	☐	☐	☐
19.	walk and avoid obstacles?	☐	☐	☐
20.	walk outdoor < 1 km?	☐	☐	☐
21.	carry and put down a heavy object?	☐	☐	☐
22.	dance?	☐	☐	☐
23.	stand for hours?	☐	☐	☐
24.	run?	☐	☐	☐

10.8. Appendix 8: Possible Adaptations of Study Protocol During COVID-19 Pandemic

Introduction

The aim of ARGX-113-1902 is to assess the long-term safety and tolerability of efgartigimod PH20 SC in participants with CIDP. This treatment, which consists of efgartigimod for subcutaneous (SC) administration coformulated with recombinant human hyaluronidase PH20 (rHuPH20), could offer clinically significant benefits to patients with CIDP.

argenx has performed a critical assessment of the use of efgartigimod during the COVID-19 pandemic. After careful evaluation, the benefit-risk profile of efgartigimod use in clinical studies has not changed in the context of this pandemic. This decision was made based on efgartigimod's mechanism of action, the safety data generated to date, and provisions made in the clinical studies with efgartigimod for safety reporting and withholding treatment upon evidence of infection. This assessment will be reviewed regularly to consider new information about the pandemic and the ongoing, continuous assessment of adverse events reported during argenx clinical studies.

Based on this benefit-risk assessment, ARGX-113-1902 can be conducted; however, because of the pandemic, argenx acknowledges that considerable difficulties for participating sites to meet all previously planned assessments might occur (refer to SoA in Section 1.3). Therefore, an Appendix with possible adaptations to this study has been developed for sites to follow if a participant cannot complete a visit at the study site (refer to following details) and a visit at home or an alternative convenient location will be performed. This Appendix describes a minimum number of assessments required to guarantee the safety and well-being of the participants during the study and to secure the collection of the critical parameters for analysis. It remains at the investigator's discretion to assess if it is in the best interest of the participant to participate/continue in the study.

Note that a "home visit" as described in this Appendix could also be a visit at an alternative convenient location. A home visit can occur when the participant cannot come to the originally scheduled visit at the study site.

Note that the home nurse, who will go to the participant in case of a home visit, could also be another qualified person to perform all tasks (eg, a physician). In addition, a home nurse can be qualified staff from an alternative convenient location if the study visit cannot be performed at the study site.

Note that in between the originally planned visits at the site during the study, home visits are also possible for IMP administration by a home nurse. These home visits for IMP administration in between the originally planned site visits remain as described in the main protocol.

Implementation of this Appendix

- Implementation for all sites includes social distancing, personal protective equipment (PPE), and a telephone call before each study visit to check for COVID-19 symptoms.
- The adaptations to the visits and procedures described in this Appendix that are acceptable alternatives to the main protocol procedures should only be implemented in exceptional cases and after approval of the sponsor and/or CRO. Approval will be granted based on the possibility of the participant going to the site and per local and/or hospital regulations.

The Appendix is intended for countries and/or sites in geographical areas where COVID-19 has affected study sites' workload, severe movement restrictions have been imposed, or when there is a risk to participants/study staff if attending study sites for study visits. The initial duration of the implementation of this Appendix will be agreed upon and may be extended based on the local epidemic status.

When a (home) visit is performed under this COVID-19 Appendix, it should be documented as a COVID-19 (home) visit on the eCRF for the applicable visit.

Testing for COVID-19

Additional testing for COVID-19 beyond that mandated by relevant local authorities is not required at the start of the study. However, argenx recommends that participants who develop symptoms of COVID-19 during the study be tested.

Protecting Home Nurses and Site Staff From COVID-19

The home nurse, site staff and qualified staff from an alternative convenient location should apply appropriate social distancing and use PPE according to the local hospital and governmental regulations/recommendations; refer to the following details.

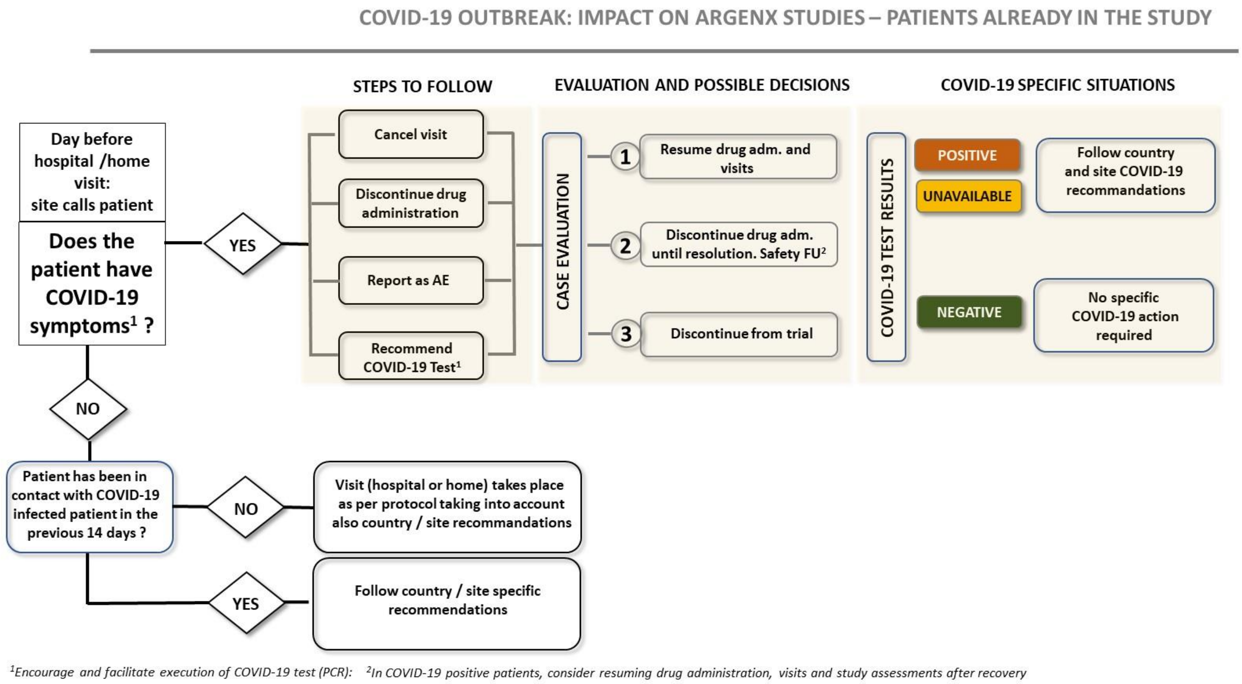
Participants With COVID-19 (Either a Positive Test or With Symptoms [Suspicion of COVID-19 Infection])

Participants with a COVID-19 infection should not enter the study. In case a participant develops a COVID-19 infection when he/she is in the study, the following applies.

The instructions to manage an infection that are in the main protocol are also applicable in case of an infection with COVID-19 (ie, it will be considered an AESI similar to all infections). Treatment should be interrupted until the participant is considered recovered from COVID-19 (according to the local recommendations).

During the pandemic, the study site staff should contact participants before each visit to inquire about COVID-19 symptoms and exposure using the following flow chart and make a decision on proceeding or postponing the visit according to the flow chart.

The following symptoms should be specifically addressed during that phone call: fever, cough, sneezing, loss of taste/smell, and difficulty breathing/chest tightness.



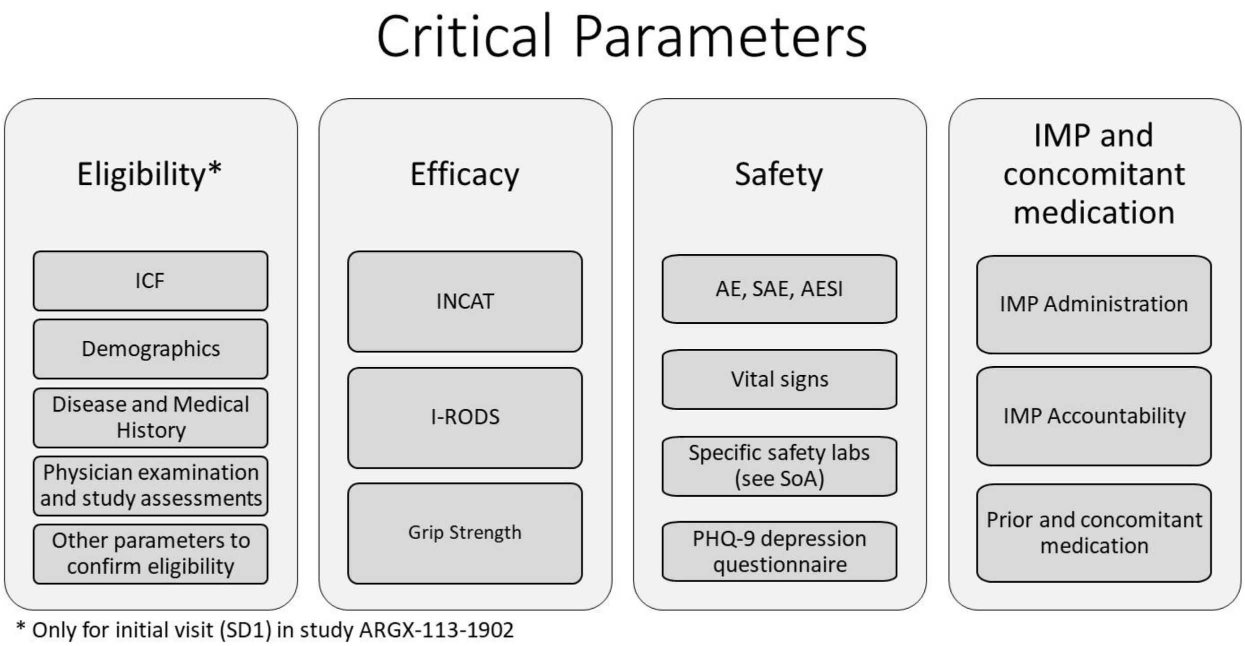
Participants with symptoms of COVID-19 infection will be treated for this infection as guided by the local health care system. Participants will be monitored by the study site by telephone contact. If the participant has to be in self-isolation for a period longer than 2 weeks, it will be discussed on a case-by-case basis with the sponsor and/or the CRO whether the participant can remain in the study.

Once a participant has recovered, the participant can continue the study and receive study medication, provided that he/she has not received/is not receiving prohibited medication (Section 6.4.2).

Critical Parameters to Be Collected During the Study

If feasible, all assessments, including those not critical (ie, indicated as assessments not mandatory in the SoA in Section 10.8.1), should be performed.

The critical parameters that are mandatory to be collected at the originally scheduled site visits (which may become home visits* during the study) are summarized as follows:



During the COVID-19 pandemic, the collection of these critical parameters is specified in the SoA of the study in Section 10.8.1 (indicated in red solid dots).

- The evaluation of eligibility for the study will be as described in the main protocol. The rollover visit (SD1) must be performed at the study site (refer to following sections).
- For the evaluation of efficacy and safety and the IMP administration and accountability, certain modifications are allowed in this Appendix compared to the main protocol (refer to following sections).

Mandatory Site Visit and Allowed Home Visits

To collect the critical parameters of the study, the rollover visit (SD1) has to be performed at the site and other visits may be at home*:

- Rollover visit (SD1)

* Note that a "home visit" as described in this Appendix could also be a visit at an alternative convenient location.

The SD1 has to be performed at the study site. If it is not possible to go to the study site for this visit due to the COVID-19 situation, the sponsor or CRO has to be contacted as soon as possible to discuss viable options.

- Other Study Visits (Originally Planned Site Visits)

All study visits, other than the rollover visit, that were originally planned to be site visits may be performed at home (or at an alternative convenient location) during the COVID-19 pandemic. A home nurse will travel to the participant’s home to conduct this visit (or the participant will go to an alternative convenient location). The investigator will talk to the participant via an audio or video interview to elicit AE, concomitant medications, and general well-being of the participant. The investigator will perform efficacy assessments using a video interview (or via a telephone only if a video interview is not possible) with the participant. The assessments via an audio or video interview will be conducted before the home nurse administers the study medication. The division of tasks between the investigator and home nurse are indicated in the following scheme.

Note that in between the originally planned visits at the site during the study, home visits are also possible for IMP administration by a home nurse. Home visits for IMP administration in between the originally planned site visits remain as described in the main protocol.

Scheme for home visits		
Critical assessments	**Performed by**	**Method of assessment**
• ICF addendum (not initial ICF at screening, explained to participant) • Adverse events • Concomitant medication • Confirmation that IMP can be administered	Investigator	Audio or video interview
• aINCAT • Grip strength		Video interview (or via telephone if video interview is not possible)
• Bring device for efficacy assessments; including I-RODS and the participants reported outcomes questionnaire PHQ-9 • Vital signs • Blood draw • Urine pregnancy test • IMP administration	Home nurse	In person at participant’s home (A ”home visit” could also be a visit at an alternative convenient location.)

Efficacy Assessments

The 3 critical efficacy assessments of the study are:

- aINCAT
- I-RODS

- grip strength

During home visits (replacing the originally planned site visits in case a participant cannot come to the site due to the COVID-19 pandemic), 2 similar devices will be used to capture these assessments. One device will remain at the site with the evaluating physician and the other device will be taken by the home nurse to the participant. The investigator will perform the aINCAT and grip strength assessments via video link between the 2 devices (or via a telephone only if a video interview is not possible). The home nurse will provide the other device to the participant to perform the I-RODS assessment.

MRC sum score and Time Up and Go (TUG) are considered noncritical variables and will not be measured at home visits.

Patient-Reported Outcomes (PROs)

- During a home visit, PROs will be performed by the participant on the device provided by the home nurse.
- The PHQ-9 depression questionnaire (suicidality question) needs to be conducted at all scheduled visits, also if this visit is performed at the participant's home (or at an alternative convenient location).
- Other PROs are not considered critical data and may be performed only if feasible during a home visit (or during a visit at an alternative convenient location).

Safety Assessments

- Adverse event

During home visits, a telephone/video interview about possible AEs will be conducted by the investigator or designee. Any suspected COVID-19 infection relevant signs or symptoms (including any abnormal laboratory findings) will be reported as an AE, with clear distinction whether it is a confirmed test positive COVID-19 infection or suspected infection if not tested.

- Vital signs

During home visits, vital signs (heart rate, body temperature, and blood pressure) will be measured by the home nurse at the beginning of the home visit and the results will be communicated to the investigator. If pyrexia is detected, the home nurse should call the study site immediately and await further instructions from the investigator.

- Safety laboratory testing

During home visits, the home nurse will take blood samples for safety laboratory testing immediately before IMP administration. Ideally, tubes from the central laboratory will be used. However, if needed, local laboratory tubes and venipuncture equipment may be used after prior agreement with the central laboratory.

During home visits, urine samples will be taken for safety laboratory testing and pregnancy testing (if applicable).

In case the transfer of laboratory samples to the central laboratory is not available or may be prolonged, a local laboratory may be used. A certification of the local laboratory is required.

Laboratory reports should be sent to and reviewed by the investigator. Any abnormalities will be evaluated for clinical significance, and any significant laboratory findings should be recorded as AEs. The laboratory report should be kept as a source document.

- ECG

At the rollover visit (SD1), an ECG is required. A local ECG machine from the site may be used in case equipment from the central ECG laboratory does not arrive at the site.

Blood Sampling for Tests Other Than Safety Laboratory Testing

During home visits, the home nurse will draw the blood samples for laboratory parameter evaluation. Blood sampling will be performed immediately before IMP administration.

Concomitant Medication

At the time of the home visit, the investigator or designee will telephone/video interview the participant about concomitant medications. It will be documented whether concomitant medication is taken for a confirmed test positive COVID-19 infection or suspected infection (if not tested).

IMP Administration

As of protocol v8.0, the source of IMP will change from a single-dose vial to a single-dose PFS. Before receiving the first PFS dose, participants will be asked to sign the current ICF version giving their consent to continue the study. Site staff, the participant, the caregiver, a home nurse, or a concierge service can administer IMP. The first 3 consecutive IMP injections via PFS must be administered at the study site with safety monitoring by a health care provider. If home nursing care is already in place, the third consecutive administration may occur at the participant's home monitored by a home nurse.

To be considered competent, a participant or caregiver will (self-)administer IMP under the supervision of an investigator or study nurse for at least 1 injection. After the participant or caregiver is considered competent, the (self-)administration can continue, and supervision may no longer be needed at the discretion of the investigator.

Note that IMP administration in between originally planned visits at the site can remain as described in the main protocol (ie, [supervised] self-administration, caregiver-supported administration, or via a home nurse or concierge service at the study site).

Study Endpoint

The criteria for the study endpoint for a participant described in the main protocol also apply to this Appendix.

10.8.1. Schedules of Activities (SoA) During COVID-19 Pandemic

In the SoA of ARGX-113-1902 (Table 5), the mandatory assessments are indicated in red solid dots. The assessments that must be performed only if feasible during the COVID-19 pandemic are indicated in black open circles. At site visits, all assessments must be performed (critical [red solid dots] and noncritical [black open circles]). At home visits (or visits at an alternative convenient location), assessments may be limited to the mandatory assessments (critical [red solid dots]).

Table 5: Schedule of Activities for the First 48-Week Treatment Period During COVID-19 Pandemic (NO LONGER APPLICABLE)

	Rollover[a]	Treatment period 1[a]					Unscheduled	Safety Follow-up[d]
	V1	V2	V3	V4	V5	V6		28 (±3) days after last IMP dose
	SD1[b]	W4	W12	W24	W36	W48/ED[a]		
Assessment/Procedure	±0 days	±2 days				−2 to +14 days	UnschV[c]	
Informed consent	●					●[e]		
Demographics	●							
Inclusion/exclusion criteria	●					●[e]		
Medical history	●							
12-lead ECGs	●	O	O	O	O	O	O	O
Physical examination[f]	●	O	O	O	O	O	O	O
Vital signs[g]	●	●	●	●	●	●	●	●
Pregnancy test (urine)	●		●	●	●	●	●	●
aINCAT	●	●	●	●	●	●	●	
MRC sum score	●	O	O	O	O	O	O	
I-RODS score	●	●	●	●	●	●	●	
Mean grip strength	●	●	●	●	●	●	●	
TUG test	●	O	O	O	O	O	O	
EQ-5D-5L	●		O	O	O	O	O	

	Rollover[a]	Treatment period 1[a]					Unscheduled	Safety Follow-up[d]
	V1	V2	V3	V4	V5	V6		28 (±3) days after last IMP dose
	SD1[b]	W4	W12	W24	W36	W48/ED[a]		
Assessment/Procedure	±0 days	±2 days				−2 to +14 days	UnschV[c]	
BPI-SF	●		O	O	O	O	O	
TSQM-9	●		O	O	O	O	O	
RT-FSS	●		O	O	O	O	O	
HADS	●		O	O	O	O	O	
Suicidality assessment[h]	●	●	●	●	●	●	●	●
Blood and urine clinical laboratory safety tests	●	●	●	●	●	●	●	●
Blood sampling for immunogenicity analysis[i]	●	O	O	O	O	O	O	O
Blood sampling for PK and PD analysis[j]	●	O	O	O	O	O	O	O
Blood sample for potential future biomarker analysis[k]	●		O	O	O	O	O	
IMP (self-)administration training[l]	●	O	O	O	O			
IMP (self-)administration (SC)[m]	●[m]	●	●	●	●[n]	●[e]		
IMP dispensation[o]	●	●	●	●	●[n]	●[e]		
IMP accountability[p]	●	●	●	●	●	●		
Adverse events[q]	‹-----------●-----------›							
Concomitant therapies[q,r]	‹-----------●-----------›							

Footnotes are on the next page

ADA=antidrug antibodies; AE=adverse event; BPI-SF=Brief Pain Inventory-Short Form; ECG=electrocardiogram; ED=early discontinuation; HADS=Hospital Anxiety and Depression Scale; ICF=informed consent form; IgG=immunoglobulin γ; IMP=investigational medical product; aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment (INCAT) disability score; I-RODS=Inflammatory-Rasch-built Overall Disability Scale; OLE=open-label extension; PD=pharmacodynamic(s); PK=pharmacokinetic(s); RT-FSS=Rasch-Transformed Fatigue Severity Scale; MRC=Medical Research Council; SAE=serious adverse event; SC=subcutaneous(ly); SD1=study day 1; TSQM-9=9-item Treatment Satisfaction Questionnaire for Medication; TUG=Timed Up and Go; Unsch=unscheduled; V=visit; W=week

Notes: In addition to the visits in this SoA, there are "IMP collection visits" as described in Section 1.3.1.

Refer to Section 10.9.4 for the SoA of the optional substudy to explore less frequent dosing. In case a participant will participate in the optional substudy, the last visit in the main part of study ARGX-113-1902 will coincide with the first visit of the substudy.

a. This visit is only for the first treatment period. Once entry criteria are confirmed, this visit will preferably be performed on the same day of the week-48 visit or ED visit of Stage B of study ARGX-113-1802.
For each next treatment period, the first day of a new treatment period will coincide with the week-48 visit of the previous treatment period. In case a next treatment period is started, all assessments at the week-48 visit are mandatory.
b. W48/ED visit of a previous treatment period will coincide with the SD visit of the next treatment period.
c. The investigator can decide which of the assessments must be performed at each unscheduled visit (Section 4.1.7).
d. The safety follow-up visit is only applicable for participants who do not start a new treatment period and for participants who prematurely discontinued the study in case IMP was stopped less than 28 days before the last study visit.
e. This requirement is only needed for participants who will start a new 48-week treatment period. The first day of a new treatment period will coincide with the week-48 visit of the previous treatment period.
f. Assessment includes physical examination and weight. Weight will be measured at the SD1 visit and during the study in case of an obvious weight change.
g. Assessment includes semisupine blood pressure and heart rate, and body temperature.
h. Suicidality will be assessed by specifically answering 1 question from the Patient Health Questionnaire-9 (PHQ-9) depression questionnaire (Section 8.13).
i. Blood samples for immunogenicity testing will be taken predose at the study visits to measure ADA directed toward efgartigimod (measured in serum). Plasma samples to determine immunogenicity against rHuPH20 plasma will be collected and stored, and will be analyzed in case of safety concerns.
j. For PK, predose (within 2 hours before start of IMP administration at the study visits) samples will be taken. At these time points, blood samples will also be taken for PD analysis. PD analysis includes the measurement of total IgG.
k. Optional blood samples will be taken for potential future biomarker analysis.
l. Training for self-administration or caregiver-supported administration will be provided on SD1 of the first treatment period and later during the study for participants or caregivers who need this until they are capable to administer IMP.
m. IMP will be administered at the site at the scheduled study visits (which could be replaced by a visit from a home nurse if the participant cannot come to the site due to the COVID-19 pandemic). The first IMP administration in ARGX-113-1902 may be within 24 hours after eligibility to roll over from ARGX-113-1802 to ARGX-113-1902. IMP administration will be after blood samples have been taken for laboratory safety, PK, PD, immunogenicity, and/or optional biomarker analyses. Other IMP administrations (between the scheduled visits at the site) will be administered by the participant (self-administration), the participant's caregiver, or via a home nurse or concierge service for visit at the study site.
n. The last planned IMP (self-)administration of each treatment period is at week 47.
o. At weeks 6, 18, 30, and 42 (ie, "IMP collection visits," Section 1.3.1), participants must come to the site (or another location as agreed upon with the site/home nurse) to collect IMP. Except for IMP dispensation and possibly also IMP administration, no other study assessments are planned at these IMP collection visits.
p. IMP kits will be returned to the site at the next site visit during which IMP accountability will be performed when the kits are available.
q. AEs and intake of concomitant medication(s) will be monitored continuously from signing the ICF until the last study-related activity. In case of early discontinuation, any AEs/SAEs should be assessed for 30 days after the ED visit or until satisfactory resolution or stabilization.
r. Any vaccination received during the study (from screening onward) will be entered as concomitant medication.

Note: For participants who provide separate consent, the titers of produced vaccination antibodies will be measured using retained blood samples (leftover blood samples taken for other tests) during the study.

Table 6: Schedule of Activities for Subsequent Treatment Periods After the First 48-Week Treatment Period During COVID-19 Pandemic

Assessment/Procedure	Treatment period n[a]					Unscheduled	Safety Follow-up[e]
	Baseline Cycle n[b]	Cycle n W12	Cycle n W24	Cycle n W36	Cycle n W48/ED[c]		
	±0 days	±2 days			−2 to +14 days	UnschV[d]	28 (±3) days after last IMP dose
Informed consent[f]	●						
Inclusion/exclusion criteria	●				●[e]		
Physical examination[g]	●	O	O	O	O	O	O
Vital signs[h]	●	●	●	●	●	●	●
Pregnancy test (urine)	●	●	●	●	●	●	●
aINCAT	●	●	●	●	●	●	
MRC sum score	●	O	O	O	O	O	
I-RODS score	●	●	●	●	●	●	
Mean grip strength	●	●	●	●	●	●	
TUG test	●	O	O	O	O	O	
EQ-5D-5L	●	O	O	O	O	O	
BPI-SF	●	O	O	O	O	O	
TSQM-9	●	O	O	O	O	O	
RT-FSS	●	O	O	O	O	O	
HADS	●	O	O	O	O	O	
Suicidality assessment[i]	●	●	●	●	●	●	●
Blood and urine clinical laboratory safety tests	●	●	●	●	●	●	●
Blood sampling for immunogenicity analysis[j]	●	O	O	O	O	O	O
Blood sampling for PD analysis[k]	●	O	O	O	O	O	O
IMP (self-)administration training[l]	●	O	O	O			
IMP (supervised) (self-)administration (SC)[l,m]	●	●	●	●[o]	●[e]		
IMP dispensation[n]	●	●	●	●[o]	●[e]		
IMP accountability	●	●	●	●	●		

Assessment/Procedure	Treatment period n[a]					Unscheduled	Safety Follow-up[e]
	Baseline Cycle n[b]	Cycle n W12	Cycle n W24	Cycle n W36	Cycle n W48/ED[c]	UnschV[d]	28 (±3) days after last IMP dose
	±0 days	±2 days			−2 to +14 days		
Adverse events[p]	←-------------------- ● --------------------→						
Concomitant therapies[p,q]	←-------------------- ● --------------------→						

aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment (INCAT) disability score; ADA=antidrug antibody(ies); AE=adverse event; BPI-SF=Brief Pain Inventory-Short Form; cycle=48-week treatment period; ED=early discontinuation; HADS=Hospital Anxiety and Depression Scale; ICF=informed consent form; IgG=immunoglobulin γ; IMP=investigational medical product; I-RODS=Inflammatory-Rasch-built Overall Disability Scale; MRC=Medical Research Council; n=treatment period number; OLE=open-label extension; PD=pharmacodynamic(s); PFS=prefilled syringe; rHuPH20=recombinant human hyaluronidase PH20; RT-FSS=Rasch-Transformed Fatigue Severity Scale; SAE=serious adverse event; SC=subcutaneous(ly); TSQM-9=9-item Treatment Satisfaction Questionnaire for Medication; TUG=Timed Up and Go; Unsch=unscheduled; V=visit; W=week

Notes: In addition to the visits in this SoA, there are "IMP collection visits" as described in Section 1.3.1.

Refer to Section 10.9.4 for the SoA of the optional substudy to explore less frequent dosing. In case a participant will participate in the optional substudy, the last visit in the main part of study ARGX-113-1902 will coincide with the first visit of the substudy.

a. Participants will return to the site for site visits at 12-week intervals; and in between these site visit, at 6-week intervals to collect IMP (ie, at weeks 6, 18, 30, and 42); refer to footnote n)
b. The W48 visit of a previous treatment period will coincide with the baseline visit of the next treatment period.
c. Participants can remain in the study until 2 years after marketing authorization in their country, until efgartigimod PH20 SC becomes commercially available or available via the continued access program for CIDP, or until 30 Apr 2027, whichever comes first.
d. The investigator can decide which of the assessments must be performed at each unscheduled visit (Section 4.1.7).
e. The safety follow-up visit is only applicable for participants who do not start a new treatment period and for participants who prematurely discontinue the study in case IMP was stopped less than 28 days before the last study visit.
f. Participants will be asked to sign the current ICF version before receiving the first PFS dose and before starting a new 48-week treatment period to continue the study. The first day of a new treatment period will coincide with the week-48 visit of the previous treatment period.
g. Assessment includes physical examination and weight. Weight will be measured in case of an obvious weight change.
h. Assessment includes semisupine blood pressure and heart rate, and body temperature.
i. Suicidality will be assessed by specifically answering 1 question from the Patient Health Questionnaire-9 (PHQ-9) depression questionnaire (Section 8.13).
j. Additionally, blood will be collected, stored, and analyzed to evaluate immunogenicity against rHuPH20 (measured in plasma) and will be analyzed if there are safety concerns.
k. For PD, predose (before IMP administration at the study visits) samples will be taken. PD analysis includes the measurement of total IgG.
l. Training for self-administration or caregiver-supported administration will be provided as needed until they are capable to administer IMP.
m. IMP will be administered via PFS; further information is provided in Section 6.2.2. IMP administration will occur after blood samples have been taken for laboratory safety, PD, and/or immunogenicity analyses. After the participant or caregiver is considered competent, the (self-)administration can continue, and supervision may no longer be needed at the discretion of the investigator.
n. At weeks 6, 18, 30, and 42 (ie, "IMP collection visits," Section 1.3.1), participants must come to the site (or another location as agreed upon with the site/home nurse) to collect IMP. The site staff will provide "PFS Instructions for Use" to the participant and caregiver at the first supervised PFS IMP administration visit. Except for IMP dispensation and possibly also IMP administration, no other study assessments are planned at these IMP collection visits.
o. The last planned IMP (self-)administration of each treatment period is at week 47.

p. AEs and intake of concomitant medication(s) will be monitored continuously from signing the ICF until the last study-related activity. In case of early discontinuation, any AEs/SAEs should be assessed for 30 days after the ED visit or until satisfactory resolution or stabilization.

q. Any vaccination received during the study (from screening onward) will be entered as concomitant medication.

Note: For participants who provide separate consent, the titers of produced vaccination antibodies will be measured using retained blood samples (leftover blood samples taken for other tests) during the study.

10.9. Appendix 9: Optional Substudy to Explore Less Frequent Dosing

10.9.1. Definitions of Terms

Term	Definition
Stable clinical condition (for participants who enroll in the optional substudy to explore less frequent dosing)	Participant's aINCAT neither worsens nor improves between the last 2 scheduled visits (ie, approximately 12 weeks).
Clinical deterioration (during the optional substudy to explore less frequent dosing)	Participant's aINCAT increases by ≥1 point (worsening) compared to the start of a specific reduced dosing frequency.
Restabilization of CIDP condition (for participants whose condition deteriorates while receiving efgartigimod PH20 SC once every 2 or 3 weeks)	• Participant receives at least 4 once-weekly efgartigimod PH20 SC doses until a scheduled site visit AND • At that scheduled visit, participant's aINCAT is equal to or lower than the one recorded at the start of a specific reduced dosing frequency AND • At the investigator's discretion.

10.9.2. Description of the Optional Substudy

For participants in the optional substudy, the last visit in ARGX-113-1902 will coincide with the substudy's first visit. The participant will be asked to sign the ICF for the optional substudy, appended to the main study's ICF, before any substudy-specific assessment is performed.

Participants with stable clinical condition (between the last 2 scheduled visits) may receive efgartigimod PH20 SC 1000 mg once every 2 weeks if they:

- Received at least 48 once-weekly doses of efgartigimod PH20 SC in ARGX-113-1902 **before** the database lock of the first interim analysis (Section 9.5.6) (**NO LONGER APPLICABLE because the first interim analysis has been performed).**
- Received at least 24 once-weekly doses of efgartigimod PH20 SC in ARGX-113-1902 **after** the database lock of the first interim analysis.

Participants may receive efgartigimod PH20 SC once every 3 weeks at the investigator's discretion if they show no clinical deterioration for at least 24 weeks while receiving doses once every 2 weeks.

Participants should arrange a site visit to assess aINCAT if clinical deterioration occurs. Participants receiving efgartigimod PH20 SC once every 2 or 3 weeks will receive once-weekly doses until their CIDP condition restabilizes (Section 10.9.1), after which participants may receive efgartigimod PH20 SC once every 2 weeks. Participants may receive efgartigimod PH20 SC once every 3 weeks if they show no clinical deterioration for at least 24 weeks while receiving doses once every 2 weeks.

Participants may receive a more frequent dosing regimen at any time, regardless of clinical deterioration. The reason for transitioning to a more frequent dosing regimen will be documented.

Participants may start a new treatment year after completing the previous 48-week treatment year. Participants with no clinical deterioration may continue the dosing frequency of efgartigimod PH20 SC they had at the end of the previous treatment year or transition to a more frequent dosing regimen at the investigator's discretion.

The SoA during the optional substudy is provided in Section 10.9.4. The assessments and procedures mentioned in the SoA for the optional substudy are described in Section 8. Furthermore, detailed instructions for IMP administration are described in the pharmacy manual. In addition, for this optional substudy, the same rules for IMP administration (Section 6.2.2), discontinuation of IMP, and study withdrawal (Section 7) apply as for the main study. Finally, supporting documentation and operational considerations, as described in Section 10 (Appendices 1 through 8), are also applicable to this substudy.

Objectives of the Optional Substudy

Primary Objective

- To assess the percentage of participants maintaining their CIDP disease control (ie, without clinical deterioration) at week 24 while receiving efgartigimod PH20 SC once every 2 weeks.

Secondary Objectives

- To assess the average number of doses administered per 24-week period.
- To assess the percentage of participants with clinical deterioration while receiving efgartigimod PH20 SC once every 2 weeks who can be restabilized on once-weekly injections of efgartigimod PH20 SC.
- To assess the percentage of participants without clinical deterioration after receiving efgartigimod PH20 SC once every 3 weeks for 24 weeks.
- To assess the long-term safety and tolerability of efgartigimod PH20 SC administered once every 2 or 3 weeks.
- To evaluate additional PROs (including patient-reported quality of life and treatment satisfaction) of participants receiving efgartigimod PH20 SC administered once every 2 or 3 weeks.
- To evaluate the PD effect of efgartigimod PH20 SC (ie, total IgG levels) administered once every 2 or 3 weeks.
- To explore self-administration of efgartigimod PH20 SC once every 2 or 3 weeks.
- To explore caregiver-supported administration of efgartigimod PH20 SC once every 2 or 3 weeks.

Rationale for the Optional Substudy

The once-weekly dose of efgartigimod PH20 SC 1000 mg provides a near-maximal reduction of IgG levels and is, therefore, expected to maximize the chance of a clinical response. However, the correlation between IgG reduction and clinical efficacy in CIDP is currently unknown. To provide a less frequent dosing regimen option to the participants, this optional substudy will explore whether participants who are well-controlled (stable clinical status) while receiving once-weekly doses of efgartigimod PH20 SC in ARGX-113-1802 and ARGX-113-1902, will remain well-controlled while receiving doses once every 2 or 3 weeks.

Statistical Analysis of the Optional Substudy

The data from the optional substudy will be analyzed and reported separately from those of the main ARGX-113-1902. The planned analysis of the data obtained in the substudy will be described in a separate statistical analysis plan.

Possible Adaptations to the Optional Substudy During the COVID-19 Pandemic

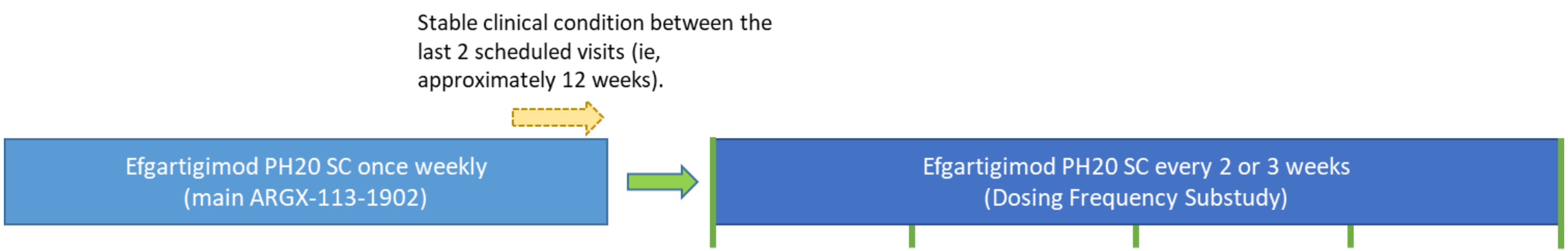
The same principles for possible adaptations during the COVID-19 pandemic apply to this optional substudy, as explained in Section 10.8 (Appendix 8) for the main ARGX-113-1902.

This implies that baseline visits must be performed on-site, during which all listed assessments for that visit must be performed. In addition, when a participant may transition to a reduced dosing frequency, the first visit (ie, the baseline of this reduced dosing frequency) also must be performed on-site. All other study visits that were originally planned to be site visits may be performed at home (or at an alternative convenient location) during the COVID-19 pandemic. The list of critical assessments, as described in Section 10.8, also applies to the substudy. If feasible, all assessments, including those not critical (ie, indicated as assessments not mandatory in the SoA in Section 10.8.1), should be performed. In case a participant has clinical deterioration while receiving efgartigimod PH20 SC once every 2 or 3 weeks, the participant will receive once-weekly doses of efgartigimod PH20 SC and a site visit is not mandatory.

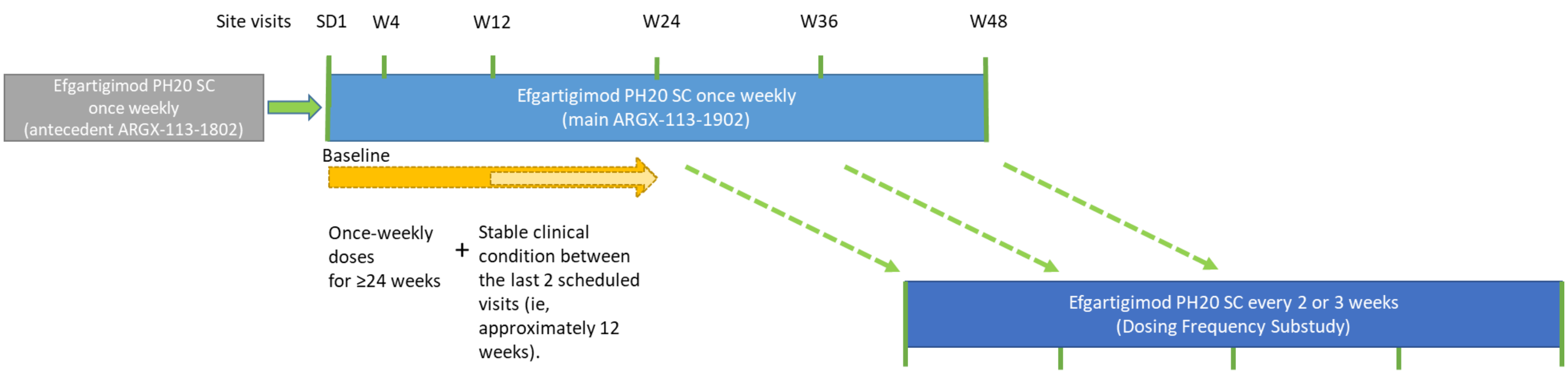
10.9.3. Schema of the Optional Substudy

Figure 2: Rollover to the Optional Substudy to Explore Less Frequent Dosing

A: (NO LONGER APPLICABLE because the first interim analysis has been performed) Received at least 48 once-weekly doses of efgartigimod PH20 SC in ARGX-113-1902 before the database lock of the first interim analysis + a stable clinical condition:

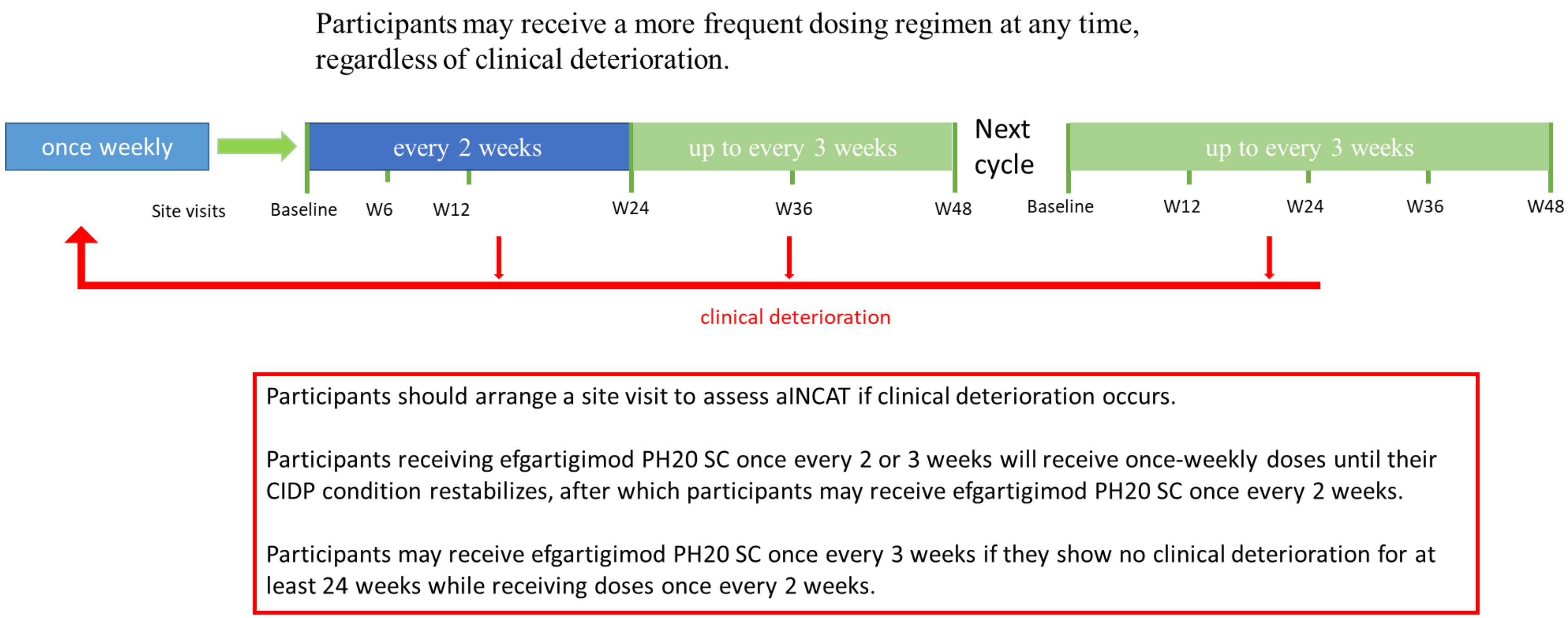


B: Received at least 24 once-weekly doses of efgartigimod PH20 SC in ARGX-113-1902 after the database lock of the first interim analysis + a stable clinical condition:



aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment (INCAT) disability score; efgartigimod PH20 SC=efgartigimod for SC administration coformulated with rHuPH20; rHuPH20= recombinant human hyaluronidase PH20; SC=subcutaneous(ly); SD1=study day 1; W=week
Note: A stable clinical condition (for participants who enroll in the optional substudy to explore less frequent dosing) is defined as a situation in which a participant's aINCAT neither worsens nor improves between the last 2 scheduled visits (ie, approximately 12 weeks).

Figure 3: Schema of the Optional Substudy to Explore Less Frequent Dosing



aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment (INCAT) disability score; cycle=48-week treatment period; efgartigimod PH20 SC=efgartigimod for SC administration coformulated with rHuPH20; rHuPH20= recombinant human hyaluronidase PH20; SC=subcutaneous(ly); W=week
Notes:

- Clinical deterioration (during the optional substudy to explore less frequent dosing) is defined as a situation in which a participant's aINCAT increases by ≥1 point (worsening) compared to the start of a specific reduced dosing frequency.
- Restabilization of CIDP condition (for participants whose condition deteriorates while receiving efgartigimod PH20 SC once every 2 or 3 weeks) is defined as a situation in which:
  - Participant receives at least 4 once-weekly efgartigimod PH20 SC doses until a scheduled site visit AND
  - At that scheduled visit, participant's aINCAT is equal to or lower than the one recorded at the start of a specific reduced dosing frequency AND
  - At the investigator's discretion.
- Site visits occur every 12 weeks, except for visit 2 at week 6; IMP collection visits in the optional substudy (Section 10.9.5) occur at weeks 18, 30, 42, etc (every 6 weeks after the prior site visit); participant may request an unscheduled visit to assess clinical deterioration.

10.9.4. Schedule of Activities (SoA) During the Optional Substudy

Table 7: Schedule of Activities During the Optional Substudy to Explore Less Frequent Dosing

		Rollover[a]	Every 2 weeks treatment period[b]			Every 2 or 3 weeks (maximum) Treatment period[b]		New treatment year			Early discontinuation[c]	Unsche-duled[d]	Safety Follow-up[e]
		V1	V2	V3	V4	V5	V6	V7	V8	Vn			28 (±3) days after last IMP dose
		SD1	W6	W12	W24	W36	W48	W60	W72	W((n-2)×12)	ED	UnschV	
Assessment/Procedure		±0 days	±2 days								−2 to +14 days		
Informed consent[f]		●											
Eligibility review		●											
Physical examination and vital signs[g]		●		●	●	●	●	●	●	●	●	●	●
Pregnancy test (urine)		●		●	●	●	●	●	●	●	●	●	●
aINCAT		●	●	●	●	●	●	●	●	●	●	●	
MRC sum score		●	●	●	●	●	●	●	●	●	●	●	
I-RODS score		●	●	●	●	●	●	●	●	●	●	●	
Mean grip strength		●	●	●	●	●	●	●	●	●	●	●	
TUG test		●	●	●	●	●	●	●	●	●	●	●	
EQ-5D-5L		●	●	●	●	●	●	●	●	●	●	●	
BPI-SF		●	●	●	●	●	●	●	●	●	●	●	
TSQM-9		●		●	●	●	●	●	●	●	●	●	
RT-FSS		●	●	●	●	●	●	●	●	●	●	●	
HADS		●		●	●	●	●	●	●	●	●	●	
Suicidality assessment[h]		●	●	●	●	●	●	●	●	●	●	●	●

		Rollover[a]	Every 2 weeks treatment period[b]			Every 2 or 3 weeks (maximum) Treatment period[b]		New treatment year			Early discontinuation[c]	Unsche-duled[d]	Safety Follow-up[e]
		V1	V2	V3	V4	V5	V6	V7	V8	Vn			28 (±3) days after last IMP dose
		SD1	W6	W12	W24	W36	W48	W60	W72	W((n-2)×12)	ED	UnschV	
Assessment/Procedure		±0 days	±2 days								−2 to +14 days		
Blood and urine clinical laboratory safety tests		●		●	●	●	●	●	●	●	●	●	●
Blood sampling for immunogenicity analysis[i]		●	●	●	●	●	●	●	●	●	●	●	●
Blood sampling for PD analysis[j]		●	●	●	●	●	●	●	●	●	●	●	●
IMP (self-) administration training (optional)		●	●	●	●	●	●	●	●	●		(●)	
IMP (supervised) (self-) administration (SC)[k]		●[k]	●	●	●	●	●[l]	●	●[l]	●[m]		(●)	
IMP dispensation		●	●	●	●	●	●[l]	●	●[l]	●[m]		(●)	
IMP accountability		●	●	●	●	●	●	●	●	●	●		
Adverse events[n]		‹--------------------●--------------------›											
Concomitant therapies[n,o]		‹--------------------●--------------------›											

aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment (INCAT) disability score; AE=adverse event; BPI-SF=Brief Pain Inventory-Short Form; ED=early discontinuation; HADS=Hospital Anxiety and Depression Scale; ICF=informed consent form; IgG=immunoglobulin γ; IMP=investigational medical product; I-RODS=Inflammatory-Rasch-built Overall Disability Scale; MRC=Medical Research Council; PD=pharmacodynamic(s); PFS=prefilled syringe; RT-FSS=Rasch-Transformed Fatigue Severity Scale; SAE=serious adverse event; SC=subcutaneous(ly); SD1=study day 1; TSQM-9=9-item Treatment Satisfaction Questionnaire for Medication; TUG=Timed Up and Go; Unsch=unscheduled; V=visit; W=week

Notes: In addition to the visits in this SoA, there are “IMP collection visits in the optional substudy” as described in Section 10.9.5.

a. This visit is only for the first treatment year of the optional substudy. In case a participant will participate in the optional substudy, the last visit in the main study ARGX-113-1902 will coincide with the first visit (V1) of the substudy.

b. Note that in case of clinical deterioration (ie, participant's aINCAT increases by ≥1 point [worsening] compared to the start of a specific reduced dosing frequency), participant receiving efgartigimod PH20 SC every 2 or 3 weeks will receive once-weekly doses until their CIDP condition restabilizes, after which participants may receive efgartigimod PH20 SC once every 2 weeks. Participants may receive efgartigimod PH20 SC once every 3 weeks if they show no clinical deterioration for at least 24 weeks while receiving doses once every 2 weeks; refer to Section 10.9.2 and Section 10.9.3.

c. Participants can remain in the study until 2 years after marketing authorization in their country, until efgartigimod PH20 SC becomes commercially available or available via the continued access program for CIDP, or until 30 Apr 2027, whichever comes first.

d. The investigator may decide which of the assessments must be performed at each unscheduled visit (Section 4.1.7).

e. The safety follow-up visit is only applicable for participants who do not start a new treatment year and for participants who prematurely discontinue the study in case IMP was stopped less than 28 days before the last study visit.

f. Participants will be asked to sign the current ICF version before receiving the first PFS dose and before starting a new 48-week treatment period to continue the study. The first day of a new treatment year will coincide with the last visit of the previous treatment year.

g. Assessment includes physical examination and weight; weight will be measured at the SD1 visit and during the study in case of an obvious weight change.
Vital signs include semisupine blood pressure and heart rate, and body temperature.

h. Suicidality will be assessed by specifically answering 1 question from the Patient Health Questionnaire-9 (PHQ-9) depression questionnaire (Section 8.13).

i. Only during the first 48-week treatment period, blood samples for immunogenicity testing will be taken predose at the study visits to measure ADA directed toward efgartigimod (measured in serum).

j. Predose (before IMP administration at the study visits) samples will be taken for the PD analysis (total IgG).

k. IMP will be administered via PFS; further information is provided in Section 6.2.2. IMP administration will occur after blood samples have been taken for laboratory safety, PD, and/or immunogenicity. After the participant or caregiver is considered competent, the (self-)administration can continue, and supervision may no longer be needed at the discretion of the investigator.
IMP kits will be returned to the site at the next site visit during which IMP accountability will be performed when the kits are available.

l. The last planned IMP (self-)administration of each treatment year is 1 week before the last visit when receiving efgartigimod PH20 SC once weekly, or 2 weeks before the last visit when receiving efgartigimod PH20 SCR once every 2 weeks, or 3 weeks before the last visit when receiving efgartigimod PH20 SC once every 3 weeks.

m. This requirement is only needed for participants who will start a new treatment year.

n. AEs and intake of concomitant medication(s) will be monitored continuously from signing the ICF until the last study-related activity. In case of early discontinuation, any AEs/SAEs should be assessed for 30 days after the ED visit or until satisfactory resolution or stabilization.

o. Any vaccination received during the study (from screening onward) will be entered as concomitant medication.

Note: For participants who provide separate consent, the titers of produced vaccination antibodies will be measured using retained blood samples (leftover blood samples taken for other tests) during the study.

10.9.5. IMP Collection Visits in the Optional Substudy

IMP collection visits are planned every 6 weeks after the prior site visit (eg, weeks 18, 30, 42, etc).

The participant will collect their IMP at the site or another location agreed upon with the site or home nurse.

The participant will be asked to return all used and unused IMP at the next scheduled visit, during which IMP accountability will be ensured.

Except for IMP dispensation and possibly IMP administration, no other study assessments are planned at these IMP collection visits. In addition, data that are collected continuously during the study (eg, AEs, concomitant medication, etc) will be collected during IMP collection visits.

As of protocol v8.0, the source of IMP will change from a single-dose vial to a single-dose PFS. The site staff will provide “PFS Instructions for Use” to the participant and caregiver at the first supervised PFS IMP administration visit (Section 6.2.2).

10.10. Appendix 10: Protocol Amendment History

General Amendment 6, Global Protocol Version 7.0:

The primary rationale for this amendment was to give participants more flexibility during the optional substudy to explore less frequent dosing.

The major changes from global protocol version 6.0 compared to global protocol version 7.0 are summarized in the following table. Minor editorial changes (eg, to improve readability or correct spelling mistakes) are not included.

Summary of Changes Between Protocol Version 6.0 and Protocol Version 7.0

Section	Change in study conduct or planned analysis	Brief rationale
10.9.2. Description of the Optional Substudy (formerly Section 11.9.1.) 10.9.3. Schema of the Optional Substudy (formerly Section 11.9.2.)	Participants with clinical deterioration during the optional substudy may now return to efgartigimod PH20 SC dosing every 2 or 3 weeks after their CIDP condition has been restabilized.	To give participants more flexibility in their dosing regimen
	The protocol was clarified to allow participants to return to a more frequent dosing regimen at any time, regardless of clinical deterioration.	
10.9.5. IMP Collection Visits in the Optional Substudy (new section) 10.9.3. Schema of the Optional Substudy (formerly Section 11.9.2.) 10.9.4. Schedule of Activities (SoA) During the Optional Substudy (formerly Section 11.9.3.)	As of visit 3 at week 12, the frequency of on-site visits in which the participant completes study assessments has been reduced from every 6 weeks to every 12 weeks. The intervening visits (eg, weeks 18, 30, 42) are now IMP collection visits in the optional substudy (ie, no study assessments).	
10.9.1. Definitions of Terms (new section) 10.9.3. Schema of the Optional Substudy (formerly Section 11.9.2.) 10.9.4. Schedule of Activities (SoA) During the Optional Substudy (formerly Section 11.9.3.)	The terms stable clinical condition, clinical deterioration, and restabilization of CIDP condition are defined in new Section 10.9.1.	To focus on disease-specific parameters (aINCAT) to simplify the assessments
6.4.2. Prohibited Medications (formerly Section 7.4.2.)	Text has been added to clarify that the time period for prohibited medications	

Section	Change in study conduct or planned analysis	Brief rationale
	or treatments ends with the last IMP administration.	To allow participants to receive the standard of care when discontinuing IMP
7.1.1. IMP Discontinuation (formerly Section 8.2.)	Text has been added to clarify that participants who discontinue IMP will attend an ED visit and an SFV only.	
7.2. Participant Discontinuation/Withdrawal From the Study (formerly Section 8.3.)	Participants whose condition deteriorates after discontinuing IMP are no longer required to withdraw from the study.	
7.1.1. IMP Discontinuation (formerly Section 8.2.)	A requirement was added for participants who develop a new or recurrent malignancy (except for basal cell carcinoma of the skin) to discontinue the IMP.	To ensure participant safety
7.1.2. IMP Interruption (formerly Section 8.1.)	Any SAE considered related to IMP by the sponsor will now lead to IMP interruption. Clinically significant active infection considered related to the IMP by the sponsor will now result in IMP interruption instead of IMP discontinuation.	To ensure participant safety and allow participants to resume efgartigimod PH20 SC after resolution of an SAE or infection
7.2. Participant Discontinuation/Withdrawal From the Study (formerly Section 8.3.)	Participants can be withdrawn from the study at the sponsor's request.	To follow the recommendation of DSMB
	Revised text (deleted text is indicated in strikethrough; a bold font is used to indicate added text): ~~If the participant also withdraws consent to disclose future information, the sponsor may retain and continue to use any data collected before such a withdrawal of consent.~~ ~~The samples from participants withdrawing from the study that have already been collected (but may not have been analyzed) before withdrawal will~~	To clarify guidance on future sample use

Section	Change in study conduct or planned analysis	Brief rationale
	~~still be used for the study but not for future research.~~ **If the participant withdraws consent to participate in the study, the sponsor can retain and continue to use any data collected before consent was withdrawn. Future research on samples collected from participants who withdraw consent to participate in the study will not be impacted unless the participant also withdraws the consent for future research.**	
2.3.2. Risk Assessment (formerly Section 3.3.3.) 8.4.7. AEs of Clinical Interest (new section)	Hypersensitivity or allergic reactions such as rash, urticaria, angioedema, serum sickness, and anaphylactoid or anaphylactic reactions are considered infusion/injection-related reactions (not injection-site reactions).	To align with efgartigimod's current risk profile
	Injection-site reactions were added as a potential clinically significant risk. In addition, the infusion/injection-related reactions and injection-site reactions were defined as AEs of clinical interest that require attention or special monitoring as specified in new Section 8.4.7.	
8.3.4. Pregnancy Testing	Participants or their partners who become pregnant during the study will be asked to consent to be followed for up to 12 months after the baby's birth.	To ensure follow-up on the baby's health
8.4. Adverse Events, Serious Adverse Events, and Other Safety Reporting (formerly Section 9.4.) 8.4.1. Time Period and Frequency for Collecting AE and SAE Information (new section) 8.4.2. Method of Detecting AEs and SAEs (new section)	Guidance on safety reporting of AEs, SAEs, AESIs, pregnancies, and AEs of clinical interest was expanded.	To emphasize the importance of safety reporting

Section	Change in study conduct or planned analysis	Brief rationale
8.4.3. Follow-Up of AEs and SAEs (new section) 8.4.4. Regulatory Reporting Requirements for SAEs (new section) 8.4.5. Pregnancy (formerly Section 9.4.1)		
10.3.3. Male Contraception (formerly Section 11.3.3.)	Male contraception is no longer required.	To align with the current efgartigimod IB: Reproductive toxicology studies showed no effect on male reproductive potential.
1.1. Synopsis 5.1. Inclusion Criteria (formerly Section 6.1.) 5.2. Exclusion Criteria (formerly Section 7.1.)	The following inclusion and exclusion criteria were renumbered: • Inclusion criteria 1, 2.a, and 4.a were renumbered as 1.a, 2.b, and 4.b, respectively. • Exclusion criteria 1, 2.a, and 3.a were renumbered as 1.a, 2.b, and 3.b, respectively.	To distinguish enumeration from previous inclusion and exclusion criteria
Table 2: Schedule of Activities for Subsequent Treatment Periods After the First 48-week Treatment Period Table 6: Schedule of Activities for Subsequent Treatment Periods After the First 48-Week Treatment Period During COVID-19 Pandemic (formerly Section 11.8.1.2.) 10.9.4. Schedule of Activities (SoA) During the Optional Substudy (formerly Section 11.9.3)	The predose 2-hour window for PD sample collection was removed.	To allow for flexibility in timing for sampling
10.9.2. Description of the Optional Substudy (formerly Section 11.9.1.)	Participants must sign the ICF for the optional substudy before any substudy-specific assessment is performed.	To clarify guidance for informed consent for the optional substudy

Section	Change in study conduct or planned analysis	Brief rationale
8.1.1. Use and Storage of Biological Samples (new section)	Directions for the use and storage of biological samples has been added to the protocol in a new section.	To provide explicit guidance on the use and storage of biological samples

General Amendment 5, Global Protocol Version 6.0:

The major changes from global protocol version 5.0 compared to global protocol version 6.0 are summarized in the following table. Added text is shown in **bold** font and deleted text is shown with ~~strikethrough~~ font. Minor editorial changes (eg, to improve readability or correct spelling mistakes) are not included.

Summary of Changes Between Protocol Version 5.0 and Protocol Version 6.0

Section(s)	Change(s)	Rationale
• Section 1.1, Section 1.2	• The following changes were made in Section 1.1. Synopsis and the footnote in Section 1.2. Schema: • The maximum number of patients in this trial will be the number of patients who entered the ARGX-113-1802 trial, ie, ~~maximum~~ **up to** 360 patients ~~of whom up to 180 patients are planned to be randomized in Stage B of the ARGX-113-1802 trial~~.	In the event-driven ARGX-113-1802 trial, a total of 88 events are required to detect a hazard ratio of 0.5 with 90% power at a 1-sided alpha level of 0.025 using a log-rank test. The protocol has been updated to avoid ambiguity in the number of patients that need to be randomized to observe the 88 events; this depends on several factors and cannot be exactly estimated (the ARGX-113-1802 protocol provides several scenarios under various assumptions related to the median time-to-event). This update clarifies that up to 360 patients are expected to be enrolled in Stage A to ensure that the number of patients randomized in Stage B is sufficient to observe the required 88 events.
• Section 3.3.2	• Updated Section 3.3.2 Risks	Updated Risk section to reflect additional safety data that have become available since the start of the trial
• Section 8.2	• The following changes were made in Section 8.2. Permanent Early Discontinuation of IMP: • ~~Development of an SAE that is likely to result in a life threatening situation or pose a serious safety risk.~~ • **Patient develops an SAE or AE that contraindicates further administration of IMP in the investigator's opinion or an AE of CTCAE grade 4 that is considered related to IMP by the sponsor.**	These changes clarify when a patient must permanently discontinue IMP.

Section(s)	Change(s)	Rationale

• Section 9.3.3, Section 11.2	• The following changes were made in Section 9.3.3. Clinical Safety Laboratory Assessments:
	  • The samples will be analyzed at a centralized certified laboratory. **In the exceptional circumstance that samples cannot be sent to a central laboratory, a local laboratory can be used.**
	  • Clinical laboratory tests will be reviewed for results of potential clinical significance at all time points throughout the trial**, with the exception of total protein and albumin results from cycle 1 baseline through week 4, which will be kept blinded for the site**. **A system will be implemented to notify the investigator in case of out-of-range values to allow for appropriate safety follow-up.**
	• The following footnote was added to “Total protein” and “Albumin” in Section 11.2. Appendix 2: Clinical Laboratory Tests:
	  • **In the exceptional circumstance that samples are not processed in a central laboratory: Total protein and albumin analysis at baseline up to and including week 4 of cycle 1 only will not be performed.**
		These changes clarify the following:
		• A local laboratory can be used in the exceptional circumstance that a central laboratory cannot be used.
		• As an additional blinding precaution of the parent trial ARGX-113-1802, total protein and albumin results from a central laboratory will not be sent to the site at the beginning of the OLE trial and must not be measured during the beginning of the OLE trial if a local laboratory is used.

• Section 11.4.4	• Section 11.4.4. Reporting of SAEs and AESIs was updated as follows:
	SAE **and AESI** Reporting ~~via paper form~~:
	  • All SAEs and AESIs (Section 9.4.2) will be recorded (within 24 hours) on the ~~paper SAE report form and the~~ AE form ~~in~~ **of** the eCRF. **SAEs will also be recorded on the paper SAE report form.**
	  • The investigator or designated site staff ~~should check that~~ **will ensure** all entered data are consistent.
	  • An alert email for the SAE **and AESI** report**s** ~~in~~ **on** the eCRF will ~~then~~ automatically be sent by email to the **sponsor or designee’s** ~~SAE coordinator’s~~ safety mailbox via the electronic data capture (EDC) system.
	  • The paper SAE report form ~~should~~ **will** be faxed or emailed to the **sponsor’s designee** ~~SAE coordinator~~ **(refer to the Serious Adverse Event Reporting details on the first page of this protocol).** ~~Contacts for SAE reporting can be found on the frontpage.~~
		These changes clarify the SAE and AESI reporting procedures.

• Section 11.9.3	• The IMP dispensation activity was deleted from the early discontinuation visit column in Section 11.9.3. Schedule of Activities (SoA) During Dosing Frequency Substudy.	This correction was implemented because IMP should not be dispensed at an early discontinuation visit.

Summary of Changes Between Protocol Version 4.0 and Protocol Version 5.0

The major changes from protocol version 4.0 compared to protocol version 5.0 are summarized below. Note that protocol version 5.0 has not been effective. Added text is shown in **bold** font and deleted text is shown with ~~strikethrough~~ font. Minor editorial changes (eg, to improve readability or correct spelling mistakes) are not included.

Section(s)	Change(s)	Rationale
• SIGNATURE OF SPONSOR	• Luc Truyen, MD, PhD is now the sponsor signatory for this protocol.	Effective 01 Apr 2022, Dr. Luc Truyen became the sponsor's chief medical officer.
• Throughout the protocol, including Section 5.2, Section 5.3, Section 11.9	• Addition of an optional dosing frequency substudy, which will offer patients who have a stable clinical condition for at least 12 weeks with weekly dosing of efgartigimod PH20 SC 1000 mg the option to receive less frequent dosing, to evaluate the maintenance of the clinical condition by administration of efgartigimod PH20 SC 1000 mg at 2 lower dosing frequencies (in a sequential manner, starting with q2w, and if the patient remains stable, at q3w). • Sections 5.2 and 5.3: Addition of the rationale for exploring less frequent dosing in a substudy.	This substudy allows patients who are well-controlled (with a stable clinical status) on the weekly dosing regimen (in ARGX-113-1802 and ARGX-113-1902) to explore a reduction of the dosing frequency to q2w and potentially to q3w, with the aim of maintaining their well-controlled disease symptoms, and potentially improving the safety profile and convenience.
• Synopsis, Section 4.2, Section 10.4.2.6	• Addition of the following objective: – To explore administration of the treatment by caregivers • Addition of the following endpoint: – Percentage of patients with treatment administered by caregiver over time	In this open-label extension (OLE) trial, efgartigimod PH20 SC can also be administered by a patient's caregiver; the percentage of patients who receive treatment via a caregiver will be explored.
• Throughout the protocol	• Addition of IMP collection visits at weeks 6, 18, 30, and 42: Except for IMP dispensation and possibly also IMP administration, no other trial assessments are planned at these IMP collection visits.	New temperature stability data allow for a broader temperature storage for a period of 6 weeks. Therefore, IMP will be dispensed every 6 weeks to allow the IMP to be stored under less strict conditions (see the Pharmacy Manual).
• Section 7.2	• Deletion of instructions to confirm appropriate temperature conditions.	
• Synopsis, Section 6.1, Section 6.2, Section 11.3	• Update of the inclusion criterion regarding the period of contraception use: An acceptable method of contraception should now be used from signing the ICF until the date of the last IMP dose (instead of from baseline to 90 days after the last IMP dose).	These changes are based on nonclinical teratogenicity and reprotoxicity data.
	• The previous inclusion criterion for male patients to not donate sperm during the trial period and 90 days thereafter was deleted.	
	• The exclusion criterion that women cannot become pregnant until at least 90 days after the last IMP dose has been updated to not becoming pregnant during the trial.	

Section(s)	Change(s)	Rationale
• Synopsis, Section 5.1, Section 11.9	• Addition of the following for the end of the trial: Patients will be offered the opportunity to continue in this trial (or substudy, if applicable) until 2 years after marketing authorization in a patient's local country or until efgartigimod PH20 SC becomes commercially available for patients with CIDP or becomes available via continued access program, whichever comes first.	This change clarifies when this trial will end.
• Synopsis, Section 10, Section 10.5, Section 10.5.6	• A first interim analysis of the OLE trial, based on data with weekly dosing, is planned around the time of the database lock of the preceding ARGX-113-1802 trial.	When the required number of events has been observed for the primary endpoint analysis for Stage B of the ARGX-113-1802 trial, an interim analysis of the ARGX-113-1902 trial is planned.
• Section 9.2, Section 9.3.1	• Assessments of efficacy and vital sign measurement should be done before IMP administration.	The order of assessments was clarified.
• Synopsis, Section 7.3, Section 10.1.3	• Deletion of the definition of the "per protocol population" and deletion of the analysis by the "per protocol population."	In this OLE trial, no analyses are planned to be performed by a "per protocol population."
• Synopsis, Section 5.1, Section 7.4.1.1, Section 11.1.3, Section 11.4.1	• Deletion of legally authorized representative (according to local regulations) and/or that this representative needs to be of adult age.	This trial is for adult patients, and no legally authorized representative is needed.
• Synopsis, Section 1.3, Section 4.2, Section 5.1, Section 9.7, Section 10.4.2.3, Section 11.2, Section 11.8.1	• Samples for PK analysis will only be taken during the first 48-week treatment cycle (and safety follow-up, if applicable) of the trial.	PK measurements for longer than 48 weeks (followed by a safety follow-up period, if applicable) are not necessary for this OLE trial.
• Section 1.3, Section 5.1, Section 9.10, Section 11.2, Section 11.8.1	• Blood samples for potential future biomarker analysis will only be collected (from patients who provided separate consent) during the first 48-week treatment cycle of the trial.	Sample collection for future biomarker analysis for longer than 48 weeks is not necessary for this OLE trial.
• Section 1.3, Section 5.1, Section 9.3.2, Section 11.8.1	• ECG measurements will only be performed during the first 48-week treatment cycle (and safety follow-up, if applicable) of the trial.	ECG recordings for longer than 48 weeks (followed by a safety follow-up period, if applicable) are not necessary for this OLE trial because no cardiovascular risk has been associated with efgartigimod PH20 SC treatment.
• Synopsis, Section 1.3, Section 4.2, Section 5.1, Section 10.4.2.2, Section 11.2, Section 11.8.1	• Blood samples (serum) for measurement of antibodies and neutralizing antibodies against efgartigimod will only be collected during the first 48-week treatment cycle of the trial.	Measurement of immunogenicity against efgartigimod for longer than 48 weeks (followed by a safety follow-up period, if applicable) is not necessary for this OLE trial.

Section(s)	Change(s)	Rationale
• Synopsis, Section 1.3, Section 4.2, Section 5.1, Section 9.11, Section 10.4.2.2, Section 11.2, Section 11.8.1	• Blood samples (plasma) for possible measurement of antibodies and neutralizing antibodies against rHuPH20 will be collected and stored, and will be analyzed in case of safety concerns.	Considering the favorable safety profile of rHuPH20, measurement of immunogenicity against rHuPH20 is not necessary, except in the case of safety concerns.
• Synopsis, Section 6.2, Section 7.4.2	• Synopsis and Section 6.2 "Exclusion criteria," addition of the following to exclusion criterion #3: – "or patients who (intend to) use prohibited medications and therapies (see Section 7.4.2) during the trial" • Section 7.4.2 "Prohibited Medications:" – For "Systemic corticosteroids, including oral, for use in CIDP," the text in brackets has been updated to the following "(Note: short-term use [up to 2 weeks] for treatment of other indications is allowed)." This replaces having to discuss corticosteroid use with the medical monitor. – Addition of the following: "Use of complementary therapies, including traditional Chinese medicines and herbal remedies, containing any of the following: naturally derived glucocorticoids for use in CIDP; medication with clinical evidence-based immunosuppressive, immunomodulatory, peripheral or central nervous system effects; or procedures (eg, acupuncture) for any neurological conditions within 4 weeks or 5 half-lives (whichever is longer) before IMP dosing and agreed not to use during the trial. Note: short-term use (up to 2 weeks) of glucocorticoids for other indications is allowed."	Prohibited medications and therapies were added.
• Section 9.4.3	• Addition of the following: An injection-site reaction is any AE developing at the injection-site. Localized injection-site reactions are frequently observed in studies in which efgartigimod is administered SC in combination with rHuPH20. The most frequently reported injection-site reaction AEs are injection-site erythema, injection-site pain, and injection-site swelling. Any injection-site reaction will be reported as an AE (see Section 9.3.1 and Section 11.4). Certain types of local reactions could be photographed and shared with the sponsor for review and assessment. As a routine precaution, participants should be trained or observed closely by a trained health care professional for any potential injection-site reaction.	These changes clarify that injection-site reactions are monitored in this trial, and certain types of local reactions could be photographed and shared with the sponsor for review and assessment.
• Section 11.11	• Addition of Appendix 11: Country-Specific Requirements	The specific requirements of certain countries were added in an Appendix

Section(s)	Change(s)	Rationale
		of the global protocol to align with the new EU Clinical Trials Regulation.

General Amendment 3, Global Protocol Version 4.0:

The major changes from Protocol Version 3.0 compared to Protocol Version 4.0 are summarized in the table below. Deleted text is indicated in strikethrough; a bold and underlined font is used to indicate added text. Minor administrative editorial changes (eg, to improve readability or correct spelling mistakes) are not summarized in the following table:

Summary of Changes Between Protocol Version 3.0 and Protocol Version 4.0

Section(s)	Change	Rationale
• Front page	• Serious Adverse Events Reporting: **<u>Parexel International</u>** **<u>8 Federal Street, Billerica, MA 01821, United States of America</u>** ~~SGS Belgium NV, Life Sciences Division, Noorderlaan 87, 2080 Antwerp, Belgium~~ • For drug safety reporting, contact the following email address: Safety Mailbox/Fax: • **Email: <u>248700ADR@parexel.com</u> ~~be.life.saefax-ma@sgs.com~~** **Fax: <u>+1 833 644-0806</u> ~~+32 (0)15 29 93 94~~**	Updated contact information for SAE reporting.
• Section 1.1, Section 1.3, Section 9.8, Section 9.10, Section 10.4.2.4, Section 11.2, Section 11.8.1	• Throughout the protocol: Deletion of measurement of IgG subtypes (IgG1, IgG2, IgG3, and IgG4): Pharmacodynamic analysis includes the measurement of total IgG (and not of IgG subtypes).	The measurement of total IgG is sufficient for pharmacodynamic analysis. Measurement of IgG subtypes is not needed.
• Section 1.1, Section 1.3, Section 5.1, Section 5.1.1, Section 5.1.2, Section 5.1.3, Section 7.2, Section 7.3, Section 7.4.1.1, Section 7.4.1.2, Section 11.8, Section 11.8.1	• Throughout the protocol: Next to self-administration by the patient, the patient's caregiver can also administer IMP. In the latter case, the patient's caregiver will be trained to administer IMP. • Section 1.1 "Synopsis" and Section 5.1 "Overall Design": … IMP will be administered at the site at the scheduled visits. Other IMP administrations will be administered by the patient (self-administration)**<u>, the patient's caregiver,</u>** or via a home nurse or concierge service for visit at the trial site. **<u>Note that the patient's caregiver who could administer IMP should be of legal age.</u>** • Section 7.2 "Preparation, Handling, Storage, Administration, and Accountability": … For self-administration or administration via **<u>the patient's caregiver or via</u>** a home nurse (for administration in between the trial visits), the patient**<u>, the patient's caregiver,</u>** or the home nurse will be trained to confirm that appropriate temperature conditions have been maintained during IMP transit. **<u>Note that the patient's caregiver who could administer IMP should be of legal age.</u>**	Addition of the patient's caregiver (who is of legal age) to administer IMP and to be trained to do so.

Section(s)	Change	Rationale
	Only patients enrolled in the trial shall receive IMP and only authorized site staff shall supply IMP, administer IMP, or train the patient**, the patient's caregiver,** or the home nurse on IMP (self-)administration. Each patient will receive a specific training for self-administration during the ARGX-113-1802 trial (and at SD1 of the first treatment cycle and later during the trial, if needed)**. In case of IMP administration by the caregiver, the caregiver will be trained from the first visit of the ARGX-113-1902 trial onwards, until the caregiver and the investigator are confident in the ability of the caregiver to administer IMP. Each patient and each caregiver who will (self-)administer IMP will receive** ~~and~~ a manual on preparation, handling, storage**,** and administration of the IMP. …	
• Section 1.3, Section 7.4.1.1, Section 11.8.1	• Section 1.3 and Section 11.8.1 "Schedule of Activities" (normal and during COVID-19 Pandemic): Addition of the following footnote to "Concomitant therapies": Any vaccination received during the trial (from screening onwards) will be entered as concomitant medication. ***Note***: For patients who provide separate consent, the titers of produced vaccination antibodies will be measured using retained blood samples (left over blood samples taken for other tests) during the trial. • Section 7.4.1.1. "**Collection of Vaccination Data**": **For patients who receive a vaccination during the trial, the date of administration and the brand name will be captured (as for other concomitant medications).** **For patients for who vaccination history data were not collected in the preceding ARGX-113-1802 trial, any vaccination information the patient, his/her legally authorized representative (according to local regulations), or caregiver can remember should be recorded on the eCRF (with the brand name of the vaccine and date of vaccine administration recorded, if possible). If in the past a same vaccine was received more than once, including as booster, only the last one is to be collected.** **For patients who provide separate consent, the titers of produced vaccination antibodies will be measured using retained blood samples (left over blood samples taken for other tests) during the trial (see Section 11.2).** • Section 11.2 "Appendix 2: Clinical Laboratory Tests": … * Blood samples for potential future biomarker analysis will be collected from all patients who provided separate consent for biomarker analysis**, as well as used for vaccination antibody testing.**	Addition of collection of vaccination data and - for those who consent - optional measurement of antibody titers.

Section(s)	Change	Rationale
• Section 3.3.1, Section 3.3.2	• Section 3.3.1 "Benefit" … **In addition, phase 3 studies with efgartigimod IV in patients with gMG showed that the trial's primary endpoint was met with statistical significance. The primary endpoint was defined as the "percentage of anti-acetylcholine receptor antibody (AChR-Ab) seropositive patients who, during the first cycle, have a reduction of ≥2 points in the Myasthenia Gravis Activities of Daily Living (MG-ADL) total score compared to study entry baseline for ≥4 consecutive weeks, with the first reduction occurring no later than 1 week after the last investigational medicinal product (IMP) infusion." A total of 67.7% of AChR-Ab positive patients treated with efgartigimod achieved the primary endpoint compared with 29.7% on placebo (p<0.0001). In the overall patient population (AChR-Ab seropositive and seronegative patients), 67.9% of the patients treated with efgartigimod were MG-ADL responders compared with 37.3% on placebo (p<0.0001). Other key secondary endpoints for this trial were also met with high statistically significance.** • Section 3.3.2 "Risks" … In clinical trials to date, efgartigimod has been well tolerated in healthy adult subjects and patients with gMG and ITP~~, separately~~**; efgartigimod was administrated either IV or SC**: the majority of treatment-emergent adverse events (TEAEs) were considered to be mild (grade 1) **or moderate (grade 2)** in severity. **In the phase 3 trials in patients with gMG, the majority of TEAEs were reported as mild (grade 1) or moderate (grade 2) in severity. The most frequently (≥5%) reported TEAEs during all cycles of efgartigimod 10 mg/kg treatment in the overall population who received at least 1 dose of efgartigimod were headache, nasopharyngitis, upper respiratory tract infection, diarrhea, urinary tract infection, nausea, myalgia, and oropharyngeal pain. TEAEs of severity of grade ≥3 that occurred in >2 patients who received efgartigimod were headache and myasthenia gravis. The only TEAE that led to efgartigimod discontinuation reported in >2 patients was myasthenia gravis. No SAEs were assessed by the investigator as related to efgartigimod treatment.** **Four fatal cases have been reported, however, none of the fatal events were assessed by the investigator as related to efgartigimod treatment. Causes of death was unknown in a patient who died at home without witness. The other 3 patients died due to myasthenia gravis crisis, lung cancer Stage IV, and acute myocardial infarction. These 4 patients were between 55 and 79 years old and all had clear comorbidities, including underlying gMG.**	Sections updated to align with the current version of the Investigator's Brochure (version 9.0).

Section(s)	Change	Rationale
	Adverse drug reactions were identified based on safety data from the phase 3 double-blind, placebo-controlled trial in patients with gMG and include upper respiratory tract infection, urinary tract infection, bronchitis, myalgia, and procedural headache. **Due to the efgartigimod mechanism of action of reducing IgG levels, AEs in the Infections and Infestations SOC have been defined as AESIs. In the phase 3 trials with efgartigimod IV in patients with gMG, the most frequently reported treatment-emergent AESIs by PT were nasopharyngitis, upper respiratory tract infection, urinary tract infection, and bronchitis. Most AESIs were mild or moderate in severity. Few patients had severe or serious AESIs reported, and no SAEs of AESIs were assessed by the investigator as related to efgartigimod treatment.** **All therapeutic proteins have the potential to elicit immune responses, potentially resulting in hypersensitivity or allergic reactions. As with any IV injection, the potential exists for infusion-related reactions (IRRs) to occur during or within 48 hours following the administration of efgartigimod. Overall, the frequency of IRRs in the clinical trials was low. At the preferred term (PT) level, no TEAEs of drug hypersensitivity or anaphylactic reaction were reported. None of the IRRs reported were serious. Most IRRs were mild in severity and no IRRs resulted in a change in efgartigimod administration. For administration of efgartigimod PH20 SC, the local injection-site will be monitored during the trial.** **In the phase 3 trial with efgartigimod IV in patients with gMG, no differences or imbalances were observed between the efgartigimod and placebo groups in the incidence or rates of abnormal values in clinical laboratory parameters. There was no reduction in levels of serum albumin with the administration of efgartigimod. In the extension trial in patients with gMG, the most frequently reported grade ≥3 abnormalities were lymphocyte count decreased.** ~~No TEAEs of grade ≥3 have been reported. The most common TEAE suspected to be related to efgartigimod is headache; however, there is no evidence that headache occurs more frequently in patients administered efgartigimod than in patients administered placebo.~~ ~~Injection or catheter site reactions are AEs for which there exists a reasonable possibility of a causal relationship with the efgartigimod administration procedure, although there is no evidence that these AEs are related to efgartigimod itself. Chills were reported in healthy subjects administered efgartigimod IV 25 mg/kg. However, infusion-related reactions (IRRs) are reactions that are unpredictable, non-dose related, generally unrelated to the drug's pharmacological activity, and usually resolve when treatment is discontinued. IRRs include hypersensitivity~~	

Section(s)	Change	Rationale
	~~reactions and cytokine release syndromes. These reactions can occur during the infusion of a cytotoxic or monoclonal antibody therapy (uniphasic reaction) and/or within hours of an infusion (biphasic/delayed reaction) and may be caused by the therapeutic agent, diluent, or delivery vehicle. IRR symptoms, caused by the release of cytokines from the cells, include nausea, headache, tachycardia, hypotension, rash, flushing, dyspnea, bronchospasm, back pain, fever, urticaria, and edema.~~ ~~Because these reactions are difficult to evaluate in prospective randomized clinical trials and due to the unexpected nature of these events, the local injection-site will be monitored during the trial.~~ ~~There has been no evidence of an increased risk of infection.~~ …	
• Synopsis, Section 6.1, Section 11.3	• Inclusion criteria: 4. Women of childbearing potential must use a highly effective **<u>or acceptable</u>** method of contraception ~~(failure rate of less than 1% per year)~~ from baseline to 90 days after the last administration of IMP 5**<u>a</u>. <u>Male patients agree not to donate sperm during the trial period and 90 days thereafter.</u>** ~~Non-sterilized male patients who are sexually active with a female partner of childbearing potential must use a condom and his partner must use a highly effective method of contraception (failure rate of less than 1% per year) from screening to 90 days after the last administration of IMP. Male patients practicing true sexual abstinence (when this is in line with the preferred and usual life style of the participant) can be included. Sterilized male patients who have had vasectomy with documented aspermia post-procedure can be included. In addition, male patients are not allowed to donate sperm from screening to 90 days after the last administration of IMP.~~ • Section 11.3 "Appendix 3: Contraceptive **<u>and Barrier</u>** Guidance": **<u>11.3.1. General Instructions and Definitions</u>** **<u>Contraceptive use by men and women should be consistent with local regulations regarding the methods of contraception.</u>** **<u>Male patients are not allowed to donate sperm during the period from signing of ICF, throughout the duration of the trial, and for 90 days after the last administration of IMP (trial medication injection).</u>** **<u>Women of childbearing potential must have a negative serum pregnancy test at the screening visit and a negative urine pregnancy test at baseline before IMP can be administered.</u>** **<u>A woman is considered of childbearing potential unless she is either:</u>**	Reproductive toxicity studies with efgartigimod IV have been completed in rats and rabbits. Efgartigimod did not show a teratogenic effect (tested in rats and rabbits), nor did it adversely affect male and female fertility or any other reproductive and developmental performance (tested in rats). Therefore, acceptable methods of contraception can be used in addition to highly effective methods.

Section(s)	Change	Rationale
	Postmenopausal, defined by continuous amenorrhea for at least 1 year without an alternative medical cause with a follicle-stimulating hormone (FSH) measurement of >40 IU/L. A historical pretreatment FSH measurement of >40 IU/L is accepted as proof of a postmenopausal state for women on hormone replacement therapy. **Surgically sterilized: Women who have had a documented permanent sterilization procedure (ie, hysterectomy, bilateral salpingectomy, or bilateral oophorectomy).** **11.3.2. Female Contraception for Women of Childbearing Potential** **Women of childbearing potential should use a highly effective or acceptable method of contraception during the trial and for 90 days after the last administration of the IMP.** **The following highly effective and/or acceptable methods of contraception, described in the Recommendations Related to Contraception and Pregnancy Testing in Clinical Trials[8], are allowed in this trial:** **Combined (estrogen and progestogen containing) hormonal contraception associated with inhibition of ovulation:** – **Oral** – **Intravaginal** – **Transdermal** **Progesterone-only hormonal contraception associated with inhibition of ovulation:** – **Oral** – **Injectable** – **Implantable** **Progesterone-only oral hormonal contraception, where inhibition of ovulation is not the primary mode of action** **Intrauterine device (IUD)** **Intrauterine hormone-releasing system (IUS)** **Bilateral tubal occlusion** **Vasectomized partner** **Sexual abstinence** **Male or female condom with or without spermicide** **Cap, diaphragm, or sponge with spermicide** **11.3.3. Male Contraception** **An acceptable method of contraception is a condom.** ~~1. Women of childbearing potential must have a negative serum pregnancy test at the screening visit and a negative urine pregnancy test at baseline before trial medication~~	

Section(s)	Change	Rationale
	~~(injection) can be administered. Women are considered of childbearing potential unless they are post-menopausal (defined by continuous amenorrhea) for at least 1 year with a follicle-stimulating hormone (FSH) of >40 IU/L or are surgically sterilized (ie, women who had a hysterectomy, both ovaries surgically removed, or have a documented permanent female sterilization procedure including tubal ligation). FSH can be used to confirm post-menopausal status in amenorrheic patients not on hormonal replacement therapy. For patients on hormonal replacement therapy, a historical FSH value that was measured before initiation of hormonal replacement therapy is acceptable to determine post-menopausal status.~~ ~~2. Women of childbearing potential should use a highly effective method of contraception (ie, pregnancy rate of less than 1% per year) during the trial and for 90 days after the last administration of the IMP. They must be on a stable regimen, for at least 1 month, of:~~ ~~• combined (estrogen and progestogen containing) hormonal contraception associated with inhibition of ovulation:~~ ~~○ oral~~ ~~○ intravaginal~~ ~~○ transdermal~~ ~~• progestogen-only hormonal contraception associated with inhibition of ovulation:~~ ~~○ oral~~ ~~○ injectable~~ ~~○ implantable~~ ~~• intrauterine device (IUD)~~ ~~• intrauterine hormone-releasing system~~ ~~• bilateral tubal occlusion~~ ~~• vasectomized partner (provided that the partner is the sole sexual partner of the trial participant and documented aspermia post procedure)~~ ~~• continuous abstinence from heterosexual sexual contact. Sexual abstinence is only allowable if it is the preferred and usual lifestyle of the patient. Periodic abstinence (calendar, symptothermal, post-ovulation methods) is not acceptable.~~ • ~~Non-sterilized male patients who are sexually active with a female partner of childbearing potential must use effective double contraception, being a condom for male patients and a highly effective form of contraception for the female partner of childbearing potential (same as for female patients described above). Male patients practicing true sexual abstinence (when this is in line with the preferred and usual lifestyle of the participant) can be included. Sterilized male patients who have had vasectomy with documented aspermia post procedure can be included. In addition, male patients are not allowed to donate sperm during this period from signing of ICF,~~	

Section(s)	Change	Rationale
	~~throughout the duration of the trial, and for 90 days after the last administration of IMP.~~	
• Section 6.4	• Section 6.4 "Screen Failures": **Eligibility to roll over must be verified at the last visit of the preceding trial ARGX-113-1802.** ~~Evaluations at the SD1 visit will be used to determine the eligibility of each patient before IMP administration on SD1.~~ Patients who consent to participate in the clinical trial but fail to meet the inclusion and exclusion criteria before IMP administration will be considered screen failures **and will be captured as "enrollment failures." Individuals who do not meet the criteria for participation in this trial (screen failures), based on the results from assessments in the preceding trial, may not be rescreened.**	Clarification of screen failures of this trial.
• Section 7.1	• Section 7.1 "IMP(s) Administered": … IMP will be provided by the sponsor **and is manufactured in accordance with Good Manufacturing Practice regulations**. The efgartigimod PH20 SC formulation will be provided in a vial at a concentration of 165 mg/mL **or 180 mg/mL (new concentration)** for efgartigimod and 2~~,~~000 U/mL for rHuPH20 (also referred to as ARGX-113/rHuPH20). Each dose of efgartigimod PH20 SC will contain 1~~,~~000 mg efgartigimod. **Note that there will be a transition period during which both formulations of efgartigimod PH20 SC (with efgartigimod at a concentration of 165 mg/mL or 180 mg/mL) will be used. After this transition period, all patients will receive the efgartigimod PH20 SC formulation with efgartigimod at the higher concentration of 180 mg/mL. The formulation with the higher concentration of efgartigimod (180 mg/mL) reduces the dosing volume for each SC injection.**	Efgartigimod PH20 SC will be used with efgartigimod at a higher concentration (180 mg/mL instead of 165 mg/mL), thereby reducing the dosing volume for each SC injection. The only difference between the 165 mg/mL and 180 mg/mL batches is a higher protein concentration (±10% higher protein concentration with 180 mg/mL compared to 165 mg/mL) and a slight reduction of the dosed amount of rHuPH20 with the new formulation. This difference is not expected to result in a different activity or safety profile.
• Section 11.4	Section 11.4 "Appendix 4: Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow-up, and Reporting." • Section 11.4.1 "Definition of AE": **AE Definition:** An AE is any untoward medical occurrence in a ~~patient or~~ clinical trial patient, … **Definition of Unsolicited and Solicited AE:** **An unsolicited AE is an AE that was not solicited using a Patient Diary and that is communicated by a patient who has signed the informed consent (or was signed by the patient's legally authorized representative [according to local regulations]). Unsolicited AEs include serious and non-serious AEs.** **Potential unsolicited AEs may be medically attended (ie, symptoms or illnesses requiring a hospitalization,**	Appendix for AEs/SAEs updated to align with the current protocol template of argenx, including addition of causality assessment and clarification of SAE reporting.

Section(s)	Change	Rationale
	or emergency room visit, or visit to/by a health care provider). The patient will be instructed to contact the site as soon as possible to report medically attended event(s), as well as any events that, though not medically attended, are of patient's concern. Detailed information about reported unsolicited AEs will be collected by qualified site personnel and documented in the patient's records. **Unsolicited AEs that are not medically attended nor perceived as a concern by the patient will be collected during interview with the patient and by review of available medical records at the next visit.** **Solicited AEs are predefined local (at the injection-site) and systemic events for which the patient is specifically questioned, and which are noted by the patient in his/her diary.** **Events to be collected as AEs ~~Meeting the AE Definition~~** … **Lack of efficacy by itself will not be reported as an AE or SAE. Such instances will be captured in the efficacy assessments. However, the signs, symptoms, and/or clinical sequelae resulting from lack of efficacy will be reported as AE or SAE if they fulfill the definition of an AE or SAE.** **Events NOT to be collected as AEs ~~Meeting the AE Definition~~** … • Section 11.4.2 "Definition of SAE": **An SAE is defined as any untoward medical occurrence that, at any dose:** **c. Requires inpatient hospitalization or prolongation of existing hospitalization** … Complications that occur during hospitalization are AEs. If a complication prolongs hospitalization or fulfills any other serious**ness** criteria, the event ~~is~~ **will be considered as** serious. … Hospitalization for elective treatment of a pre-existing condition that did not worsen from baseline is not ~~considered~~ **collected as** an AE. • Section 11.4.3 "Recording and Follow-up of AE and/or of SAE **AE and SAE Recording:** … It is **not** acceptable for the investigator to send photocopies of the patient's medical records to **the SAE coordinator/sponsor** ~~SGS~~ in lieu of completion of the AE/SAE CRF page **or SAE form**. There may be instances when copies of medical records for certain cases are requested by eg, **the SAE coordinator** ~~SGS~~ or the sponsor. …	

argenx	**Assessment of Event Severity:** **The investigator will make an assessment of severity for each AE and SAE reported during the trial.** All events (AEs and SAEs) observed will be graded using the National Cancer Institute (NCI) common terminology criteria for adverse events (CTCAE) version 5.0. ~~The NCI CTCAE is a descriptive terminology, which can be utilized for AE reporting. A grading (severity) scale is provided for each AE term.~~ The grade refers to the severity of the AE. If a particular AE's severity is not specifically graded by the guidance document, the investigator is to use the general NCI CTCAE definitions of grade 1 through grade 5 following his or her best medical judgment**, using**~~. The CTCAE displays grades 1 through 5 with unique clinical descriptions of severity for each AE based on~~ the following general guideline: • Grade 1: … • Grade 2: moderate; minimal, local or noninvasive intervention indicated; limiting age-appropriate instrumental activities of daily living **(eg, preparing meals, shopping for groceries or clothes, using the telephone)**. • Grade 3: severe or medically significant but not immediately life threatening; hospitalization or prolongation of hospitalization indicated; disabling; limiting self-care activities of daily living **(ie, bathing, dressing and undressing, feeding self, using the toilet, taking medications, and not bedridden)**. • Grade 4: …~~;~~ **or** urgent intervention indicated. **Assessment of Causality:** The investigator is obligated to assess the relationship between IMP and each occurrence of each AE/SAE~~.~~**, using one of the following categories:** • **Not related: events can be classified as "not related" if there is not a reasonable possibility that the IMP caused the AE.** • **Unlikely related: an "unlikely" relationship suggests that only a remote connection exists between the IMP and the reported AE. Other conditions, including chronic illness, progression or expression of the disease state, or reaction to concomitant medication, appear to explain the reported AE.** • **Possibly related: a "possible" relationship suggests that the association of the AE with the IMP is unknown; however, the AE is not reasonably supported by other conditions.** • **Probably related: a "probable" relationship suggests that a reasonable temporal sequence of the AE with drug administration exists and, in the Investigator's clinical judgment, it is likely that a causal relationship exists between the drug administration and the AE, and other conditions (concurrent illness, progression or expression of disease state, or**	

Section(s)	Change	Rationale
	concomitant medication reactions) do not appear to explain the AE. • **Related: a "related" relationship suggests that the AE follows a reasonable temporal sequence from administration of IMP, it follows a known or expected response pattern to the IMP, and it cannot reasonably be explained by known characteristics of patient's clinical state.** **In final evaluation for reporting, the assigned relationship, as per Council for International Organizations of Medical Sciences (CIOMS), will be converted into a "binary determination" as follows: Events with an assigned relationship of "unrelated" or "unlikely related" will be grouped into the "unrelated" category. Events with an assigned relationship of "related," "possibly related," or "probably related" will be grouped into the "related" category.** … There may be situations in which an SAE has occurred, and the investigator has minimal information to include in the initial report to **the SAE coordinator/sponsor** ~~SGS~~. However, it is important that the investigator always makes an assessment of causality for every event before the initial transmission of the SAE data to **the SAE coordinator/sponsor** ~~SGS~~. … **Follow-up of AE and SAE:** The investigator is obligated to perform or arrange for the conduct of supplemental measurements and/or evaluations as medically indicated or as requested by **the SAE coordinator/sponsor** ~~SGS~~ to elucidate the nature and/or causality of the AE or SAE as fully as possible. … Thereafter, new/updated information will be collected in the safety database at **the SAE coordinator** ~~SGS~~. The investigator will submit any updated SAE data to **the SAE coordinator** ~~SGS~~ within 24 hours of receipt of the information. • Section 11.4.4 "Reporting of SAEs **SAE Reporting via Paper Form:** ~~Facsimile transmission of the paper SAE form is the preferred method to transmit this information to SGS.~~ ~~In rare circumstances and in the absence of facsimile equipment, notification by telephone is acceptable with a copy of the SAE data collection tool sent by overnight mail or courier service.~~ ~~Initial notification via telephone does not replace the need for the investigator to complete and sign the SAE CRF pages within the designated reporting time frames.~~ • All SAEs will be recorded **(within 24 hours)** on the paper SAE report form and the AE form on the eCRF. The investigator or designated site staff should check that all entered data are consistent. An alert email for the SAE	

Section(s)	Change	Rationale
	report on the eCRF will then automatically be sent by email to the ~~sponsor's designated CRO~~ **SAE coordinator's** safety mailbox via the electronic data capture (EDC) system. The paper SAE report form should be faxed or emailed to the ~~sponsor's designated CRO~~ **SAE coordinator**.	
• Section 11.8	Throughout the Appendix: Efficacy is planned to be assessed using a video interview between the investigator and the patient. However, it has been added that a **telephone interview** can be performed instead, in case a video interview is not possible. Scheme for Home Visits: Tasks for Investigator • **Confirmation that IMP can be administered** IMP Administration IMP can be administered at home by a qualified person (eg, home nurse) **after confirmation of the investigator**.	Further clarification how to perform a home visit during the COVID-19 pandemic
• Section 11.8.1	• Section 11.8.1 "Schedule of Activities During COVID-19 Pandemic": Addition of the following to footnote "a": … **In case a next cycle is started, all assessments at the week-48 visit are mandatory.**	Explanation that all assessments are mandatory at the week-48 visit for patients who start a new cycle.
• Section 11.9	Section 11.9 "Appendix 9: Administrative Structure": • Project Management and Clinical Research Organization (except for Georgia), and Safety Reporting for ~~Serbia and~~ Japan • Pharmacovigilance (Except Safety Reporting for ~~Serbia and~~ Japan) ~~and Coordination of Data Safety Monitoring Board~~ **Parexel International, 8 Federal Street, Billerica, MA 01821, United States of America** ~~SGS Life Sciences (SGS LS), a division of SGS Belgium NV, Generaal de Wittelaan 19A b5, 2800 Mechelen, Belgium~~ • Data Management and Biostatistics**, and Coordination of Data Safety Monitoring Board**	Updated administrative structure
• Section 12	• Section 12 "References": **Clinical Trials Facilitation and Coordination Group. Recommendations Related to Contraception and Pregnancy Testing in Clinical Trials, Version 1.1. Heads of Medicine Agencies (website). Available at: https://www.hma.eu/fileadmin/dateien/Human_Medicines/01-About_HMA/Working_Groups/CTFG/2020_09_HMA_CTFG_Contraception_guidance_Version_1.1_updated.pdf. Published 21 September 2020, accessed 10 November 2020.** Efgartigimod (ARGX-113) Investigator's Brochure, version ~~8~~**9**.0, ~~April~~**November** 2020.	Updated references to add contraception/pregnancy recommendations and to the new edition of IB.

General Amendment 2, Global Protocol Version 3.0:

The major changes from Protocol Version 2.0 compared to Protocol Version 3.0 are summarized in the table below. Deleted text is indicated in strikethrough; a bold and underlined font is used to indicate added text. Minor administrative editorial changes (eg, to improve readability or correct spelling mistakes) are not summarized in the following table:

Summary of Changes Between Protocol Version 2.0 and Protocol Version 3.0

Section(s)	Change	Rationale
• Page 2, Section 11.8 “Appendix 8”	A notification has been added in the beginning of the protocol (on page 2) that adaptations can be made during the COVID-19 pandemic. An Appendix has been added to this protocol with the minimum assessments required to perform the trial during the COVID-19 pandemic in case the patient cannot attend the visit at the trial site.	Based on this risk-benefit assessment, the clinical study ARGX-113-1902 can be conducted, however, because of the pandemic it may not be possible to perform all assessments as planned for this trial (as outlined in the Schedule of Activities). Therefore, an Appendix to the protocol for this trial has been developed, with the minimum assessments required to guarantee the safety and well-being of the patients during the trial, and to secure the collection of the critical parameters for analysis.
• Footnote “h” from Schedule of Activities and Section 9.3.1	• Schedule of Activities: h. Includes physical examination, weight, **<u>semisupine</u>** blood pressure~~,~~ **<u>and</u>** heart rate, and body temperature. Weight will be measured at the SD1 visit and during the trial in case of an obvious weight change. • Section 9.3.1 “Vital Signs and Physical Examination”: The assessment of vital signs (~~supine~~ systolic and diastolic blood pressure, heart rate, and body temperature) will be performed at the time points indicated in the SoA (see Section 1.3). Vital signs will be measured before blood collections. Blood pressure (~~supine~~ systolic and diastolic) and heart rate will be measured using standard equipment in semisupine position after having rested for at least 5 minutes.	The vital signs blood pressure and heart rate are measured in a semi-supine position.
• Schedule of Activities, Section 5.1, Section 9.10, Section 11.2 “Appendix 2”	• Schedule of Activities: **<u>l. Optional blood samples will be taken for potential future biomarker analysis.</u>** n. IMP will be administered at the site at the scheduled trial visits. This will be after blood samples have been taken for laboratory safety, PK, PD, immunogenicity, and/or **<u>optional</u>** biomarker analyses. • Section 5.1 “Overall Design”: In addition, serum samples will be stored for potential future analysis of biomarkers **<u>from all patients who provided separate consent for biomarker analysis</u>**.	Clarification that samples for biomarker analysis are optional.

Section(s)	Change	Rationale
	• Section 9.10 "Biomarkers": Blood samples (serum) will be collected **<u>from all patients who provided separate consent for biomarker analysis</u>** at the time points provided in the SoA… • Section 11.2 "Appendix 2": * Blood samples for potential future biomarker analysis will be collected from all patients who provided separate consent for biomarker analysis.	
• Section 3.3.2, Section 12	• Section 3.3.2 "Risks": ~~No deaths have occurred in any efgartigimod clinical trial.~~ • Section 12 "References": 10. Efgartigimod (ARGX-113) Investigator's Brochure, version ~~7~~**<u>8</u>**.0, **April 2020** ~~June 2019~~.	Sentence deleted to align with the current version of the Investigator's Brochure (version 8.0).
• Section 3.3.2, Section 10.6, Section 11.1.5	• Section 3.3.2 "Risks": **<u>An independent Data Safety Monitoring Board (DSMB) will be responsible for ongoing safety monitoring during the trial and will meet on a regular basis (see Section 11.6).</u>** • Section 10.6 "Independent Data Safety Monitoring Board (DSMB)": **<u>10.6. Independent Data Safety Monitoring Board (DSMB)</u>** **<u>Details on the DSMB are provided in Section 12.1.5 (Appendix 1).</u>** • Section 11.1.5 "Independent Data Safety Monitoring Board (DSMB)": **<u>11.1.5. Independent Data Safety Monitoring Board (DSMB)</u>** **<u>An independent DSMB will be used in this trial (as well as for the preceding ARGX-113-1802 trial).</u>** **<u>The sponsor will appoint a DSMB consisting of an independent group of clinical experts, who are not participating in the trial. They will be supplemented by an independent statistician. The objective of the DSMB will be to review all safety data (including the overall number of patients treated up to that point, rates, and patient-level details).</u>** **<u>The DSMB will review data from the ARGX-113-1802 and ARGX-113-1902 trials together before the ARGX-113-1802 trial is completed. Thereafter, data from the ARGX-113-1902 trial will be presented to the board every 6 months or otherwise as specified by the DSMB, based on findings from the ARGX-113-1802 trial. In addition, ad hoc meetings can be requested at any time during the trial by either the sponsor or the DSMB.</u>**	An independent Data Safety Monitoring Board (DSMB) appointed for the preceding ARGX-113-1802 trial will also review the safety data of this open-label extension trial (ARGX-113-1902).

Section(s)	Change	Rationale
	The DSMB will advise the sponsor concerning continuation, modification, or termination of the trial after every meeting. **The composition, objectives, and role and responsibilities of the DSMB will be described in a charter. The DSMB charter will also define and document the content of the safety summaries and general procedures (including communications).**	
• Section 5.1, Section 7.6, Section 8.3	• Section 5.1 “Overall Design”: Patients will be treated as considered appropriate by the **patient’s treating physician** ~~investigator~~, after they are withdrawn from the trial. • Section 7.6 “Intervention After the End of the Trial”: After the end of the trial, patients will no longer receive IMP and will be treated as considered appropriate by the **patient’s treating physician** ~~investigator~~. • Section 8.3 “Early Withdrawal From the Trial” The **patient’s** treating physician will then offer the patient the best available alternative treatment.	The patient’s treating physician will decide the appropriate CIDP treatment after the patient is withdrawn from the trial.
• Section 7.4.1 and Section 7.4.2	• Section 7.4.1 “Permitted Medication”: Any kind of vaccination**, except for live-attenuated vaccines,** is allowed at the discretion of the investigator. • Section 7.4.2 “Prohibited Medications”: • **Live-attenuated vaccines (Note: Receiving an inactivated, sub-unit, polysaccharide, or conjugate vaccine any time before screening is not prohibited).**	Live-attenuated vaccines are not allowed from 28 days before screening through 28 days after the last IMP dose, consistent with the updated text in the efgartigimod Investigator’s Brochure.
• Sections 9.2.3 and 9.2.4	• Section 9.2.3 “Inflammatory Rasch-built Overall Disability Scale (I-RODS)” and Section 9.2.4 “Mean Grip Strength”: ~~When assessed at home, the patient will record the measurements.~~	The Inflammatory Rasch-built Overall Disability Scale (I-RODS) and mean grip strength do not need to be documented at home in this trial.
• Section 11.1.3	• Section 11.1.3 “Informed Consent Process” … legally authorized representative **(according to local regulations)** …(3 times)	Adapted because “legal representatives” are not allowed in some countries.
• Section 11.4.4	• Section 11.4.4 “Reporting of SAEs”: ~~**SAE Reporting to SGS via an Electronic Data Collection Tool**~~ ~~The primary mechanism for reporting an SAE to SGS will be the electronic data collection tool.~~ ~~If the electronic system is unavailable, then the site will use the paper SAE data collection tool (see next section) in order to report the event within 24 hours.~~ ~~The site will enter the SAE data into the electronic system as soon as it becomes available.~~	Clarification of SAE reporting system.

Section(s)	Change	Rationale
	~~After the trial is completed at a given site, the electronic data collection tool will be taken off-line to prevent the entry of new data or changes to existing data.~~ ~~If a site receives a report of a new SAE from a trial patient or receives updated data on a previously reported SAE after the electronic data collection tool has been taken off-line, then the site can report this information on a paper SAE form (see next section) or to SGS by telephone.~~ ~~Contacts for SAE reporting can be found on the frontpage.~~ **SAE Reporting ~~to SGS~~ via Paper Form** … **<u>All SAEs will be recorded on the paper SAE report form and the AE form on the eCRF. The investigator or designated site staff should check that all entered data are consistent. An alert email for the SAE report on the eCRF will then automatically be sent by email to the sponsor's designated CRO safety mailbox via the electronic data capture (EDC) system. The paper SAE report form should be faxed or emailed to the sponsor's designated CRO.</u>**	
• Section 11.9 "Appendix 9"	An Appendix has been added to this protocol with the administrative structure.	For completeness, the vendors contracted by argenx have been added.

General Amendment 1, Global Protocol Version 2.0:

The major changes from Protocol Version 1.0 compared to Protocol Version 2.0 are summarized in the table below. Deleted text is indicated in strikethrough; a bold and underlined font is used to indicate added text. Minor administrative editorial changes (eg, to improve readability or correct spelling mistakes) are not summarized in the following table:

Summary of Changes Between Protocol Version 1.0 and Protocol Version 2.0

Section(s)	Change	Rationale
• General change throughout the protocol: Simplification of 1 of the entry criteria for ARGX-113-1902	Patients will be given the opportunity to continue efgartigimod PH20 SC treatment in this OLE trial in the event any of the following 4 conditions are occurring, provided the patient has not permanently discontinued IMP: • The patient experiences a clinical deterioration, (ie, ~~confirmed~~ increase [worsening] in adjusted Inflammatory Neuropathy Cause and Treatment [INCAT] disability score **<u>in Stage B of ARGX-113-1802 as described in the ARGX-113-1802</u>**	Simplification of wording for entry criteria.

Section(s)	Change	Rationale
	protocol, ~~further referred to as ‘adjusted INCAT score,’ by ≥1 point with respect to the Stage B baseline of the ARGX-113-1802 trial~~	
• General change throughout the protocol: The safety follow-up visit should be performed 28 days after the last IMP dose	When the patient will complete the OLE trial (after 1 or more 48-week treatment cycles), he/she will have a safety follow-up visit 28 days (±3 days) after ~~week 48~~ **the last IMP dose** (of the last treatment cycle). The follow-up visit will occur 28 days (±3 days) ~~after the week 48 visit or~~ after the last IMP injection. If the patient stops the OLE trial after 1 treatment cycle, the total trial duration is ~~52~~ **51** weeks.	The safety follow-up visit should be performed 28 days after the last IMP dose instead of 28 days after the week-48 visit. As the last IMP dose is administered at week 47, the safety follow-up visit should be performed at week 51, instead of week 52.
• Title page	Addition of the logo of the trial.	Addition of the logo of the trial.
• Time window for the W48/ED visit (Schedule of Activities, Section 5.1)	**−2 to +14 days**	Time window for the W48/ED visit has been added.
• Footnote “d” to the Schedule of Activities	**W48/ED visit of a previous treatment cycle will coincide with the SD visit of the next treatment cycle.**	Clarification that these 2 visits will coincide
• Section 5.1.1	Once the patient has been included in the OLE trial and all assessments have been completed (as described in the SoA in Section 1.3), the patient will receive **refresher** training on self-administration~~, if needed~~, and **perform** the first administration ~~will be performed~~ under the supervision of a professional. **For patients who are unable to self-administer IMP, the site staff will administer IMP.**	Clarification for administration of the first IMP dose in this trial.

Section(s)	Change	Rationale
• Section 5.1.3	The visit at week 48 ~~will~~ **<u>can</u>** be on the same day as SD1 visit for next consecutive cycle for those patients who are eligible and willing to continue the study treatment in a following treatment cycle. **<u>The period between the week-48 visit and the SDI visit of a new treatment cycle should be within 14 days.</u>**	Clarification of the allow period between a previous and a new treatment cycle.
• Section 5.1.6	An unscheduled visit is to be planned for safety reasons ~~and for confirmation of clinical deterioration (increase in adjusted INCAT score by ≥1 point compared to SD1)~~, if no trial visit is planned at that time.	A deterioration of the adjusted INCAT score does not need to be confirmed in this trial and no unscheduled visit should therefore be planned.
• Section 7.4.2	If the administration of a non-permitted concomitant drug becomes necessary during the clinical trial, eg, because of AEs, the patient has <u>to</u> **<u>discontinue IMP</u>** ~~be withdrawn from the trial~~, as described in Section **<u>7.1</u>**~~7.2~~.	In case non-permitted concomitant drug is taken, the patient has to discontinue IMP (not the trial).
• Section 8.3 • Section 8.2 (reference to Section 8.3)	A patient **must** be withdrawn from further trial participation in the event of any of the following: • For patients who permanently discontinued IMP and continue the trial-related assessments for the remaining visits of the treatment cycle according to the SoA (see Section 1.3): In case they deteriorate (defined as a**<u>n</u>** ~~confirmed~~ increase of adjusted INCAT score by ≥1 point with respect to baseline of their current stage).	Confirmation of deterioration is not needed to stop the ARGX-113-1902 trial for patients who permanently discontinued IMP and deteriorate.
• Section 9.2.1	**<u>For the "adjusted" INCAT score, changes in the function of the upper limbs from 0 (normal) to 1 (minor symptoms) or from 1 to 0 will not be recorded as deterioration or improvement because these changes are not considered clinically significant.</u>**	Addition of definition of the "adjusted" INCAT score.
• Section 9.2.2 and	The MRC sum score is evaluated bilaterally on 6 muscle groups of upper and lower limbs in order to obtain a summed score between 0 and 60:	Updated to the original names of the movement of the muscles.

Section(s)	Change	Rationale
Section 11.6 ‘Appendix 6’	• **Arm** abductors~~ion of the arm~~ • **Elbow** flexors~~ion of the forearm~~ • **Wrist** extensors~~ion of the wrist~~ • **Hip** flexors~~ion of the leg or hip flexion~~ • **Knee** extensors~~ion of the knee~~ • **Foot** dorsiflexors~~al flexion of the foot~~	

10.11. Appendix 11: Country-Specific Requirements

10.11.1. Germany-Specific Protocol

The Germany-specific protocol requirements are summarized in the table below. Changes from the global protocol are indicated as follows: deleted text is indicated in strikethrough; a bold and underlined font is used to indicate added text.

Germany-Specific Protocol Requirements

Section(s)	Change vs Global Protocol	Rationale
• Throughout the protocol (ie, Synopsis, Section 1.3 [Schedule of Activities], Section 5.1, Section 5.1.4, Section 5.1.5, Section 5.2, Section 8.2, Section 8.3, Section 11.8 [Appendix 8]), Section 11.9 [Appendix 9])	• Extension of safety follow-up period from 28 days to 42 days: …~~28~~ <u>**42**</u> days…	Based upon feedback of the German Competent Authority (Paul Ehrlich Institut), the safety follow-up duration is extended from 28 days to 42 days.

10.11.2. UK-Specific Protocol

The UK-specific requirements are summarized in the table below. Changes from the global protocol are indicated as follows: deleted text is indicated in strikethrough; a bold and underlined font is used to indicate added text.

UK-Specific Protocol Requirements

Section(s)	Change vs Global Protocol	Rationale
• Section 8.2	• Psychiatric conditions. ***Note****: In the case of severe depression, suicidal ideation, or attempted suicide, trial medication must be at least temporarily interrupted until the patient's psychiatric condition has been fully evaluated*~~*. A*~~ <u>***by a***</u> *psychiatrist* ~~*should be consulted whenever necessary*~~*. Treatment may be resumed only* <u>***when psychiatric condition has been excluded and***</u> ~~*after consultation*~~ *with* ~~*the*~~ *sponsor*<u>***'s agreement***</u>*.*	Based upon feedback of the MHRA, in case of possible depression, the trial medication will be discontinued, and the patient will be referred to psychiatrist before trial medication can be resumed.

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